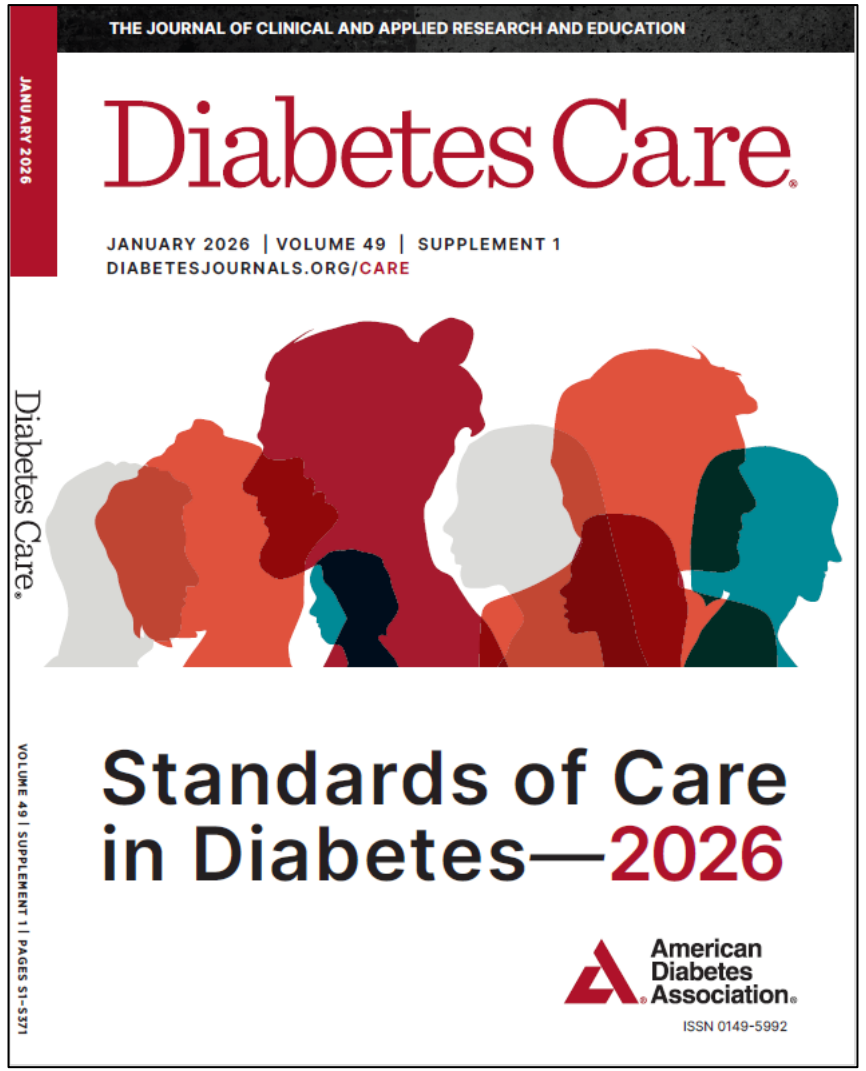
The American Diabetes Association's *Standards of Care in Diabetes—2026*

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Standards of Care in Diabetes—2026

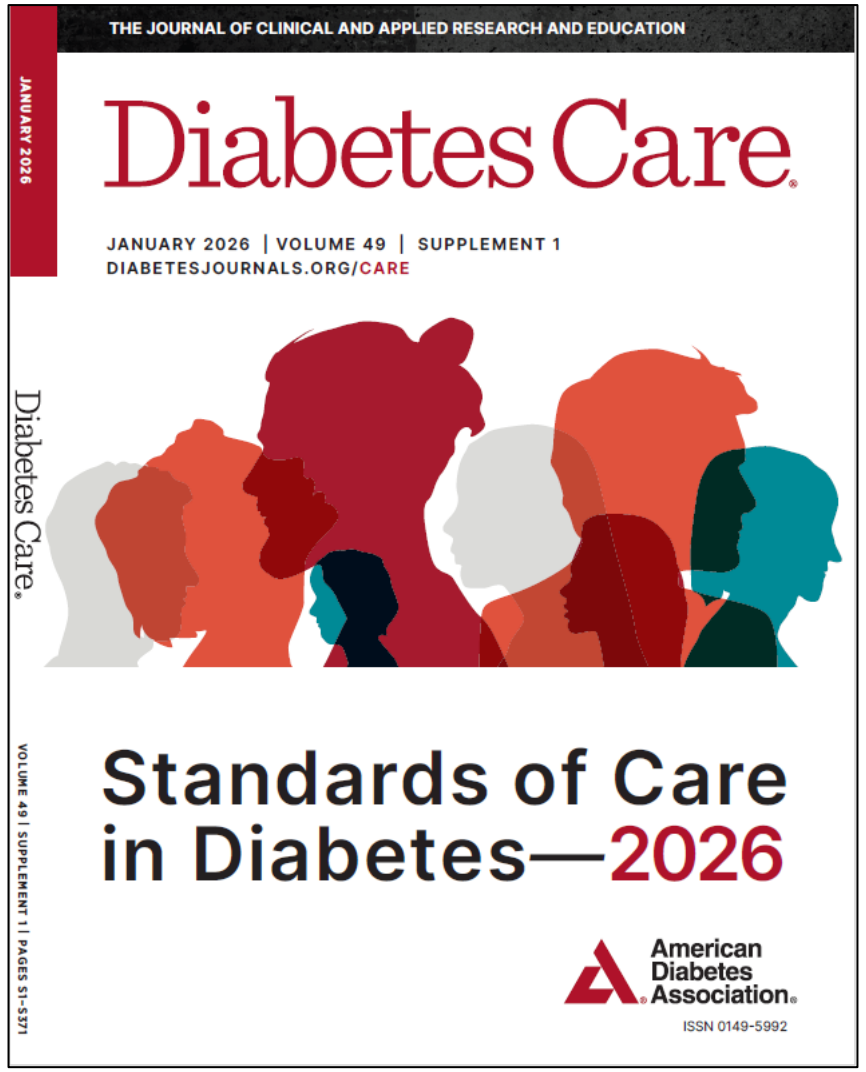
(*Standards of Care*)



American Diabetes Association

Standards of Care in Diabetes—2026

Intended to provide clinicians, patients, researchers, payers, and other interested individuals with the components of diabetes care, general treatment goals, and tools to evaluate the quality of care.



American Diabetes Association

EVIDENCE



- Search of scientific diabetes literature over past year
- Recommendations revised per new evidence

PROCESS



- Professional Practice Committee
- External peer review
- Reviewed by the ADA's Board of Directors
- Living Standards

FUNDING



- Funded out of the ADA's general revenues
- Does not use industry support

American Diabetes Association.

The ADA's *Standards of Care*—a Living Document

- Beginning with the ADA's 2018 *Standards of Care in Diabetes*, the Standards document became a "living" document where notable updates are incorporated
- A description of the Living Standards updates is available at professional.diabetes.org/standards-of-care/living-standards-update

American Diabetes Association.

The ADA's *Standards of Care*—a Living Document

- Updates will be made in response to important events inclusive of, but not limited to:
  - Approval of new treatments (medications or devices) with the potential to impact care;
  - Publication of new findings that support a change to a recommendation and/or evidence level of a recommendation; or
  - Publication of a consensus document endorsed by the ADA that necessitates an update of the Standards to align content of the documents

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Evidence-Grading System

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Evidence-Grading System

Table 1—ADA evidence-grading system for "Standards of Care in Diabetes"

Level of evidence	Description
A	Clear evidence from well-conducted, generalizable randomized controlled trials that are adequately powered, including: • Evidence from a well-conducted multicenter trial • Evidence from a meta-analysis that incorporated quality ratings in the analysis Supportive evidence from well-conducted randomized controlled trials that are adequately powered, including: • Evidence from a well-conducted multicenter trial • Evidence from a meta-analysis that incorporated quality ratings in the analysis
B	Evidence from randomized controlled trials with an identified flaw that may limit validity of the results Supportive evidence from well-conducted cohort studies, including: • Evidence from a well-conducted prospective cohort study or registry • Evidence from a well-conducted meta-analysis of cohort studies Supportive evidence from a well-conducted case-control study
C	Supportive evidence from poorly controlled or uncontrolled studies, including: • Evidence from randomized clinical trials with one or more major or three or more minor methodological flaws that could invalidate the results • Evidence from observational studies with high potential for bias (such as case series with comparison with historical controls) • Evidence from case series or case reports Conflicting evidence with the weight of evidence supporting the recommendation
E	Expert consensus or clinical experience

Introduction:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S1-S5

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Table of Contents

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Table of Contents

1. Improving Care and Promoting Health in Populations
2. Diagnosis and Classification of Diabetes
3. Prevention or Delay of Diabetes and Associated Comorbidities
4. Comprehensive Medical Evaluation and Assessment of Comorbidities
5. Facilitating Positive Health Behaviors and Well-being to Improve Health Outcomes
6. Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
7. Diabetes Technology
8. Obesity and Weight Management for the Prevention and Treatment of Diabetes
9. Pharmacologic Approaches to Glycemic Treatment
10. Cardiovascular Disease and Risk Management
11. Chronic Kidney Disease and Risk Management
12. Retinopathy, Neuropathy, and Foot Care
13. Older Adults
14. Children and Adolescents
15. Management of Diabetes in Pregnancy
16. Diabetes Care in the Hospital
17. Diabetes and Advocacy

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Section 1.

Improving Care and Promoting Health in Populations

(https://doi.org/10.2337/dc26-S001)

Diabetes and Population Health

1.1 Ensure treatment decisions are timely, rely on evidence-based guidelines, address social determinants ofhealth, and incorporate shared decision-making based on individual values, preferences, prognoses,comorbidities, and informed financial considerations. **B**

1.2 Align approaches to diabetes management with evidence-based care models. These models emphasize person-centered team care, integrated long-term treatment approaches to diabetes and comorbidities, and ongoing collaborative communication and goal setting among all team members and with people with diabetes. **A**

1.3 Care systems should facilitate in-person and virtual team-based care, include those knowledgeable and experienced in diabetes management as part of the team, and utilize patient registries, decision support tools, proactive care planning, and community involvement to meet needs of individuals with diabetes. **B**

1.4 Assess diabetes management, risk factors, and complications using reliable and relevant data metrics to improve processes of care and health outcomes, with attention to individual values, preferences, goals for care, and treatment burden including costs of care. **B**

1.5 Health systems should adopt a culture of continuous quality improvement, implement benchmarking programs, and engage interprofessional teams to support sustainable and scalable process changes to improve quality of care and health outcomes. **A**

Tailoring Treatment for Social Context

1.6 Health systems should assess and address gaps in diabetes care and health outcomes (e.g., by stratifying clinical quality data by factors such as insurance status, race, ethnicity, preferred language for health care discussions, disability, and social determinants of health). **B**

1.7 During clinical encounters, assess for social determinants of health, including food insecurity, **A** housing insecurity, financial barriers, health insurance and health care access, environmental and neighborhood factors, and social capital/social community support, **B** to inform treatment decisions, with referral to appropriate local community resources.

1.8 Provide people with diabetes additional self-management support from lay health coaches, navigators, or community health workers when available. **A** Digital self-management tools or coaches may be considered as appropriate. **B**

1.9 Consider the involvement of community health workers to support management of diabetes and cardiovascular and kidney risk factors, especially in underserved communities and health care systems. **B**

Table 1.1—Considerations for engaging interprofessional members of a comprehensive, person-centered diabetes care team to identify and meet the needs of people with diabetes across the life span

Subpopulation of a person with diabetes	Team members to engage in care	Unique care considerations
All adults with diabetes	Primary care clinician, CDCES, RDN, and other specialists as available and appropriate to treat comorbidities (**Table 4.1**)	Assess for and address social determinants of health.
Adults treated with intensive insulin therapy, including multiple daily injections of insulin and insulin pump therapy	Clinicians and other health care team members experienced in advanced diabetes management, including technology use	
All children and adolescents with diabetes	Primary care clinician, pediatric endocrinologist, CDCES, RDN, other specialists as available and appropriate to treat comorbidities (**Table 14.1**), daycare or school nurse or other professional, behavioral health professional (as needed), and parent(s) or caregiver(s)	Assess for and address social determinants of health and barriers to safety, well-being, and academic performance in school. Engage professionals within the school and extracurricular/after-school activities to ensure safe diabetes management. An individualized diabetes medical management plan should be developed in collaboration with school professionals and parent(s) or caregiver(s). Support gradual developmentally appropriate transfer of self-management from caregivers to the children and adolescents with diabetes.

Table 1.1
Improving Care and Promoting Health in Populations:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S13-S26

Individuals with diabetes and diabetes-related complications or comorbidities	Specialist referrals as appropriate and available (e.g., behavioral health professional, cardiologist, endocrinologist, eye specialist, gastroenterologist or hepatologist, neurologist, nephrologist, obesity medicine specialist, or podiatrist), care coordinator/navigator or case manager, and clinical pharmacist (for those with polypharmacy or complex medication plans)	Screen for functional, cognitive, financial, and logistical barriers to self-management and evidence that self-care demands exceed capacity and available resources and support systems.
Individuals with social and/or structural barriers to care	Care coordinator/navigator, social services professional, insurance specialist/navigator, peer-to-peer support (as available), community health worker and/or community paramedic (as available), public health professional, and interpreter (as applicable)	Consider each person's psychosocial needs, available resources, and support systems.
Older adults	Geriatric medicine specialist, clinical pharmacist (for those with polypharmacy or complex medication plans), CDCES and/or RDN with expertise in the care for older adults, social services professional, case manager, community services provider, and physical and/or occupational therapist as available and appropriate based on functional status and independence	Consider the older adult's nutritional status, including ability to afford (financial barriers), acquire (accessibility), prepare (cooking), and consume (oral health) nutritious food. Assess for and address needs related to vision, hearing, dexterity, cognition, mobility, and other challenges.

Table 1.1 (continued)
Improving Care and Promoting Health in Populations:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S13-S26

Table 1.1 (continued)

Individuals in long-term care settings	Long-term care facility clinicians, nurses, other health care professionals, physical and occupational therapists, and RDN	Engage professionals within the long-term care facility to ensure safe and appropriate diabetes management.
Pregnant individuals with diabetes	Maternal-fetal medicine specialist or obstetrician experienced in the care of pregnant individuals with diabetes (particularly for individuals with type 1 diabetes or requiring intensive insulin therapy), CDCES, RDN, eye specialist (particularly for individuals with preexisting type 1 or type 2 diabetes), other specialists as appropriate, and lactation consultant as appropriate	Ensure appropriate postpartum follow-up and care, including transition from obstetric care to established primary care.
Individuals with behavioral health conditions	Behavioral health professional, care coordinator/ navigator, and social services professional as age and situation appropriate	Use age- and situation-appropriate screening protocols for general and diabetes-related psychosocial concerns.

CDCES, certified diabetes care and education specialist; RDN, registered dietitian nutritionist.

Table 1.1
Improving Care and Promoting Health in Populations:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S13-S26

Section 2.

Diagnosis and Classification of Diabetes

(https://doi.org/10.2337/dc26-S002)

Diagnostic Tests for Diabetes

2.1a Diagnose diabetes based on A1C or plasma glucose criteria. Plasma glucose criteria include either fasting plasma glucose (FPG), 2-h plasma glucose (2-h PG) during a 75-g oral glucose tolerance test (OGTT), or random glucose accompanied by classic hyperglycemic symptoms or crises (**Table 2.1**). **B**

2.1b In the absence of unequivocal hyperglycemia (e.g., hyperglycemic crises), diagnosis requires confirmatory testing (**Table 2.1**). **B**

Table 2.1—Criteria for the diagnosis of diabetes in nonpregnant individuals

A1C ≥6.5% (≥48 mmol/mol). The test should be performed in a laboratory using a method that is NGSP certified and standardized to the DCCT assay.*
OR
FPG ≥126 mg/dL (≥7.0 mmol/L). Fasting is defined as no caloric intake for at least 8 h.*
OR
2-h PG ≥200 mg/dL (≥11.1 mmol/L) during OGTT. The test should be performed as described by the WHO, using a glucose load containing the equivalent of 75 g anhydrous glucose dissolved in water.*
OR
In an individual with classic symptoms of hyperglycemia or hyperglycemic crisis, a random plasma glucose ≥200 mg/dL (≥11.1 mmol/L). Random is any time of the day without regard to time since previous meal.

DCCT, Diabetes Control and Complications Trial; FPG, fasting plasma glucose; OGTT, oral glucose tolerance test; NGSP, National Glycohemoglobin Standardization Program; WHO, World Health Organization; 2-h PG, 2-h plasma glucose. *In the absence of unequivocal hyperglycemia, diagnosis requires two abnormal results from different tests which may be obtained at the same time (e.g., A1C and FPG), or the same test at two different time points.

Table 2.1
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Use of A1C for Screening and Diagnosis of Diabetes

2.2a The A1C test should be performed using a method that is certified by the National Glycohemoglobin Standardization Program (NGSP) as traceable to the Diabetes Control and Complications Trial (DCCT) reference assay. **B**

2.2b Point-of-care A1C testing for diabetes screening and diagnosis should be restricted to devices approved for diagnosis by the U.S. Food and Drug Administration at Clinical Laboratory Improvement Amendments–certified laboratories that perform testing of moderate complexity or higher by trained personnel. **B**

2.3 Evaluate for the possibility of a problem or interference with either test when there is consistent and substantial discordance between blood glucose values and A1C test results. **B**

2.4 In conditions associated with an altered relationship between A1C and glycemia, such as some hemoglobin variants, pregnancy, glucose-6-phosphate dehydrogenase deficiency, HIV, and conditions that may alter red blood cell turnover, plasma glucose criteria should be used to diagnose diabetes. **B**

Table 2.2—Criteria defining prediabetes in nonpregnant individuals

A1C 5.7–6.4% (39–47 mmol/mol)
OR
FPG 100 mg/dL (5.6 mmol/L) to 125 mg/dL (6.9 mmol/L) (IFG)
OR
2-h PG during 75-g OGTT 140 mg/dL (7.8 mmol/L) to 199 mg/dL (11.0 mmol/L) (IGT)

For all three tests, risk is continuous, extending below the lower limit of the range and becoming disproportionately greater at the higher end of the range. FPG, fasting plasma glucose; IFG, impaired fasting glucose; IGT, impaired glucose tolerance; OGTT, oral glucose tolerance test; 2-h PG, 2-h plasma glucose.

Table 2.2
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Classification

2.5 Classify people with hyperglycemia into appropriate diagnostic categories to aid in personalized management. **E**

Classification (continued)

- Type 1 diabetes (due to autoimmune β-cell destruction, usually leading to absolute insulin deficiency, including latent autoimmune diabetes in adults)
- Type 2 diabetes (due to a nonautoimmune progressive loss of adequate β-cell insulin secretion, frequently on the background of insulin resistance)
- Specific types of diabetes due to other causes, e.g., monogenic diabetes syndromes, diseases of the exocrine pancreas, and drug- or chemical-induced diabetes
- Gestational diabetes mellitus (diabetes diagnosed in the second or third trimester of pregnancy that was not clearly overt diabetes prior to gestation or other types of diabetes occurring throughout pregnancy, such as type 1 diabetes)

Type 1 Diabetes

2.6 Screen for presymptomatic type 1 diabetes by testing autoantibodies: insulin (IA), glutamic acid decarboxylase (GAD), islet antigen 2 (IA-2), or zinc transporter 8 (ZnT8). **B**

2.7 Autoantibody-based screening for presymptomatic type 1 diabetes should be offered to those with a family history of type 1 diabetes or otherwise known elevated genetic risk. **B**

2.8a Individuals with screening results positive for one or more islet autoantibodies should be evaluated for stage 3 (overt) type 1 diabetes (using A1C, urinalysis, and/or plasma glucose), which would require prompt clinical management and education. **B**

2.8b Individuals with multiple confirmed islet autoantibodies and without overt type 1 diabetes have a high risk for progression to stage 3 type 1 diabetes and should be referred to a specialized center for metabolic staging (**Table 2.4**), education, and consideration of prevention trials or approved treatments (e.g., teplizumab). **E**

Type 1 Diabetes (continued)

2.9 Individuals with a single confirmed IA-2 autoantibody should be monitored similarly to individuals with multiple islet autoantibodies, as IA-2 autoantibody positivity is an independent risk factor for progression. **B** Individuals with a single confirmed islet autoantibody should undergo repeat antibody testing every 6 months to 3 years (depending on age) to assess for persistence or seroconversion. **E**

2.10 Standardized islet autoantibody tests are recommended for classification of diabetes in adults who have phenotypic risk factors that overlap with those for type 1 diabetes (e.g., younger age at diagnosis, unintentional weight loss, ketoacidosis, or short time to insulin treatment). **E**

Table 2.3—Considerations related to the use and interpretation of laboratory measurements of glucose and A1C

	Glucose	A1C
Cost	Inexpensive and available in most laboratories across the world	More expensive than glucose and not as widely available globally
Time frame of hyperglycemia	Acute measure	Chronic measure of glucose exposure over the past ~2–3 months
Preanalytic stability	Poor; plasma must be separated immediately or samples must be kept on ice to prevent glycolysis	Good
Sample	Measurement can vary depending on sample type (plasma, serum, whole blood) and source (capillary, venous, arterial)	Requires whole-blood sample
Assay standardization	Not standardized	Well standardized
Fasting	Fasting or timed samples required	Nonfasting test; no participant preparation is needed
Within-person variability	High	Low
Acute factors that can affect levels	Food intake, stress, recent illness, activity	Unaffected by recent food intake, stress, illness, activity
Other individual factors that can affect test results	Diurnal variation, medications, alcohol, smoking, bilirubin	Altered erythrocyte turnover (e.g., anemia, iron status, splenectomy, blood loss, transfusion, hemolysis, glucose-6-phosphate dehydrogenase deficiency, erythropoietin), HIV, cirrhosis, renal failure, dialysis, pregnancy
Test interferences	Depends on specific assay: sample handling/ processing time, hemolysis, severe hypertriglyceridemia, severe hyperbilirubinemia	Depends on specific assay: hemoglobin variants, severe hypertriglyceridemia, severe hyperbilirubinemia

Data are from Selvin (236).

Table 2.32
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

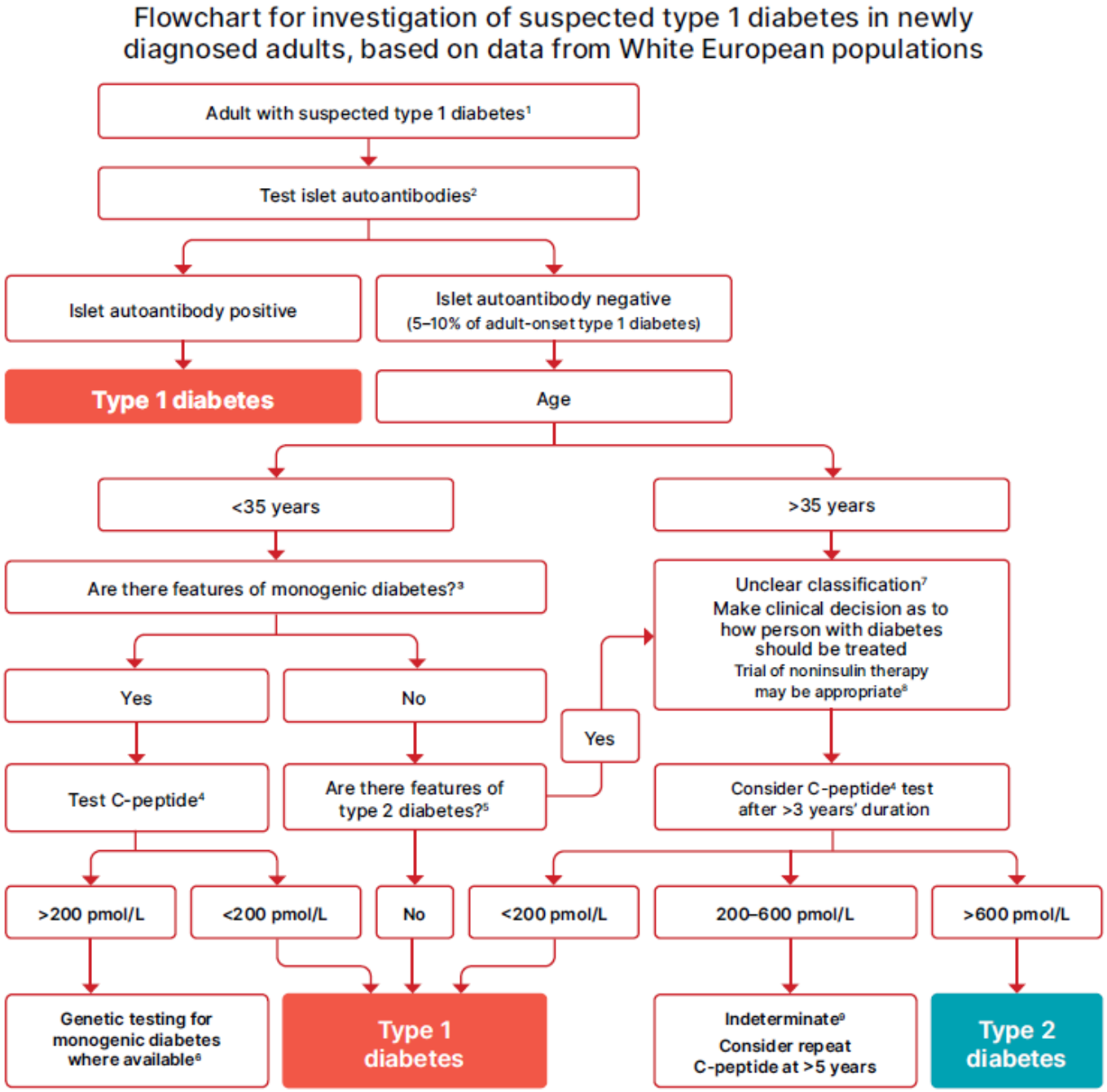


Figure 2.1
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Figure 2.1—Flowchart for investigation of suspected type 1 diabetes in newly diagnosed adults, based on data from White European populations. [1]No single clinical feature confirms type 1 diabetes in isolation. [2]Glutamic acid decarboxylase (GAD) should be the primary antibody measured and, if negative, should be followed by islet tyrosine phosphatase 2 (IA-2) and/or zinc transporter 8 (ZnT8) where these tests are available. In individuals who have not been treated with insulin, antibodies against insulin may also be useful. In those diagnosed at <35 years of age who have no clinical features of type 2 diabetes or monogenic diabetes, a negative result does not change the diagnosis of type 1 diabetes, since 5–10% of people with type 1 diabetes do not have antibodies. [3]Monogenic diabetes is suggested by the presence of one or more of the following features: A1C <58 mmol/mol (<7.5%) at diagnosis, one parent with diabetes, features of a specific monogenic cause (e.g., renal cysts, partial lipodystrophy, maternally inherited deafness, and severe insulin resistance in the absence of obesity), and monogenic diabetes prediction model probability >5% (diabetesgenes.org/exeter-diabetes-app/ModyCalculator). [4]A C-peptide test is only indicated in people receiving insulin treatment. A random sample (with concurrent glucose) within 5 h of eating can replace a formal C-peptide stimulation test in the context of classification. If the result is ≥600 pmol/L (≥1.8 ng/mL), the circumstances of testing do not matter. If the result is <600 pmol/L (<1.8 ng/mL) and the concurrent glucose is <4 mmol/L (<70 mg/dL) or the person may have been fasting, consider repeating the test. Results showing very low levels (e.g., <80 pmol/L [<0.24 ng/mL]) do not need to be repeated. Where a person is insulin treated, C-peptide must be measured prior to insulin discontinuation to exclude severe insulin deficiency. Do not test C-peptide within 2 weeks of a hyperglycemic emergency. [5]Features of type 2 diabetes include increased BMI (≥25 kg/m^2), absence of weight loss, absence of ketoacidosis, and less marked hyperglycemia. Less discriminatory features include non-White ethnicity, family history, longer duration and milder severity of symptoms prior to presentation, features of metabolic syndrome, and absence of a family history of autoimmunity. [6]If genetic testing does not confirm monogenic diabetes, the classification is unclear and a clinical decision should be made about treatment. [7]Type 2 diabetes should be strongly considered in older individuals. In some cases, investigation for pancreatic or other types of diabetes may be appropriate. [8]A person with possible type 1 diabetes who is not treated with insulin will require careful monitoring and education so that insulin can be rapidly initiated in the event of glycemic deterioration. [9]C-peptide values 200–600 pmol/L (0.6–1.8 ng/mL) are usually consistent with type 1 diabetes or maturity-onset diabetes of the young but may occur in insulin-treated type 2 diabetes, particularly in people with normal or low BMI or after long duration. Reprinted and adapted from Holt et al. (32).

Figure 2.1 (continued)
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Prediabetes and Type 2 Diabetes

2.11 Screening for risk of prediabetes and type 2 diabetes with an assessment of risk factors or validated risk calculator should be done in asymptomatic adults. **B**

2.12a Testing for prediabetes or type 2 diabetes in asymptomatic people should be considered in adults of any age with overweight or obesity who have one or more risk factors (**Table 2.5**). **B**

2.12b For all other people, screening should begin at age 35 years. **B**

2.12c In people without prediabetes or diabetes after screening, repeat screening recommended at a minimum of 3-year intervals is reasonable, sooner with symptoms or change in risk (e.g., weight gain). **C**

2.13 To screen for prediabetes and type 2 diabetes, FPG, 2-h PG during 75-g OGTT, and A1C are each appropriate (**Table 2.1** and **Table 2.2**). **B**

Prediabetes and Type 2 Diabetes (continued)

2.14 When using OGTT as a screening tool for prediabetes or diabetes, adequate carbohydrate intake (at least 150 g/day) should be assured for 3 days prior to testing. **C**

2.15 Risk-based screening for prediabetes or type 2 diabetes should be considered after the onset of puberty or after 10 years of age, whichever occurs earlier, in children and adolescents with overweight (BMI ≥ 85th percentile) or obesity (BMI ≥ 95th percentile) and who have one or more risk factors for diabetes. (See **Table 2.6** for evidence grading of risk factors.) **B**

2.16a Consider screening people for prediabetes or diabetes if they are on certain medications, such as statins, thiazide diuretics, and some HIV medications, as these agents are known to increase the risk of these conditions. **C**

2.16b In people who are prescribed second-generation antipsychotic medications, screen for prediabetes and diabetes at baseline and repeat 12–16 weeks after medication initiation or sooner, if clinically indicated, and annually thereafter. **B**

Prediabetes and Type 2 Diabetes (continued)

2.17 People with HIV should be screened for diabetes and prediabetes with an FPG test before starting antiretroviral therapy, at the time of switching antiretroviral therapy, and 3–6 months after starting or switching antiretroviral therapy. If initial screening results are normal, FPG should be checked annually. **E**

2.18 Monitor postprandial or random glucose levels with recurrent or long-term use of glucocorticoids. **B**

Table 2.4—Staging of type 1 diabetes

	Stage 1	Stage 2	Stage 3
Characteristics	• Autoimmunity • Normoglycemia • Presymptomatic	• Autoimmunity • Dysglycemia • Presymptomatic	• Autoimmunity • Overt hyperglycemia • Symptomatic
Diagnostic criteria	• Multiple islet autoantibodies • No IGT or IFG, normal A1C	• Islet autoantibodies (usually multiple) • Dysglycemia: ◦ IFG: FPG 100–125 mg/dL (5.6–6.9 mmol/L) or ◦ IGT: 2-h PG 140–199 mg/dL (7.8–11.0 mmol/L) or ◦ A1C 5.7–6.4% (39–47 mmol/mol) or ≥10% increase in A1C	• Autoantibodies may become absent • Diabetes by standard criteria

Adapted from Skyler et al. (35). FPG, fasting plasma glucose; IFG, impaired fasting glucose; IGT, impaired glucose tolerance; 2-h PG, 2-h plasma glucose. Alternative additional stage 2 diagnostic criteria of 30-, 60-, or 90-min plasma glucose on oral glucose tolerance test ≥200 mg/dL (≥11.1 mmol/L) and confirmatory testing in those aged ≥18 years have been used in clinical trials (82). Dysglycemia can be defined by one or more criteria as outlined in the table.

Table 2.4
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Table 2.5—Criteria for screening for diabetes or prediabetes in asymptomatic adults

1. Testing should be considered in adults with overweight or obesity (BMI ≥25 kg/m^2 or ≥23 kg/m^2 in individuals of Asian ancestry) who have one or more of the following risk factors:
   - First-degree relative with diabetes
   - High-risk race, ethnicity, and ancestry (e.g., African American, Latino, Native American, Asian American)
   - History of cardiovascular disease
   - Hypertension (≥130/80 mmHg or on therapy for hypertension)
   - HDL cholesterol level <35 mg/dL (<0.9 mmol/L) and/or triglyceride level >250 mg/dL (>2.8 mmol/L)
   - Individuals with polycystic ovary syndrome
   - Physical inactivity
   - Other clinical conditions associated with insulin resistance (e.g., severe obesity, acanthosis nigricans, metabolic dysfunction–associated steatotic liver disease)
2. People with prediabetes (A1C ≥5.7% [≥39 mmol/mol], IGT, or IFG) should be tested yearly.
3. People who were diagnosed with GDM should have testing at least every 1–3 years.
4. For all other people, testing should begin at age 35 years.
5. If results are normal, testing should be repeated at a minimum of 3-year intervals, with consideration of more frequent testing depending on initial results and risk status.
6. Individuals in other high-risk groups (e.g., people with HIV, exposure to high-risk medicines, evidence of periodontal disease, history of pancreatitis) should also be closely monitored

GDM, gestational diabetes mellitus; IFG, impaired fasting glucose; IGT, impaired glucose tolerance.

Table 2.5
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Table 2.6—Risk-based screening for type 2 diabetes or prediabetes in asymptomatic children and adolescents in a clinical setting

Screening should be considered in youth* who have overweight (≥85th percentile) or obesity (≥95th percentile) and who have one or more additional risk factors:

- Maternal history of diabetes or GDM during the child's gestation
- Family history of type 2 diabetes in first- or second-degree relative
- High-risk race, ethnicity, and ancestry (see **Table 2.5**)
- Signs of insulin resistance or conditions associated with insulin resistance (acanthosis nigricans, hypertension, dyslipidemia, polycystic ovary syndrome, large- or small-for-gestational-age birth weight)

GDM, gestational diabetes mellitus. *After the onset of puberty or after 10 years of age, whichever occurs earlier. If tests are normal, repeat testing at a minimum of 3-year intervals (or more frequently if BMI is increasing or risk factor profile is deteriorating) is recommended. Reports of type 2 diabetes before age 10 years exist, and this can be considered with numerous risk factors.

Table 2.6
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Diabetes Induced by Systemic Anti-Cancer Therapy

2.19 People starting cancer treatment with immune checkpoint inhibitors (ICI), including anti-PD-1 or anti-PDL-1 therapy (e.g., nivolumab, pembrolizumab, avelumab), phosphoinositidylinositol 3-kinase α (PI3Kα) inhibitors (e.g., alpelisib, inavolisib), or mammalian target of rapamycin (mTOR) inhibitors (e.g., everolimus), should be educated regarding risks, symptoms, and signs of hyperglycemia and hyperglycemic crises. **E**

2.20 In people treated with ICIs, fasting or random plasma glucose should be tested before initiating treatment, during each visit, or if symptoms and signs of hyperglycemia develop during or after treatment cessation. **E**

2.21 In people treated with PI3Kα inhibitors, fasting or random plasma glucose and A1C should be tested before initiating treatment, and random plasma glucose should be tested weekly for the first 2 weeks of treatment and then every 4 weeks during treatment. **C** Consider testing A1C every 3 months during treatment. **E**

2.22 In people treated with mTOR inhibitors, fasting or random plasma glucose should be tested before starting and at each visit throughout the duration of treatment. Consider testing A1C every 3 months during treatment. **C**

Pancreatic Diabetes or Diabetes in the Context of the Exocrine Pancreas

2.23 Screen people for diabetes within 3–6 months following an episode of acute pancreatitis and annually thereafter. Screening for diabetes is recommended annually for people with chronic pancreatitis. **E**

Cystic Fibrosis-Related Diabetes

2.24a Annual screening for cystic fibrosis–related diabetes (CFRD) should begin by age 10 years in all people with cystic fibrosis, preferably using OGTT. **B**

2.24b If an OGTT is not feasible, A1C can be used as an alternative method as part of a two-step screening strategy. Individuals with A1C values between 5.5% and 6.4% (37 and 47 mmol/mol, respectively) should undergo an OGTT within 3 months. **C** An A1C value of ≥6.5% (≥48 mmol/mol) is consistent with a diagnosis of CFRD. **B**

2.25 Beginning 5 years after the diagnosis of CFRD, annual monitoring for complications of diabetes is recommended. **E**

PosttransIplantation Diabetes Mellitus

2.26 After organ transplantation, screening for hyperglycemia should be done. A formal diagnosis of posttransplantation diabetes mellitus (PTDM) is best made once the individual is stable on an immunosuppressive plan and in the absence of an acute infection. **B**

2.27 The OGTT is the preferred test to make a diagnosis of PTDM. **B**

2.28 Immunosuppressive plans shown to provide the best outcomes for individuals and graft survival should be used, irrespective of PTDM risk. **E**

Monogenic Diabetes Syndromes

2.29a Regardless of current age, all people diagnosed with diabetes in the first 6 months of life should have genetic testing for neonatal diabetes. **A**

2.29b Children and young adults who do not have typical characteristics of type 1 or type 2 diabetes and have a family history of diabetes in successive generations (suggestive of an autosomal-dominant pattern of inheritance) should have genetic testing for maturity-onset diabetes of the young (MODY). **A**

2.29c In both instances, consultation with a center specializing in diabetes genetics is recommended to understand the significance of genetic mutations and how best to approach further evaluation, treatment, and genetic counseling. **E**

Table 2.7—Most common causes of monogenic diabetes

	Gene	Inheritance	Clinical features
MODY	*HNF1A*	AD	HNF1A-MODY: progressive insulin secretory defect with presentation in adolescence or early adulthood; lowered renal threshold for glucosuria; large rise in 2-h PG level on OGTT (>90 mg/dL [>5 mmol/L]); low hs-CRP; sensitive to sulfonylureas
	GCK	AD	GCK-MODY: higher glucose threshold (set point) for glucose-stimulated insulin secretion, causing stable, nonprogressive elevated fasting blood glucose; typically does not require treatment in nonpregnant individuals; microvascular complications are rare; small rise in 2-h PG level on OGTT (<54 mg/dL [<3 mmol/L])
	HNF4A	AD	HNF4A-MODY: progressive insulin secretory defect with presentation in adolescence or early adulthood; may have large birth weight (macrosomia) and transient neonatal hypoglycemia; sensitive to sulfonylureas
	HNF1B	AD	HNF1B-MODY: developmental renal disease (typically cystic); genitourinary abnormalities; atrophy of the pancreas; hyperuricemia; gout
Neonatal diabetes			
Permanent	*KCNJ11*	AD	IUGR; possible developmental delay and seizures; responsive to sulfonylureas
	ABCC8	AD	IUGR; rarely developmental delay; responsive to sulfonylureas
	INS	AD	IUGR; insulin requiring
	FOXP3	X-linked	Immunodysregulation, polyendocrinopathy, enteropathy X-linked (IPEX) syndrome: autoimmune diabetes, autoimmune thyroid disease, exfoliative dermatitis; insulin requiring
Transient	6q24 (*PLAGL1*, *HYMA1*)	AD for paternal duplications	IUGR; macroglossia; umbilical hernia; may be treatable with medications other than insulin

Adapted from Carmody et al. (171). AD, autosomal dominant; AR, autosomal recessive; IUGR, intrauterine growth restriction; OGTT, oral glucose tolerance test; UPD6, uniparental disomy of chromosome 6; 2-h PG, 2-h plasma glucose.

Table 2.7
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

The Diagnosis of Monogenic Diabetes

The diagnosis of monogenic diabetes should be considered in children and adults diagnosed with diabetes in early adulthood with the following findings:

- Diabetes diagnosed within the first 6 months of life
- Diabetes without typical features of type 1 or type 2 diabetes (negative diabetes-associated autoantibodies, no obesity, and lacking other metabolic features, especially strong family history of diabetes)
- Stable, mild fasting hyperglycemia (100–150 mg/dL [5.6–8.5 mmol/L]), stable A1C between 5.6% and 7.6% (between 38 and 60 mmol/mol), especially if no obesity

Gestational Diabetes Mellitus

2.30 In individuals who are planning pregnancy, screen those with risk factors (**Table 2.5**) **B** and consider testing all individuals of childbearing potential for undiagnosed prediabetes or diabetes. **E**

2.31a Before 15 weeks of gestation, test individuals with risk factors (**Table 2.5**) **B** and consider testing all individuals **E** for undiagnosed diabetes at the first prenatal visit using standard diagnostic criteria if not screened preconception.

2.31b Before 15 weeks of gestation, screen for abnormal glucose metabolism (defined as A1C 5.9–6.4% [41–47 mmol/mol] or FPG 110–125 mg/dL [6.1–6.9 mmol/L]) to identify individuals who are at higher risk of adverse pregnancy and neonatal outcomes and are at high risk of a later gestational diabetes mellitus (GDM) diagnosis. **B**

2.32 Screen for GDM at 24–28 weeks of gestation in pregnant individuals not previously found to have diabetes or high-risk abnormal glucose metabolism detected earlier in the current pregnancy. **A**

Gestational Diabetes Mellitus (continued)

2.33 Screen individuals with GDM for prediabetes or diabetes at 4–12 weeks postpartum, using the 75-g OGTT and clinically appropriate nonpregnancy diagnostic criteria. **B**

2.34 Individuals with a history of GDM should have lifelong screening for the development of prediabetes or diabetes every 1–3 years. **B**

Gestational Diabetes Mellitus

GDM diagnosis (**Table 2.8**) can be accomplished with either of two strategies:

- The “one-step” 75-g OGTT derived from the IADPSG criteria, or
- The older “two-step” approach with a 50-g (nonfasting) screen followed by a 100-g OGTT for those who screen positive based on the work of Carpenter-Coustan’s interpretation of the older O’Sullivan and Mahan criteria.

Table 2.8—Screening for and diagnosis of GDM

One-step strategy

Perform a 75-g OGTT, with plasma glucose measurement when an individual is fasting and at 1 and 2 h, at 24–28 weeks of gestation in individuals not previously diagnosed with diabetes.

The OGTT should be performed in the morning after an overnight fast of at least 8 h.

The diagnosis of GDM is made when any of the following plasma glucose values are met or exceeded:

- Fasting: 92 mg/dL (5.1 mmol/L)
- 1 h: 180 mg/dL (10.0 mmol/L)
- 2 h: 153 mg/dL (8.5 mmol/L)

Two-step strategy

Step 1:

Perform a 50-g GLT (nonfasting), with plasma glucose measurement at 1 h, at 24–28 weeks of gestation in individuals not previously diagnosed with diabetes.

If the plasma glucose level measured 1 h after the load is ≥130, 135, or 140 mg/dL (7.2, 7.5, or 7.8 mmol/L, respectively),* proceed to a 100-g OGTT.

Step 2:

The 100-g OGTT should be performed when the individual is fasting.

The diagnosis of GDM is made when at least two>† of the following four plasma glucose levels (measured fasting and at 1, 2, and 3 h during OGTT) are met or exceeded (Carpenter-Coustan criteria [208]):

- Fasting: 95 mg/dL (5.3 mmol/L)
- 1 h: 180 mg/dL (10.0 mmol/L)
- 2 h: 155 mg/dL (8.6 mmol/L)
- 3 h: 140 mg/dL (7.8 mmol/L)

GDM, gestational diabetes mellitus; GLT, glucose load test; OGTT, oral glucose tolerance test. *American College of Obstetricians and Gynecologists (ACOG) recommends any of the commonly used thresholds of 130, 135, or 140 mg/dL for the 1-h 50-g GLT (223). †ACOG notes that one elevated value can be used for diagnosis (223).

Table 2.8
Diagnosis and Classification of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S27-S49

Section 3.

Prevention or Delay of Diabetes and Associated Comorbidities

(https://doi.org/10.2337/dc26-S003)

Overall Recommendations

3.1 In people with prediabetes, monitor for the development of diabetes at least annually; modify frequency of testing based on individual risk assessment. **E**

3.2 In people with presymptomatic type 1 diabetes, monitor for disease progression using A1C approximately every 6 months and 75-g oral glucose tolerance test (i.e., fasting and 2-h plasma glucose) annually; modify frequency of monitoring and consider augmenting with other glycemic assessment tools such as continuous glucose monitoring metrics based on individual risk assessment based on age, number and type of autoantibodies, and glycemic metrics. **E**

Lifestyle Behavior Change for Type 2 Diabetes Prevention

3.3 Refer adults with overweight or obesity at high risk of type 2 diabetes to a diabetes prevention program to achieve and maintain a weight reduction of at least 5–7% of initial body weight through a healthy reduced-calorie eating pattern and ≥150 min/week of moderate-intensity physical activity. **A**

3.4 Prescribe an evidence-based eating pattern (e.g., Mediterranean, low carbohydrate) to individuals with prediabetes to prevent type 2 diabetes. **B**

3.5 Offer diabetes prevention programs to adults at high risk for type 2 diabetes. **A** Diabetes prevention programs should be covered by third-party payors, and inconsistencies in access should be addressed. **E**

3.6 Based on individual preference, certified technology-assisted diabetes prevention programs through smartphones, web-based applications, and telehealth can be effective in preventing type 2 diabetes and should be considered. **B**

Pharmacologic Interventions to Delay Type 2 Diabetes

3.7 Metformin for the prevention of type 2 diabetes should be considered in adults at high risk of type 2 diabetes, as typified by the Diabetes Prevention Program, especially those aged 25–59 years with BMI ≥35 kg/m^2, higher fasting plasma glucose (e.g., ≥110 mg/dL [≥6 mmol/L]), and higher A1C (e.g., ≥6.0% [≥42 mmol/mol]), and in individuals with prior gestational diabetes mellitus. **A**

3.8 Consider using metformin to prevent hyperglycemia in high-risk individuals treated with a phosphatidylinositol 3-kinase α (PI3Kα) inhibitor (e.g., alpelisib and inavolisib). **B**

3.9 Consider using metformin to prevent hyperglycemia in high-risk individuals treated with high-dose glucocorticoids. **B**

3.10 Consider periodic assessment of vitamin B12 levels in individuals receiving long-term metformin therapy, especially in those with anemia or peripheral neuropathy. **B**

Prevention of Vascular Disease and Mortality

3.11 Prediabetes is associated with heightened cardiovascular risk; therefore, screening for and treatment of modifiable risk factors for cardiovascular disease are suggested. **B**

3.12 Statin therapy may increase the risk of type 2 diabetes in people at high risk of developing type 2 diabetes. In such individuals, glucose status should be monitored regularly and diabetes prevention approaches reinforced. It is not recommended that statins be avoided or discontinued for this adverse effect. **B**

3.13 In people with a history of stroke and evidence of insulin resistance and prediabetes, pioglitazone may be considered to lower the risk of stroke or myocardial infarction. However, this benefit needs to be balanced with the increased risk of weight gain, edema, and fractures. **A** Lower doses may mitigate the risk of adverse effects but may be less effective. **C**

Person-Centered Care Goals

3.14 In adults with overweight or obesity at high risk of type 2 diabetes, care goals should include weight loss and maintenance, minimizing the progression of hyperglycemia, and attention to cardiovascular risk. **B**

3.15 Pharmacotherapy (e.g., for weight management, minimizing the progression of hyperglycemia, and cardiovascular risk reduction) should be considered to support person-centered care goals. **A**

3.16 More intensive preventive approaches should be considered in individuals who are at particularly high risk of progression to diabetes, including individuals with BMI ≥35 kg/m^2, those with higher glucose levels (e.g., fasting plasma glucose 110–125 mg/dL [6.1–6.9 mmol/L], 2-h post-challenge glucose 173–199 mg/dL [9.6–11.0 mmol/L], and A1C ≥6.0% [≥42 mmol/mol]), and individuals with a history of gestational diabetes mellitus. **A**

Pharmacologic Interventions to Delay Symptomatic Type 1 Diabetes

3.17 Teplizumab-mzwv infusion to delay the onset of symptomatic type 1 diabetes (stage 3) should be discussed with selected individuals aged ≥8 years with stage 2 type 1 diabetes. Treatment should be in a setting with appropriately trained personnel. **B**

Section 4.

Comprehensive Medical Evaluation and Assessment of Comorbidities

(https://doi.org/10.2337/dc26-S004)

Person-Centered Collaborative Care

4.1 A communication style that uses person-centered, culturally sensitive, and strength-based language and active listening; elicits individual preferences and beliefs; and assesses literacy, numeracy, and potential barriers to care should be used to optimize health outcomes and health-related quality of life. **B**

4.2 People with diabetes can benefit from a coordinated interprofessional team that may include but is not limited to diabetes care and education specialists, primary care and subspecialty clinicians, nurses, registered dietitian nutritionists, exercise specialists, pharmacists, dentists, podiatrists, and behavioral health professionals. **C**

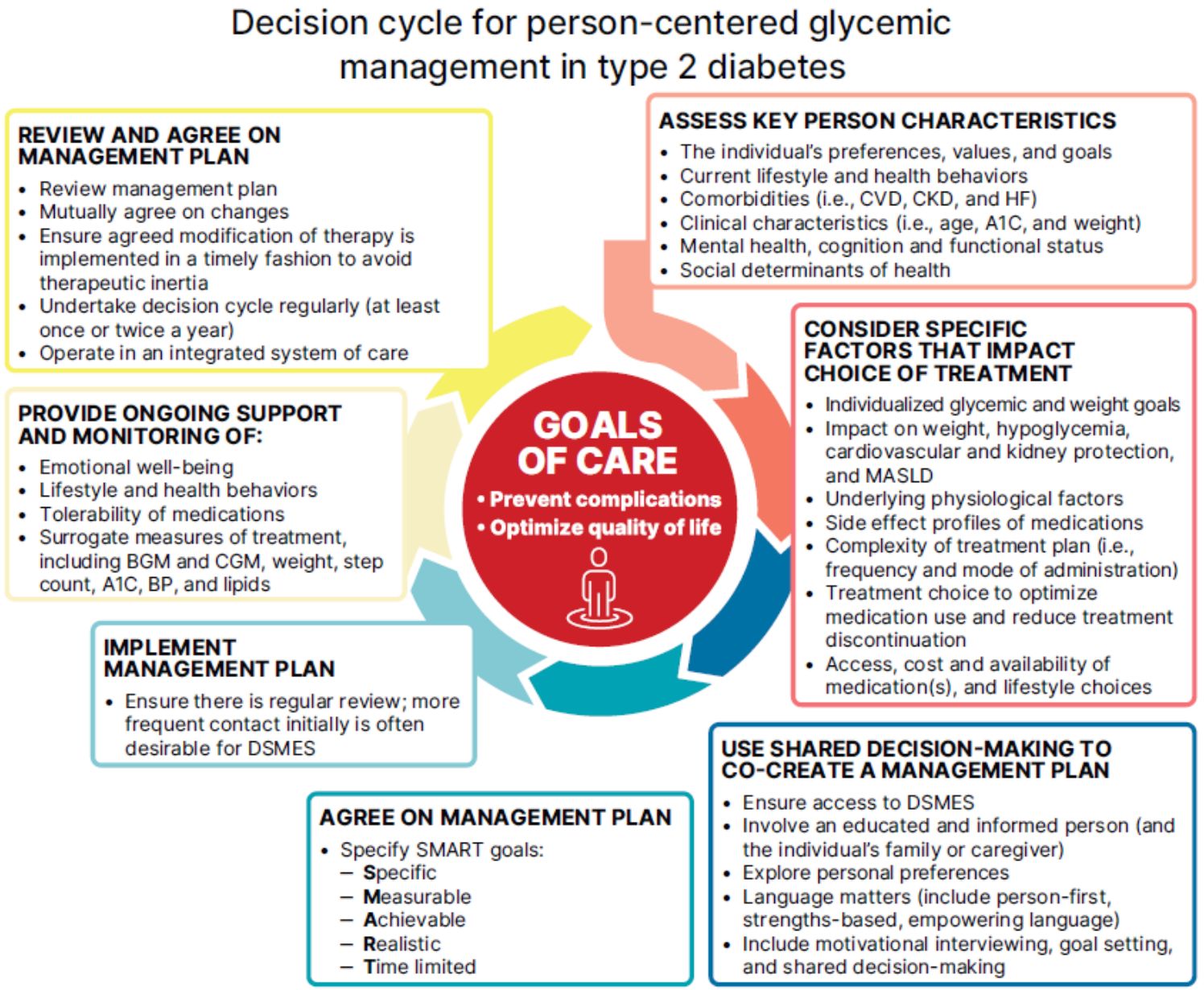


Figure 4.1
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Figure 4.1—Decision cycle for person-centered glycemic management in type 2 diabetes. BGM, blood glucose monitoring; BP, blood pressure; CGM, continuous glucose monitoring; CKD, chronic kidney disease; CVD, cardiovascular disease; DSMES, diabetes self-management education and support; HF, heart failure. MASLD, metabolic dysfunction-associated steatotic liver disease. Adapted from Davies et al. (307).

Figure 4.1 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Comprehensive Medical Evaluation

4.3 A complete medical evaluation should be performed at the initial visit and follow-up, as appropriate, to:

- Confirm the diagnosis and classify diabetes. **A**
- Assess glycemic status and previous and current treatment. **A**
- Evaluate for diabetes complications, potential comorbid conditions, and overall health status. **A**
- Identify care partners, support systems, and available resources. **E**
- Assess social determinants of health and structural barriers to optimal health and health care. **A**
- Review risk factor management in the person with diabetes. **A**
- Begin engagement with the person with diabetes in the formulation of a care management plan including goals of care. **A**
- Develop a plan for continuing care. **A**

Comprehensive Medical Evaluation (continued)

4.4 Ongoing management should be guided by the assessment of overall health and functional status, diabetes complications, cardiovascular risk, hypoglycemia risk, and shared decision making to set therapeutic goals. **B**

Table 4.1—Components of the comprehensive diabetes medical evaluation at initial, follow-up, and annual visits

	Visit		
	Initial	Every follow-up	Annual
Past medical and family history			
Diabetes history			
• Characteristics at onset (e.g., age and symptoms and/or signs)	✓		
• Review of previous treatment plans and response	✓		
• Assess frequency, cause, and severity of past hospitalizations	✓		
Family history			
• Family history of diabetes in a first-degree relative	✓		
• Family history of autoimmune disorders	✓		
Personal history of complications and common comorbidities			
• Common comorbidities (e.g., obesity, OSA, and MASLD)	✓		✓
• High blood pressure or abnormal lipids	✓		✓
• Macrovascular and microvascular complications	✓		✓
• Hypoglycemia: awareness, frequency, causes, and timing of episodes	✓	✓	✓
• Presence of hemoglobinopathies or anemias	✓		✓
• Last dental visit	✓		✓
• Last foot exam	✓		✓
• Last dilated eye exam	✓		✓
• Visits to specialists	✓		
• Disability assessment and use of assistive devices (e.g., physical, cognitive, vision and auditory, history of fractures, and podiatry)	✓	✓	✓
• Personal history of autoimmune disease	✓		
Surgical and procedure history			
• Surgeries (e.g., metabolic surgery and transplantation)	✓	✓	✓
Interval history			
• Changes in medical or family history since last visit		✓	✓
Behavioral factors			
• Physical activity, sleep behaviors, eating patterns and weight history	✓	✓	✓
• Assess familiarity with carbohydrate counting (e.g., type 1 diabetes or type 2 diabetes treated with MDI)	✓		✓
• Screen for OSA	✓	✓	✓
• Tobacco, alcohol, and substance use	✓		✓
Medications and vaccinations			
• Current medication plan	✓	✓	✓
• Medication-taking behavior, including rationing of medications and/or medical equipment	✓	✓	✓
• Medication intolerance or side effects	✓	✓	✓
• Complementary and alternative medicine use	✓	✓	✓
• Vaccination history and needs	✓		✓
Technology use			
• Assess use of health apps, online education, patient portals, etc.	✓	✓	✓
• Glucose monitoring (meter/CGM): results and data use	✓	✓	✓
• Review insulin pump settings and use and connected pen and glucose data	✓	✓	✓
Social life assessment			
Social network			
• Identify existing social supports	✓		✓
• Identify surrogate decision maker and advanced care plan	✓		✓
• Identify social determinants of health (e.g., food security, housing stability, transportation access, financial security, and community safety)	✓		✓

Table 4.1
Comprehensive Medical Evaluation and Assessment of Comorbidities: *Standards of Care in Diabetes - 2026. Diabetes Care* 2026;49(Suppl. 1):S61-S88

Table 4.1—Continued

	Visit		
	Initial	Every follow-up	Annual
• Assess daily routine and environment, including school or work schedules and ability to engage in diabetes self-management	✓	✓	✓
Physical examination			
• Height, weight, and BMI; growth and pubertal development in children and adolescents	✓	✓	✓
• Blood pressure determination	✓	✓	✓
• Orthostatic blood pressure measures (when indicated)	✓		✓
• Fundoscopic examination (refer to eye specialist)	✓		✓
• Thyroid palpation	✓		✓
• Skin examination (e.g., acanthosis nigricans, insulin injection or insertion sites, and lipodystrophy)	✓	✓	✓
• Comprehensive foot examination, determination of temperature, vibration or pinprick sensation, and 10-g monofilament exam	✓		✓
• Visual inspection (e.g., skin integrity, callous formation, foot deformity or ulcer, and toenails)*	✓	✓	✓
• Check pedal pulses and screen for PAD with ABI testing if a PAD diagnosis would change management	✓		✓
• Screen for depression, anxiety, diabetes distress, fear of hypoglycemia, and disordered eating	✓		✓
• Assessment for cognitive performance if indicated	✓		✓
• Assessment for functional performance if indicated	✓		✓
• Assessment for bone health (e.g., loss of height and kyphosis)	✓		✓
Laboratory evaluation			
• A1C, if the results are not available within the past 3 months or if earlier assessment is necessary	✓	✓	✓
• Lipid profile, including total, LDL, and HDL cholesterol and triglycerides‡	✓		
• Liver function tests (i.e., FIB-4)‡	✓		✓
• Spot urinary albumin-to-creatinine ratio	✓		✓
• Serum creatinine and estimated glomerular filtration rate§	✓		✓
• Thyroid-stimulating hormone in people with type 1 diabetes‡	✓		✓
• Celiac disease screening in people with type 1 diabetes‖	✓		
• Vitamin B12 if taking metformin for >5 years	✓		✓
• CBC with platelets	✓		✓
• Serum potassium levels in people treated with ACE inhibitors, ARBs, or diuretics§	✓		✓
• Calcium, vitamin D, and phosphorous as appropriate	✓		✓

ABI, ankle brachial index; ARBs, angiotensin receptor blockers; CBC, complete blood count; CGM, continuous glucose monitor; FIB-4, fibrosis-4 index; MASLD, metabolic-associated steatotic liver disease; MDI, multiple daily injections; OSA, obstructive sleep apnea; PAD, peripheral arterial disease. *Should be performed at every visit in people with diabetes with sensory loss, previous foot ulcers, or amputations. †At 65 years of age or older. ‡May also need to be checked after initiation or dose changes of medications that affect these laboratory values (i.e., diabetes medications, blood pressure medications, cholesterol medications, or thyroid medications). §May be needed more frequently in people with diabetes with known chronic kidney disease or with changes in medications that affect kidney function and serum potassium (see **Table 11.2**). ‖In people with presence of gastrointestinal symptoms, signs, laboratory manifestations, or clinical suspicion suggestive of celiac disease.

Table 4.1 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Table 4.2—Essential components for assessment, planning, and referral

Assessing risk of diabetes complications

- ASCVD and heart failure history
- ASCVD risk factors and 10-year ASCVD risk assessment
- Staging of chronic kidney disease (see **Table 11.2**)
- Hypoglycemia risk (see section 6, "Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises")
- Assessment for retinopathy
- Assessment for neuropathy
- Assessment for MASLD and MASH

Goal setting

- Set A1C, blood glucose, and time-in-range goals
- Set lipid goal
- If hypertension is present, establish blood pressure goal
- Weight management and physical activity goals
- Diabetes self-management goals

Therapeutic treatment plans

- Lifestyle management (e.g., registered dietitian nutritionist)
- Pharmacologic therapy: glucose lowering
- Pharmacologic therapy: cardiovascular and kidney disease risk factors
- Weight management with pharmacotherapy or metabolic surgery, as appropriate
- Use of glucose monitoring and insulin delivery devices
- Referral to diabetes education and medical specialists (as needed)

Table 4.2
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Referrals for initial care management

- Eye care professional for annual dilated eye exam
- Family planning for individuals of childbearing potential
- Registered dietitian nutritionist for medical nutrition therapy
- Diabetes self-management education and support
- Dentist for comprehensive dental and periodontal examination
- Behavioral health professional, if indicated
- Audiology, if indicated
- Social worker and community resources, if indicated
- Rehabilitation medicine or another relevant health care professional for physical and cognitive disability evaluation, if indicated
- Other appropriate health care professionals

Assessment and treatment planning are essential components of initial and all follow-up visits. ASCVD, atherosclerotic cardiovascular disease; MASH, metabolic dysfunction-associated steatohepatitis; MASLD, metabolic dysfunction-associated steatotic liver disease.

Table 4.2 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. *Diabetes Care* 2026;49(Suppl. 1):S61-S88

Immunizations

4.5 Provide routinely recommended vaccinations for children and adults with diabetes as indicated by age (see **Table 4.3**). **A**

Table 4.3—Highly recommended immunizations for people with diabetes (from the Advisory Committee on Immunization Practices and Centers for Disease Control and Prevention)

Vaccine	Recommended ages	Schedule	GRADE evidence type*	References
COVID-19	All people 6 months of age and older	Current initial vaccination and boosters	3	Centers for Disease Control and Prevention, Interim Clinical Considerations for Use of COVID-19 Vaccines in the United States (305)
Hepatitis B	Adults with diabetes aged <60 years; for adults aged ≥60 years, hepatitis B vaccine may be administered at the discretion of the treating clinician based on the person's likelihood of acquiring hepatitis B infection		1	Sandul et al., Updated Recommendation for Universal Hepatitis B Vaccination in Adults Aged 19–59 Years - United States, 2024 (306)
Influenza	All people with diabetes advised to receive a trivalent influenza vaccine and not to receive live attenuated influenza vaccine	Annual	3	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)

Table 4.3
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Pneumonia (PPSV23 [Pneumovax])	19–64 years of age, vaccinate with Pneumovax	One dose is recommended for those who previously received PCV13; if PCV15 was used, follow with PPSV23 ≥1 year later; PPSV23 is not indicated after PCV20; adults who received only PPSV23 may receive PCV15 or PCV20 ≥1 year after their last dose	2	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)
	≥65 years of age	One dose is recommended for those who previously received PCV13; if PCV15 was used, follow with PPSV23 ≥1 year later; PPSV23 is not indicated after PCV20; adults who received only PPSV23 may receive PCV15 or PCV20 ≥1 year after their last dose	2	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)
PCV20 or PCV15	Adults 19–64 years of age with an immunocompromising condition (e.g., chronic renal failure), cochlear implant, or cerebrospinal fluid leak Adults 19–64 years of age, immunocompetent ≥65 years of age, immunocompetent, have shared decision-making	One dose of PCV15 or PCV20 is recommended by the Centers for Disease Control and Prevention For those who have never received any pneumococcal vaccine, the Centers for Disease Control and Prevention	2	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19

Continued on p. S68

Table 4.3 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Table 4.3—Continued

Vaccine	Recommended ages	Schedule	GRADE evidence type*	References
	discussion with health care professionals	recommends one dose of PCV15 or PCV20 One dose of PCV15 or PCV20; PCSV23 may be given ≥8 weeks after PCV15; PPSV23 is not indicated after PCV20		Years or Older - United States, 2025 (6,7)
RSV	Older adults ≥60 years of age with diabetes appear to be a risk group	Adults aged ≥75 years and those aged ≥60 years and at high risk may receive a single dose of an RSV vaccine	1	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)
Tetanus, diphtheria, pertussis (Tdap)	All adults; pregnant individuals should have an extra dose	Booster every 10 years	2	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)

Table 4.3 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Zoster	≥50 years of age	Two-dose Shingrix, even if previously vaccinated	1	Advisory Committee on Immunization Practices Recommended Immunization Schedule for Children and Adolescents Aged 18 Years or Younger - United States, 2025, and Advisory Committee on Immunization Practices Recommended Immunization Schedule for Adults Aged 19 Years or Older - United States, 2025 (6,7)

For a comprehensive list of vaccines, refer to the Centers for Disease Control and Prevention web site at cdc.gov/vaccines/. Advisory Committee on Immunization Practices recommendations can be found at cdc.gov/acip/?CDC_AAref_Val=https://cdc.gov/vaccines/acip/recommendations. COVID-19, coronavirus disease 2019; GRADE, Grading of Recommendations Assessment, Development, and Evaluation; PCV13, 13-valent pneumococcal conjugate vaccine; PCV15, 15-valent pneumococcal conjugate vaccine; PCV 20, 20-valent pneumococcal conjugate vaccine; PPSV23, 23-valent pneumococcal polysaccharide vaccine; RSV, respiratory syncytial virus. *Evidence type: 1, randomized controlled trials (RCTs) or overwhelming evidence from observational studies; 2, RCTs with important limitations or exceptionally strong evidence from observational studies; 3, observational studies or RCTs with notable limitations; 4, clinical experience and observations, observational studies with important limitations, or RCTs with several major limitations.

Table 4.3 (continued)
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Autoimmune Diseases

4.6 Screen people with type 1 diabetes for autoimmune thyroid disease soon after diagnosis and thereafter at repeated intervals if clinically indicated. **B**

4.7 Adults with type 1 diabetes should be screened for celiac disease in the presence of gastrointestinal symptoms, signs, laboratory manifestations, or clinical suspicion suggestive of celiac disease. **B**

Bone Health

4.8 Assess fracture risk in older adults with diabetes as a part of routine care in diabetes clinical practice, according to risk factors and comorbidities. **A**

4.9 Monitor bone mineral density using dual-energy X-ray absorptiometry in older adults with diabetes (aged ≥65 years) and younger individuals with diabetes and multiple risk factors every 2–3 years (**Table 4.4**). **A**

4.10 Consider the potential adverse impact on skeletal health when selecting pharmacological options to lower glucose levels in people with diabetes. Avoiding medications with a known association with higher fracture risk (e.g., thiazolidinediones and sulfonylureas) is recommended, particularly for those at elevated risk for fractures. **B**

Bone Health (continued)

4.11 To reduce the risk of falls and fractures, glycemic management goals should be individualized for people with diabetes at a higher risk of fracture. **C** Prioritize use of glucose-lowering medications that are associated with low risk for hypoglycemia to avoid falls. **B**

4.12 Advise people with diabetes on their intake of calcium (1,000–1,200 mg/day) and vitamin D to ensure it meets the recommended daily allowance for those at risk for fracture, either through their food choices or supplemental means. **B**

4.13a Consider osteoporosis drug therapy in older adults with diabetes who are at increased risk of fracture, including those with low bone mineral density (T-score ≤−2.5), history of fragility fracture, or elevated Fracture Risk Assessment Tool score (≥3% for hip fracture or ≥20% for major osteoporotic fracture). **B**

4.13b Treatment may be considered for adults with diabetes with a T-score between −2.0 and −2.5 in the presence of additional risk factors for fracture. **C**

Table 4.4—Diagnostic assessment

Individuals who should receive BMD testing
People aged ≥65 years
Postmenopausal women and men aged ≥50 years with history of adult-age fracture or with diabetes-specific risk factors: • Frequent hypoglycemic events • Diabetes duration >10 years • Diabetes medications: insulin, thiazolidinediones, sulfonylureas • A1C >8% • Peripheral or autonomic neuropathy, retinopathy, nephropathy • Frequent falls • Glucocorticoid use: prednisone at doses >2.5 mg per day for ≥3 months

Table 4.4
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Cognitive Impairment/Dementia

4.14 In the presence of cognitive impairment, diabetes treatment plans should be simplified as much as possible and tailored to minimize the risk of hypoglycemia. **B**

Dental Care

4.15 People with diabetes should be referred for a dental exam at least once per year. **E**

4.16 Coordinate efforts between the medical and dental teams to appropriately adjust glucose-lowering medication and treatment plans prior to and in the post-dental procedure period as needed. **B**

Disability

4.17 Assess for disability at the initial visit and for decline in function at each subsequent visit in people with diabetes. If a disability is impacting functional ability or capacity to manage their diabetes, a referral should be made to an appropriate health care professional specializing in disability (e.g., physical medicine and rehabilitation specialist, physical therapist, occupational therapist, or speech-language pathologist). **C**

Sexual Health in Men

4.18 In men with diabetes or prediabetes, screen for erectile dysfunction (ED), particularly in those with high cardiovascular risk, retinopathy, cardiovascular disease, chronic kidney disease, peripheral or autonomic neuropathy, longer duration of diabetes, depression, and hypogonadism and in those who are not meeting glycemic goals. **B**

4.19 In men with diabetes or prediabetes, inquire about sexual health (e.g., low libido and ED). If symptoms and/or signs of hypogonadism are detected, screen with a morning serum total testosterone level. **B**

Sexual Health in Women

4.20 In women with diabetes or prediabetes, inquire about sexual health by screening for desire (libido), arousal, and orgasm difficulties, particularly in those who experience depression and/or anxiety and those with recurrent urinary tract infections. **B**

4.21 In postmenopausal women with diabetes or prediabetes, screen for symptoms and/or signs of genitourinary syndrome of menopause, including vaginal dryness and dyspareunia. **B**

Metabolic-Associated Steatotic Liver Disease and Metabolic-Associated Steatohepatitis - Screening

4.22a Screen adults with type 2 diabetes or with prediabetes, particularly those with obesity or other cardiometabolic risk factors or established cardiovascular disease, for their risk of having or developing cirrhosis related to metabolic dysfunction-associated steatohepatitis (MASH) using a calculated fibrosis-4 index (FIB-4) (derived from age, ALT, AST, and platelets [mdcalc.com/calc/2200/fibrosis4-fib-4-index-liver-fibrosis]), even if they have normal liver enzymes. **B**

4.22b Adults with diabetes or prediabetes with persistently elevated plasma aminotransferase levels for >6 months and low FIB-4 should be evaluated for other causes of liver disease. **B**

4.23 Adults with type 2 diabetes or prediabetes with a FIB-4 ≥1.3 should have additional risk stratification by liver stiffness measurement with transient elastography, or, if unavailable, the enhanced liver fibrosis (ELF) test. **B**

4.24 Refer adults with type 2 diabetes or prediabetes at higher risk for significant liver fibrosis (i.e., as indicated by FIB-4, liver stiffness measurement, or ELF) to a gastroenterologist or hepatologist for further evaluation and management. **B**

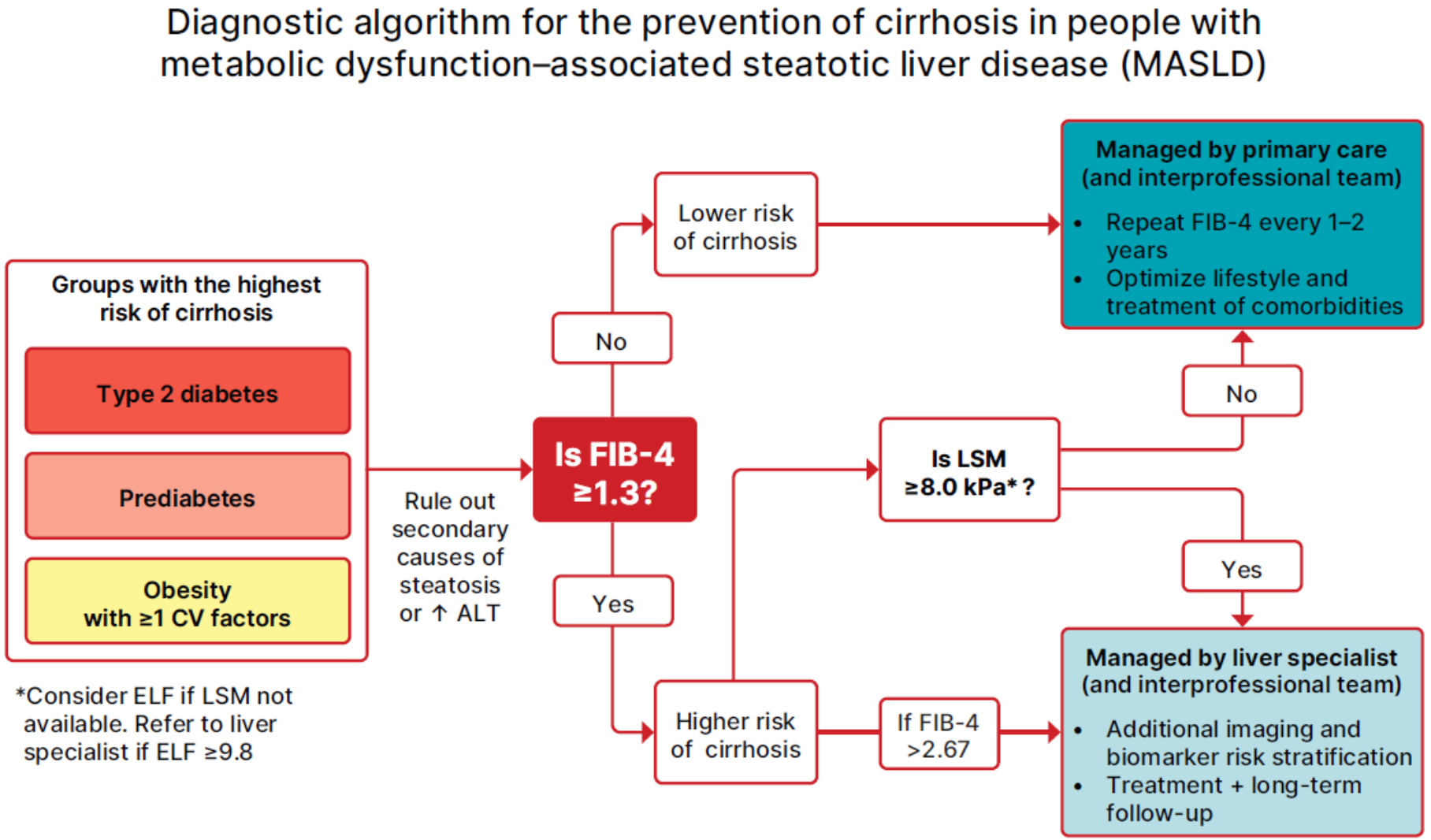


Figure 4.2—Diagnostic algorithm for risk stratification and the prevention of cirrhosis in individuals with metabolic dysfunction-associated steatotic liver disease (MASLD). CV, cardiovascular; ELF, enhanced liver fibrosis test; FIB-4, fibrosis-4 index; LSM, liver stiffness measurement, as measured by vibration-controlled transient elastography. *In the absence of LSM, consider ELF a diagnostic alternative. If ELF ≥9.8, an individual is at high risk of metabolic dysfunction-associated steatohepatitis with advanced liver fibrosis (≥F3–F4) and should be referred to a liver specialist.

Figure 4.2
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Metabolic-Associated Steatotic Liver Disease and Metabolic-Associated Steatohepatitis - Management

4.25 Adults with type 2 diabetes or prediabetes, particularly with overweight or obesity, who have metabolic dysfunction–associated steatotic liver disease (MASLD) should be recommended lifestyle changes using an interprofessional approach that promotes weight loss, ideally within a structured nutrition plan and physical activity program for cardiometabolic benefits **B** and histological improvement. **C**

4.26 In adults with type 2 diabetes, MASLD, and overweight or obesity, consider using a glucagon-like peptide 1 receptor agonist (GLP-1 RA) with demonstrated benefits in MASH **A** or a dual glucose-dependent insulinotropic polypeptide (GIP) and GLP-1 RA with potential benefits in MASH **B** for the treatment of obesity as an adjunctive therapy to lifestyle interventions for weight loss.

4.27a In adults with type 2 diabetes and biopsy-proven MASH or those at high risk for liver fibrosis (based on noninvasive tests), a GLP-1 RA is preferred for glycemic management due to beneficial effects on MASH. **A** Pioglitazone **B** or a dual GIP and GLP-1 RA **B** can be considered for glycemic management due to potential beneficial effects on MASH.

4.27b Combination therapy with pioglitazone plus GLP-1 RA can be considered for the treatment of hyperglycemia in adults with type 2 diabetes with biopsy-proven MASH or those at high risk of liver fibrosis (identified with noninvasive tests) because of potential beneficial effects on MASH. **B**

Metabolic-Associated Steatotic Liver Disease and Metabolic-Associated Steatohepatitis – Management (continued)

4.28 For consideration of treatment with a thyroid hormone receptor-β agonist in adults with type 2 diabetes or prediabetes with MASLD with moderate (F2) or advanced (F3) liver fibrosis on liver histology, or by a validated imaging-based or blood-based test, refer to a gastroenterologist or hepatologist with expertise in MASLD management. **A**

4.29 Treatment initiation and monitoring should be individualized and within the context of an interprofessional team that includes a gastroenterologist or hepatologist, consideration of individual preferences, and a careful shared-decision cost-benefit discussion. **B**

4.30a In adults with type 2 diabetes and MASLD, use of glucose-lowering therapies other than pioglitazone or GLP-1 RAs may be continued as clinically indicated, but these therapies lack evidence of benefit in MASH. **B**

4.30b Insulin therapy is the preferred agent for the treatment of hyperglycemia in adults with type 2 diabetes with decompensated cirrhosis. **C**

Metabolic-Associated Steatotic Liver Disease and Metabolic-Associated Steatohepatitis – Management (continued)

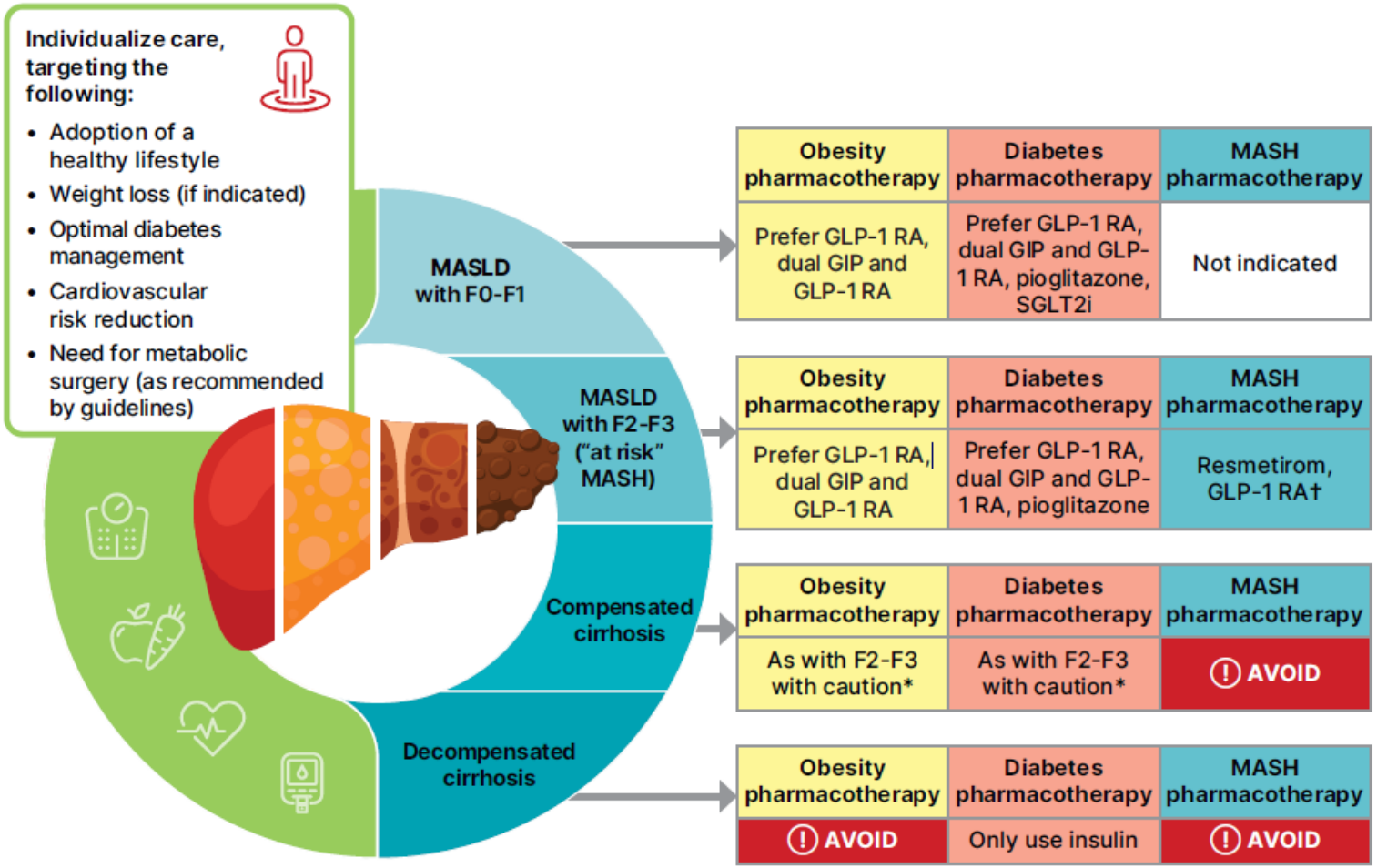
4.31a Adults with type 2 diabetes and MASLD are at increased cardiovascular risk; therefore, comprehensive management of cardiovascular risk factors is recommended. **B**

4.31b Statin therapy is safe in adults with type 2 diabetes and compensated cirrhosis from MASLD and should be initiated or continued for cardiovascular risk reduction as clinically indicated. **B** In people with decompensated cirrhosis, statin therapy should be used with caution, and close monitoring is needed, given limited safety and efficacy data. **B**

4.32a Consider metabolic surgery in appropriate candidates as an option to treat MASH in adults with type 2 diabetes **B** and to improve cardiovascular outcomes. **B**

4.32b Metabolic surgery should be used with caution in adults with type 2 diabetes with compensated cirrhosis from MASLD B and is not recommended in decompensated cirrhosis. **B**

Metabolic dysfunction–associated steatotic liver disease (MASLD) treatment algorithm



† Only semaglutide among GLP-1 RAs has been approved by the FDA for treatment of MASH.
* Individualized care and close monitoring needed in compensated cirrhosis given limited safety data available.

Figure 4.3—Metabolic dysfunction-associated steatotic liver disease (MASLD) treatment algorithm. F0-F1, no to minimal fibrosis; F2-F3, moderate fibrosis; F4, cirrhosis; GIP, glucose-dependent insulinotropic polypeptide; GLP-1 RA, glucagon-like peptide 1 receptor agonist; MASH, metabolic dysfunction-associated steatohepatitis; SGLT2i, sodium–glucose cotransporter 2 inhibitor.

Figure 4.3
Comprehensive Medical Evaluation and Assessment of Comorbidities:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S61-S88

Section 5.

Facilitating Positive Behaviors and Well-being to Improve Health Outcomes

(https://doi.org/10.2337/dc26-S005)

Diabetes Self-Management Education and Support

5.1 Advise all people with diabetes to participate in developmentally and culturally appropriate diabetes self-management education and support (DSMES) to facilitate informed decision-making, self-care behaviors, problem-solving, and active collaboration with the health care team. **A**

5.2 Provide DSMES at diagnosis, annually and/or when not meeting treatment goals, when complicating factors develop (e.g., medical, functional, and psychosocial), and when transitions in life and care occur. **E**

5.3 Assess clinical outcomes, health status, and well-being as key goals of DSMES on an individualized timeframe. **C**

5.4 Use behavioral strategies (e.g., motivational interviewing, goal setting, problem-solving) to support DSMES and engagement in behaviors known to optimize health-related quality of life and outcomes. **A**

Diabetes Self-Management Education and Support (continued)

5.5 Provide culturally and socially appropriate DSMES responsive to personal preferences and needs in group or individual settings. **A** Communicate DSMES participation with the diabetes care team. **E**

5.6 Offer DSMES via telehealth and/or digital interventions to meet individual preferences, reduce access barriers, and improve satisfaction. **B**

5.7 DSMES can improve outcomes and reduce costs, so reimbursement by third-party payors is recommended. **B**

5.8 Identify and address barriers to DSMES that exist at the payor, health system, clinic, health care professional, and individual levels. **E**

5.9 Assess the social determinants of health to guide and design delivery of DSMES to maximize health equity across populations. **C**

Diabetes Self-Management Education and Support (continued)

In addition to providing DSMES upon diagnosis, there are additional critical time points when the need for DSMES should be evaluated by the health care professional and/or interprofessional team, with referrals made as needed:

- Annually and/or when not meeting treatment goals, whichever is more frequent
- When complicating factors (e.g., health conditions, physical or functional limitations, emotional factors, and basic living needs) that influence self-management develop
- When transitions in life and care occur

Medical Nutrition Therapy

5.10 Provide individualized medical nutrition therapy by referring people with prediabetes or diabetes to a registered dietitian nutritionist, preferably one who has comprehensive experience in diabetes care. **A**

5.11 Diabetes medical nutrition therapy can result in cost savings **B** and improved cardiometabolic outcomes **A** and should be reimbursed by insurance. **E**

5.12 Provide an overweight/obesity treatment plan based on their nutrition, physical activity, and behavioral health status for all people with overweight or obesity, aiming for at least 5–7% weight loss. **A**

5.13 For diabetes prevention and management of people with prediabetes or diabetes, recommend individualized meal plans that keep nutrient quality, total calories, and metabolic goals in mind. **B**

Medical Nutrition Therapy (continued)

5.14 Eating patterns should emphasize key nutrition principles (inclusion of nonstarchy vegetables, whole fruits, legumes, lean proteins, whole grains, nuts and seeds, and low-fat dairy or nondairy alternatives) and minimize consumption of red meat, sugar-sweetened beverages, sweets, refined grains, processed and ultraprocessed foods in people with prediabetes and diabetes. **B**

5.15 Consider reducing carbohydrate intake for some adults with diabetes to improve glycemia. An effective way to achieve this is by limiting consumption of processed foods. **B**

5.16 Assess intake of supplements, as supplementation with micronutrients (e.g., vitamins and minerals, such as magnesium or chromium) or herbs or spices (e.g., cinnamon and aloe vera) is not recommended for glycemic benefits. **C**

5.17 Counsel against β-carotene supplementation, as there is evidence of harm for certain individuals and it confers no benefit. **B**

Medical Nutrition Therapy (continued)

5.18 Advise adults with diabetes and those at risk for diabetes who consume alcohol to not exceed the recommended daily limits. **B** Advise abstainers to not start drinking alcohol, even in moderation. **B**

5.19. Counsel people with diabetes about the signs, symptoms, and self-management of delayed hypoglycemia and the importance of monitoring glucose after drinking alcohol to reduce hypoglycemia risk, especially when using insulin or insulin secretagogues. **B**

5.20 Counsel people with diabetes to limit sodium consumption to <2,300 mg/day, as clinically appropriate, and the best way to achieve this is through limiting consumption of processed foods. **B**

5.21 Encourage people with diabetes and those at risk for diabetes to consume water over other beverages. **A**

Medical Nutrition Therapy (continued)

5.22 Counsel people with diabetes and those at risk for diabetes that nonnutritive sweeteners can be used in place of sugar-sweetened products if consumed in moderation and for the short term to reduce overall calorie and carbohydrate intake. **B**

5.23. Counsel and regularly monitor individuals pursuing intentional weight loss to ensure adequate nutritional intake, with particular attention to preventing protein insufficiency and micronutrient deficiencies. **E**

5.24 Emphasize minimally processed, nutrient-dense, high-fiber sources of carbohydrate (at least 14 g fiber per 1,000 kcal). **B**

5.25 Advise people with diabetes and those at risk for diabetes to replace sugar-sweetened beverages (including any juices) with water or low-calorie or no-calorie beverages and minimize foods with added sugar to manage glycemia and reduce risk for cardiometabolic disease. **B**

Medical Nutrition Therapy (continued)

5.26 Educate individuals with diabetes who are at risk for developing diabetic ketoacidosis and who are treated with sodium–glucose cotransporter inhibition on the risks and signs of ketoacidosis and methods of risk mitigation management, provide them with appropriate tools for ketone measurement (i.e., serum β-hydroxybutyrate), and discourage a ketogenic eating pattern. **E**

5.27 Provide education on the glycemic impact of carbohydrate, **A** fat, and protein **B** tailored to an individual's needs, insulin plan, and preferences for care to optimize mealtime insulin dosing.

5.28 Counsel people using fixed insulin doses about consistent patterns of carbohydrate intake with respect to time and amount while considering the insulin action time, as it can result in improved glycemia and reduce the risk for hypoglycemia. **B**

Medical Nutrition Therapy (continued)

5.29 Counsel people with diabetes and those at risk for diabetes to incorporate more plant-based protein sources (e.g., nuts, seeds, and legumes) as part of an overall diverse eating pattern to reduce cardiovascular disease risk. **B**

5.30 Counsel people with diabetes and those at risk for diabetes to consider an eating plan emphasizing elements of a Mediterranean eating pattern, which is rich in fatty fish, nuts, and seeds, to reduce cardiovascular disease risk **A** and improve glucose metabolism. **B**

5.31 Counsel people with diabetes and those at risk for diabetes to limit intake of foods high in saturated fat to help reduce cardiovascular disease risk. **B**

Table 5.2—Nutrition behaviors to encourage

- Vegetables—especially nonstarchy vegetables that are dark green, red, and orange in color; fresh, frozen, or low-sodium canned are all acceptable vegetable options.
- Legumes—dried beans, peas, and lentils.
- Fruits—especially whole fruit—fresh, frozen, or canned in own juice (or no added sugar) are all acceptable fruit options.
- Foods with at least 3 g of fiber per serving are generally considered higher fiber choices. Whole-grain foods—where culturally appropriate, whole-grain versions of commonly consumed foods, such as 100% whole-wheat breads or pastas and brown rice. When not culturally appropriate, focus more on portion control.
- Water should be the primary beverage of choice.
- For individuals who do not prefer plain water, no-calorie alternatives are the next best choice. Options include adding lemon, lime, berries, or cucumber slices to water; sparkling no-calorie water or flavored no-calorie waters; no-calorie carbonated beverages.
- Plant-based proteins can include legumes (e.g., soybeans, pinto beans, black beans, garbanzo beans, dried peas, and lentils), nuts, and seeds.
- Meats and poultry should be from fresh, frozen, or low-sodium canned and in lean forms (e.g., chicken breast and ground turkey).
- Heart-healthy wild-caught fatty fish such as salmon, tuna, sardines, and mackerel. Fresh, frozen, or low-sodium canned are all acceptable options.
- Use herbs (e.g., basil, fennel, mint, parsley, rosemary, and thyme) and spices (e.g., cinnamon, garam masala, ginger, pepper, and turmeric) to season foods instead of salt or salt-containing preparations.
- Incorporate onions, garlic, celery, carrots, and other vegetables as a base for preparing various homemade foods.
- Cook with vegetable oil (e.g., avocado, canola, and olive) in place of fats high in saturated fat (e.g., butter, coconut oil, lard, and shortening).
- Plan out meals for the week. Grocery shop using a list. Cook on a day off so there are ready-to-eat and ready-to-reheat homemade meals waiting in the fridge or freezer.
- Include family or roommates in meal preparation; share the responsibilities of grocery shopping and cooking and use time off for meal preparation in advance when possible.

Table 5.2
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Religious Fasting

5.32 Use the updated International Diabetes Federation along with Diabetes and Ramadan International Alliance comprehensive prefasting risk assessment to generate a risk score for the safety of religious fasting. Provide fasting-focused education to minimize risks. **B**

5.33 Assess and optimize treatment plan, dose, and timing for people with diabetes well in advance of religious fasting to reduce risk of hypoglycemia, dehydration, hyperglycemia, and/or ketoacidosis. **B**

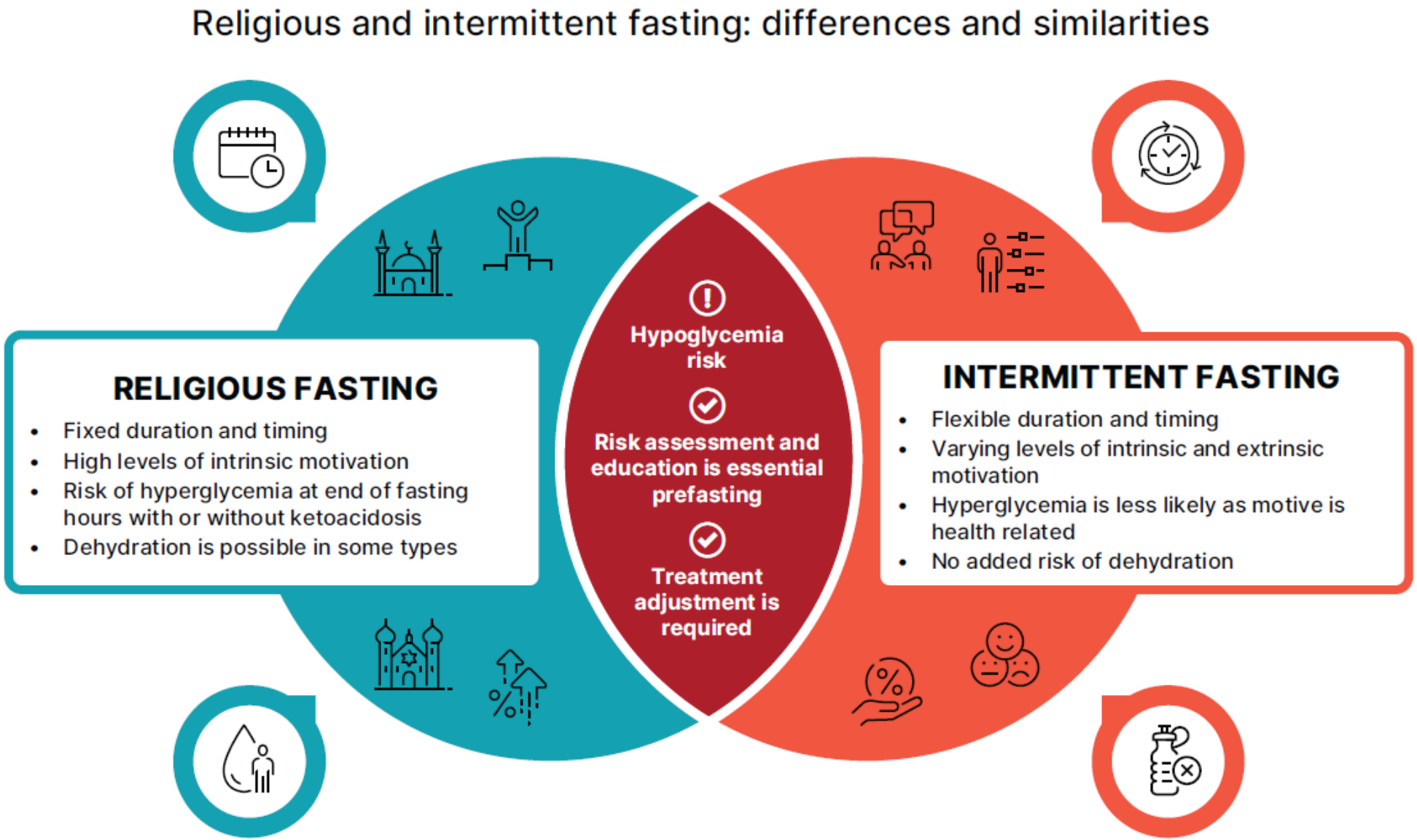


Figure 5.1—Differences and similarities between religious and intermittent fasting for people with diabetes.

Figure 5.1
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Table 5.3—Elements for risk calculation and suggested risk score for people with diabetes who seek to fast during Ramadan

Fasting risk element	Risk score
1. Pregnancy with any type of diabetes	
• Yes	6.5
2. Diabetes type	
• Type 1 diabetes or LADA	1
• Type 2 diabetes or any other type of diabetes	0
3. Duration of diabetes (years)	
• >20 years	1
• 10–20 years	0.5
• <10 years	0
4. Type of diabetes treatment (select all that are relevant)	
• Multiple daily premixed insulin injections	2
• Once-daily premixed insulin	1.5
• Open-loop insulin pump	1.5
• Automated insulin delivery system	1
• Standard basal insulin (NPH, detemir, or glargine 100)	1
• Ultra-long-acting basal insulin (glargine 300 or degludec)	0.75
• Short-acting insulin	0.75
• Glibenclamide or glipizide	0.75
• Modern sulfonylurea (gliclazide, gliclazide MR, glimepride) or repeglanide	0.5
• ≥2 glucose-lowering medications excluding insulin or sulfonylurea	0.25
• Nutrition modification only or monotherapy (excluding insulin or sulfonylurea)	0
5. Presence of hypoglycemia	
• Impaired hypoglycemia awareness	6.5
• Severe* hypoglycemia during last 4 weeks	5
• Hypoglycemia more than once daily	4
• 6–7 episodes of hypoglycemia/week	3
• 3–5 episodes of hypoglycemia/week	2
• 1–2 episodes of hypoglycemia/week	1
• Hypoglycemia <1 time per week	0.5
• No hypoglycemia in last 4 weeks	0
6. Level of A1C	
• >9% (>75 mmol/mol)	1
• 7.5–9% (58–75 mmol/mol)	0.5
• <7.5% (<58 mmol/mol)	0
7. Glucose monitoring	
• Not done	2
• Done suboptimally	1
• Done as indicated	0
• Any type of CGM	–0.5
8. Hyperglycemic emergencies	
• DKA or HHS in the last month	3.5
• DKA or HHS in last 2–3 months	2
• DKA or HHS in last 4–6 months	1
• No DKA or HHS in last 6 months	0
9. Macrovascular** complications	
• Unstable macrovascular disease	6.5
• Stable macrovascular disease	2
• No macrovascular disease	0
10. Microvascular complications	
a. Nephropathy	
• eGFR <30 mL/min	6.5
• eGFR 30–45 mL/min	4
• eGFR 45–60 mL/min	2
• eGFR >60 mL/min	0
b. Neuropathy, foot complications, or diabetic retinopathy	
• 3 microvascular complications	3
• 2 microvascular complications	2
• 1 microvascular complication	1
• 0 microvascular complications	0

Continued on p. S102

Table 5.3
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Table 5.3—Continued

Fasting risk element	Risk score
11. Cognitive function, frailty, and age	
• Impaired cognitive function	6.5
• Advanced frailty	6.5
• Mild to moderate frailty	4
• Age >70 years with no home support	1
• Normal cognitive function and no frailty	0
12. Physical labor	
• High intensity	4
• Moderate intensity	2
• Low intensity	0
13. Fasting-focused education	
• Yes	0
• No	1
14. Fasting hours	
• ≥16 h	1
• <16 h	0

Based on risk scoring, people with diabetes can be categorized as having:
- Score 0–3.0: low risk, fasting is probably safe
- Score 3.5–6.0: moderate risk, fasting safety is uncertain
- Score >6.0: high risk, fasting is probably unsafe

AID, automated insulin delivery; CGM, continuous glucose monitoring; DKA, diabetic ketoacidosis; eGFR, estimated glomerular filtration rate; HHS, hyperglycemic hyperosmolar state; LADA, latent autoimmune diabetes in adults (type 1 diabetes). *Hypoglycemia requiring assistance for treatment. **Macrovascular disease includes cardiac, cerebral, or peripheral. Adapted from Hassanein et al. (224).

Table 5.3
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes: *Standards of Care in Diabetes - 2026. Diabetes Care* 2026;49(Suppl. 1):S89-S131

Table 5.4—Changes in medications during fasting

Medication name	Risk of hypoglycemia	Timing	Total daily dose
Metformin, SGLT2 inhibitor, DPP-4 inhibitor, GLP-1 receptor agonist, acarbose, or pioglitazone	Low	• If once daily, then take at main mealtime. • If twice daily, then split dose between the two meals. • If once weekly, no change of time.	• No change
New generation of sulfonylurea (glimepiride and gliclazide)	Low to moderate	• If once daily, then take at main mealtime. • If twice daily, then split dose between the two meals.	• Reduce dose if glucose levels are within individualized goal range and if no hypoglycemia or hyperglycemia is present at baseline.
Older generation of sulfonylurea (glyburide)	Moderate to high	• Take at time of main meal	• Replace with newer-generation sulfonylurea or reduce dose by 50%.
Basal insulin	Moderate to high	• For longer-acting basal analogs (glargine 300 or degludec), no need to change timing. • For other basal insulins, take at beginning of breaking fast meal.	• Choose the insulin with lower risk of hypoglycemia among the class. • Reduce dose by 25–35% if not well managed.
Prandial insulin	High	• At mealtime	• Reduce dose of insulin for the meal followed by fasting (35–50%). • For other meals, insulin dose should match carbohydrate intake.
Mixed insulin and insulin coformulations	High	• If once daily, then take at main mealtime. • If twice daily, then split dose between the two meals	• Reduce dose of insulin for the meal followed by fasting (35–50%). • For other meals, no change of dose.

DPP-4, dipeptidyl peptidase 4; GLP-1, glucagon-like peptide 1; SGLT2, sodium–glucose cotransporter 2.

Table 5.4
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Physical Activity

5.34 Evaluate baseline physical activity and sedentary time for all people with diabetes and those at risk for diabetes. For people who do not meet activity guidelines, encourage an increase in physical activities above baseline with the goal of meeting activity guidelines. **B** Counsel that prolonged sitting should be interrupted at least every 30 min for blood glucose and other benefits. **C**

5.35 Counsel children and adolescents with type 1 diabetes **C** or type 2 diabetes **B** to engage in 60 min/day or more of moderate- or vigorous-intensity aerobic activity, with muscle-strengthening and bone-strengthening activities at least 3 days/week, and to limit the amount of time being spent sedentary, including recreational screen time. **C**

5.36 Counsel most adults with type 1 diabetes **C** and type 2 diabetes **B** to engage in 150 min or more of moderate-to vigorous-intensity aerobic activity per week, spread over at least 3 days/week, with no more than 2 consecutive days without activity. Shorter durations (minimum 75 min/week) of vigorous-intensity or interval training may be sufficient for more physically fit individuals.

Physical Activity (continued)

5.37 Counsel adults with type 1 diabetes **C** and type 2 diabetes **B** to engage in 2–3 sessions/week of resistance exercise on nonconsecutive days.

5.38 Counsel most older adults with diabetes to engage in flexibility training and balance training 2–3 times/week. **C**

5.39 Counsel all people with diabetes who are treated with obesity pharmacotherapy or metabolic surgery that meeting physical activity recommendations, in particular muscle-strengthening exercises, may be beneficial for maintaining lean body mass. **C**

Smoking Cessation: Tobacco, e-Cigarettes, and Cannabis

5.40. Ask people with diabetes routinely about the use of tobacco or vape products. **A** Advise complete avoidance of tobacco and vaping. **A** For individuals who use these products, provide or refer for combination treatment consisting of tobacco and/or vape product(s) cessation counseling and pharmacologic therapy. **A**

5.41 Advise people with type 1 diabetes **C** and those with other forms of diabetes at risk for diabetic ketoacidosis not to use recreational cannabis in any form. **E**

Psychosocial Care

5.42 Provide psychosocial care to all people with diabetes as part of routine medical care delivered by trained health care professionals using a collaborative, person-centered, culturally informed approach. **A**

5.43 Implement screening protocols for psychosocial concerns, preferably using age-appropriate standardized and validated tools. Screen at least annually or when there is a change in health status, treatment, or life circumstances. **C**

5.44 Refer to behavioral health professionals or other trained health care professionals, ideally those with experience in diabetes, for further assessment and treatment of psychosocial concerns as indicated. **B**

Table 5.5—Situations that warrant referral of a person with diabetes to a qualified behavioral health professional for evaluation and treatment

- A positive screen on a validated screening tool for depressive symptoms, diabetes distress, anxiety, fear of hypoglycemia, suicidality, or cognitive impairment
- The presence of symptoms or suspicions of disordered eating behavior, an eating disorder, or disrupted patterns of eating
- Intentional omission or underdosing of insulin or noninsulin medication to cause weight loss
- A serious mental illness is suspected
- In children and adolescents and families with behavioral self-care difficulties, repeated hospitalizations for diabetic ketoacidosis, failure to achieve expected developmental milestones, or significant distress
- Low engagement in diabetes self-management behaviors, including declining or impaired ability to perform diabetes self-management behaviors
- Before undergoing metabolic surgery and after surgery, if assessment reveals an ongoing need for adjustment support

Table 5.5
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Table 5.6—Psychosocial concerns and their association with diabetes-related outcomes in adults with type 1 diabetes

	Increased A1C	Increased blood pressure	Increased cholesterol	Increased macrovascular complications	Increased microvascular complications	Decreased self-care behaviors	Comorbid psychosocial concerns	Decreased quality of life	Increased mortality
Diabetes distress (576–579)	+++	?	+	+++	+++	+++	+++	+++	?
Depression and depressive symptoms (577,578,580,581)	+++	?	+++	+++	+++	+++	+++	+++	+++
Anxiety (383,582,583)	+++	?	?	?	?	+++	+++	+++	?
Disordered eating behaviors (insulin omission) (495,584)	+++	?	?	?	+++	+++	+++	+++	+++
Serious mental illness (schizophrenia, personality disorders) (585–587)	+++	?	+	+++	+++	?	+++	?	+++
Cognitive impairment (588–592)	+++	++ +	+++	+++	+++	++	+++	?	+++

+++, strong evidence (consistent findings in multiple studies of good methodological quality or one study of excellent methodological quality); ++, moderate evidence (consistent findings in multiple studies of fair methodological quality or one study of good methodological quality); +, limited evidence (evidence from one study of fair methodological quality); ?, no data available.

Table 5.6
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Table 5.7—Psychosocial concerns and their association with diabetes-related outcomes in adults with type 2 diabetes

	Increased A1C	Increased blood pressure	Increased dyslipidemia	Increased macrovascular complications	Increased microvascular complications	Decreased self-care behaviors	Comorbid psychosocial concerns	Decreased quality of life	Increased mortality
Diabetes distress (593–599)	+++	+	+	+++	+++	+++	+++	+++	+++
Depression and depressive symptoms (600–607)	+++	++	+++	+++	+++	+++	+++	+++	+++
Anxiety (382,430,601,608–611)	+++	++	+	+++	+	+++	+++	+++	+++
Disordered eating behaviors (binge eating disorder, night eating syndrome) (492,496,612–615)	+/–	+	?	?	+	+	+++	+++	?
Serious mental illness (schizophrenia, bipolar disorder) (616–624)	+/–	++	++	+++	+++	+++	+++	+++	+++
Cognitive impairment (625–632)	+++	+++	+++	+++	+++	+++	+++	+++	+++

+++, strong evidence (consistent findings in multiple studies of good methodological quality or one study of excellent methodological quality); ++, moderate evidence (consistent findings in multiple studies of fair methodological quality or one study of good methodological quality); +, limited evidence (evidence from one study of fair methodological quality); +/–, inconclusive evidence; ?, no data available.

Table 5.7
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Diabetes Distress

5.45 Screen for diabetes distress at least annually in people with diabetes, caregivers, and family members, and repeat screening when treatment goals are not met, at transitional times, and/or in the presence of diabetes complications. Health care professionals should consider referral to a qualified behavioral health professional, ideally one with experience in diabetes, for further assessment and treatment if not adequately addressed during medical appointments. **B**

Anxiety

5.46 Screen for anxiety symptoms at least annually in people with diabetes. Health care professionals can address anxiety symptoms within the scope of their practice. Consider referral to a qualified behavioral health professional for further assessment and treatment if anxiety symptoms interfere with diabetes self-management behaviors or quality of life, if not adequately addressed during medical appointments. **B**

5.47 Screen individuals at high risk for hypoglycemia or with severe and/or frequent hypoglycemia for fear of hypoglycemia at least annually and when clinically appropriate. **E** Refer to a trained health care professional for evidence-based intervention. **A**

Depression

5.48 Screen for depressive symptoms in all people with diabetes at least annually and more frequently among those with a history of depression. **B** Refer to qualified behavioral health professionals or other health care professionals with experience using evidence-based treatment approaches for depression in collaboration with the diabetes care team. **A**

5.49 Rescreen for depression at diagnosis of complications or when there are significant changes in medical status. **B**

Disordered Eating Behavior

5.50 Screen for disordered or disrupted eating using validated screening measures. Review the medical treatment plan to identify potential treatment-related effects on hunger/caloric intake. **B**

5.51 Reevaluate the treatment plan of people with diabetes who present with symptoms of disordered eating behavior, an eating disorder, or disrupted patterns of eating, ideally in consultation with a qualified professional. **B**

Serious Mental Illness

5.52 Provide an increased level of support for people with diabetes and serious mental illness through enhanced monitoring of and assistance with diabetes self-management behaviors. **B**

5.53 Monitor changes in body weight, glycemia, and lipids in adolescents and adults with diabetes who are prescribed second-generation antipsychotic medications; adjust the treatment plan accordingly, if needed. **C**

Cognitive Capacity and Impairment

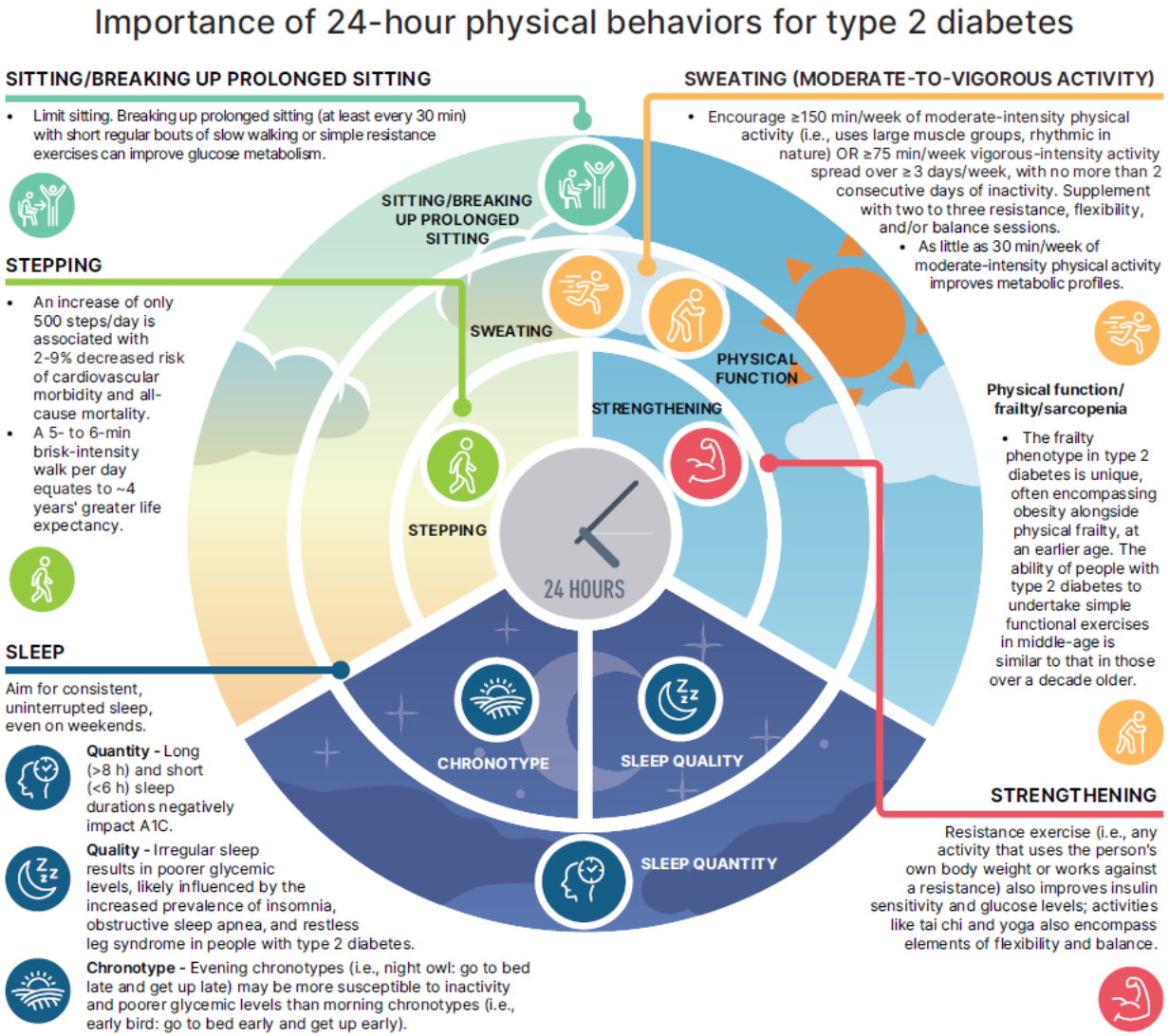
5.54 Monitor cognitive capacity throughout the life span for all individuals with diabetes, particularly in those who have documented cognitive disabilities, those who experience severe hypoglycemia, very young children, and older adults. **B**

5.55 Consider referral for a formal assessment if cognitive capacity changes or appears to be suboptimal for decision-making and/or behavioral self-management. **E**

Sleep Health

5.56 Screen for sleep health in people with prediabetes, diabetes, and those at risk for diabetes, including sleep disorders and diabetes-related sleep disruptions. Refer to sleep medicine specialists and/or qualified behavioral health professionals or diabetes care team as indicated. **B**

5.57 Counsel people with diabetes to practice sleep-promoting routines and habits. **A**



	Glucose/ insulin	Blood pressure	A1C	Lipids	Physical function	Depression	Quality of life
SITTING/BREAKING UP PROLONGED SITTING	↓	↓	↓	↓	↑	↓	↑
STEPPING	↓	↓	↓	↓	↑	↓	↑
SWEATING (MODERATE-TO-VIGOROUS ACTIVITY)	↓	↓	↓	↓	↑	↓	↑
STRENGTHENING	↓	↓	↓	↓	↑	↓	↑
ADEQUATE SLEEP DURATION	↓	↓	↓	↓	?	↓	↑
GOOD SLEEP QUALITY	↓	↓	↓	↓	?	↓	↑
CHRONOTYPE/CONSISTENT TIMING	↓	?	↓	?	?	↓	?

IMPACT OF PHYSICAL BEHAVIORS ON CARDIOMETABOLIC HEALTH IN PEOPLE WITH TYPE 2 DIABETES

↑ Higher levels of improvement (physical function, quality of life) ↓ Lower levels of improvement (glucose/insulin, blood pressure, A1C, lipids, depression)
? No data available
↑ Green arrows = strong evidence ↑ Yellow arrows = medium-strength evidence ↑ Red arrows = limited evidence

Figure 5.2—Importance of 24-h physical behaviors for type 2 diabetes. Adapted from Davies et al. (633).

Figure 5.2
Facilitating Positive Heath Behaviors and Well-being to Improve Health Outcomes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S89-S131

Section 6.

Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises

American Diabetes Association

(https://doi.org/10.2337/dc26-S006)

Glycemic Assessment

6.1 Assess glycemic status by A1C **A** and/or continuous glucose monitoring (CGM) metrics such as time in range, time above range, and time below range. **B** Fructosamine or CGM can be used for glycemic monitoring when an alternative to A1C is required. **B**

6.2 Assess glycemic status at least two times a year, and more frequently (e.g., every 3 months) for individuals not meeting glycemic goals or with recent treatment changes, frequent or severe hypoglycemia or hyperglycemia, or changes in health status, or during periods of rapid growth and development in children and adolescents. **E**

Table 6.1—Equivalent A1C levels and estimated average glucose (eAG)

A1C (%)	A1C (mmol/mol)	eAG mg/dL*	eAG mmol/L
5	31	97 (76–120)	5.4 (4.2–6.7)
6	42	126 (100–152)	7.0 (5.5–8.5)
7	53	154 (123–185)	8.6 (6.8–10.3)
8	64	183 (147–217)	10.2 (8.1–12.1)
9	75	212 (170–249)	11.8 (9.4–13.9)
10	86	240 (193–282)	13.4 (10.7–15.7)
11	97	269 (217–314)	14.9 (12.0–17.5)
12	108	298 (240–347)	16.5 (13.3–19.3)

Data in parentheses are 95% CI. A calculator for converting A1C results into eAG, in either mg/dL or mmol/L, is available at professional.diabetes.org/eAG. *These estimates are based on ADAG data of ~2,700 glucose measurements over 3 months per A1C measurement in 507 adults with type 1, type 2, or no diabetes. The correlation between A1C and average glucose was 0.92 (19,20). Adapted from Nathan et al. (19).

Table 6.1
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Table 6.2—CGM metrics for clinical care in nonpregnant individuals with type 1 or type 2 diabetes

Metric	Interpretation	Goals
Metrics for valid CGM wear		
Wear time	Number of days CGM device is worn	≥14-day wear for pattern management
Active percentage time	Percent of time CGM device is active	70% of time active out of 14 days
Glycemic metrics		
Mean glucose	Mean of glucose values	*
Glucose management indicator (GMI)	Calculated value approximating A1C (not always equivalent)	*
Glucose coefficient of variation (CV)	Spread of glucose values	≤36%†
TAR >250 mg/dL (>13.9 mmol/L)	Percent of time in level 2 hyperglycemia	<5% (most adults); <10% (older adults with complex/intermediate health)
TAR >180 mg/dL (>10 mmol/L)	Percent of time in level 1 hyperglycemia	<25% (most adults); <50% (older adults with complex/intermediate health)‡
TIR 70–180 mg/dL (3.9–10.0 mmol/L)	Percent of time in range	>70% (most adults); >50% (older adults with complex/intermediate health)
TBR <70 mg/dL (<3.9 mmol/L)	Percent of time in level 1 hypoglycemia	<4% (most adults); <1% (older adults with complex/intermediate health)§
TBR <54 mg/dL (<3.0 mmol/L)	Percent of time in level 2 hypoglycemia	<1%

CGM, continuous glucose monitoring; TAR, time above range; TBR, time below range; TIR, time in range. *Goals for these values are not standardized. †Some studies suggest that lower coefficient of variation targets (<33%) provide additional protection against hypoglycemia for those receiving insulin or sulfonylureas. ‡Goals are for level 1 and level 2 hyperglycemia combined. §Goals are for level 1 and level 2 hypoglycemia combined. Adapted from Battelino et al. (33).

Table 6.2
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Glycemic Goals

6.3a An A1C goal of <7% (<53 mmol/mol) is appropriate for many nonpregnant adults without severe hypoglycemia or hypoglycemia affecting health or quality of life. **A**

6.3b A goal time in range of >70% in people using CGM is appropriate for many nonpregnant adults**. B**

6.3c A goal percent time <70 mg/dL (<3.9 mmol/L) of <4% (or <1% for older adults) and a goal percent time <54 mg/dL (<3.0 mmol/L) of <1% are recommended in people using CGM to prevent hypoglycemia. Deintensify or modify therapy if these goals are not met. **B**

6.4 Lower A1C goals (e.g., <6.5% [<48 mmol/mol]) may be appropriate for individuals with diabetes with good health and function and low treatment risks (e.g., hypoglycemia) and burdens (see Fig. 6.1). **B**

6.5 Less stringent glycemic goals may be appropriate for individuals with significant cognitive and/or functional limitations, frailty, or severe comorbidities or where the harms of treatment, including hypoglycemia, are greater than the benefits. **B**

Glycemic Goals (continued)

6.6 Deintensify hypoglycemia-causing medications (insulin, sulfonylureas, or meglitinides), or switch to a medication class with lower hypoglycemia risk, for individuals who are at high risk for hypoglycemia, within individualized glycemic goals. **B**

6.7 Deintensify diabetes medications for individuals for whom the harms and/or burdens of treatment may be greater than the benefits, within individualized glycemic goals. **B**

6.8 Reassess glycemic goals based on the individualized criteria shown in **Fig 6.1**. **E**

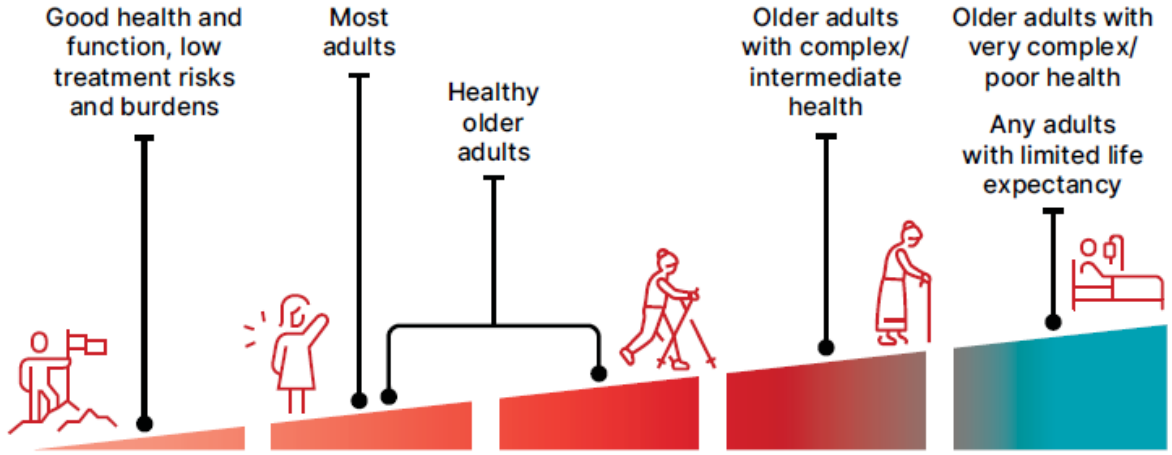
6.9 Set a glycemic goal during consultations to improve outcomes. **A**

Table 6.3—Summary of glycemic goals for many nonpregnant adults with diabetes

A1C	<7.0% (<53 mmol/mol)*†
Preprandial capillary plasma glucose	80–130 mg/dL* (4.4–7.2 mmol/L)
Peak postprandial capillary plasma glucose‡	<180 mg/dL* (<10.0 mmol/L)

*More or less stringent glycemic goals may be appropriate for certain individuals. †CGM may be used to assess glycemic status as noted in Recommendations 6.3b and 6.3c. Goals should be individualized based on duration of diabetes, age and life expectancy, comorbid conditions, known cardiovascular disease or advanced microvascular complications, impaired awareness of hypoglycemia, and individual considerations (per **Fig. 6.1**). ‡Postprandial glucose may warrant special attention if A1C goals are not met despite reaching preprandial glucose goals. Postprandial glucose measurements should be made 1–2 h after the beginning of the meal, which is generally the timing for peak levels in people with diabetes.

Table 6.3
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149



A1C goals	<6.5%	<7.0%	<7.5%	<8.0%	No A1C goal
CGM goals TIR:	—	>70%	—	>50%	—
• TBR <70	—	<4%	—	<1%	<1%
• TBR <54	—	<1%	—	<1%	<1%
• TAR >180	—	<25%	—	<50%	Avoid symptomatic hyperglycemia
• TAR >250	—	<5%	—	<10%	

Modifying Factors

Favor more stringent goal	Favor less stringent goal
Short diabetes duration	Long diabetes duration
Low hypoglycemia risk	High hypoglycemia risk
Low treatment risks and burdens	High treatment risks and burdens
Pharmacotherapy with cardiovascular, kidney, weight, or other benefits	Pharmacotherapy without nonglycemic benefits
No cardiovascular complications	Established cardiovascular complications
Few or minor comorbidities	Severe, life-limiting comorbidities

Figure 6.1
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Figure 6.1—Individualized A1C and CGM goals for nonpregnant adults. Select the glycemic goal based on individual health and function as described at the top of the figure. Consider modifying to a more or less stringent goal according to the factors listed in the table. Older adults are classified as healthy (few coexisting chronic illnesses, intact cognitive and functional status), as having complex/intermediate health (multiple coexisting chronic illnesses, two or more instrumental impairments to activities of daily living, or mild to moderate cognitive impairment), or as having very complex/poor health (long-term care or end-stage chronic illnesses, moderate to severe cognitive impairment, or two or more impairments to activities of daily living). Select glycemic goals that avoid symptomatic hypoglycemia and hyperglycemia in all individuals. Consider individuals' resources and support systems to safely achieve glycemic goals. Incorporate the preferences and goals of people with diabetes through shared decision-making. CGM, continuous glucose monitoring; TAR, time above range; TBR, time below range; TIR, time in range.

Figure 6.1 (continued)
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Glucose Lowering and Cardiovascular Disease Outcomes

As outlined in more detail in section 9, “Pharmacologic Approaches to Glycemic Treatment,” and section 10, “Cardiovascular Disease and Risk Management,” the cardiovascular benefits of SGLT2 inhibitors or GLP-1 Ras are not contingent upon A1C lowering; therefore, initiation should be considered in people with type 2 diabetes and CVD, kidney disease, heart failure, and/or obesity independent of the current A1C, A1C goal, or metformin therapy. Based on these considerations, the following two strategies are offered (65):

1. If already on dual therapy or multiple glucose-lowering therapies and not on an SGLT2 inhibitor or a GLP-1 RA or dual GIP and GLP-1 RA, consider switching to one of these agents with proven cardiovascular benefit.

2. Introduce SGLT2 inhibitors or GLP-1 RAs or a dual GIP and GLP-1 RA in people with CVD at A1C goal (independent of metformin) for cardiovascular benefit, independent of baseline A1C or individualized A1C goal.

Hypoglycemia Assessment, Prevention, and Treatment

6.10 Review history of hypoglycemia at every clinical encounter for all individuals at risk for hypoglycemia, and evaluate hypoglycemic events as indicated. **C**

6.11 Screen individuals at risk for hypoglycemia for impaired hypoglycemia awareness at least annually and when clinically appropriate. **E** Refer to a trained health care professional for evidence-based intervention to improve hypoglycemia awareness. **A**

6.12 Screen individuals at high risk for hypoglycemia or with severe and/or frequent hypoglycemia for fear of hypoglycemia at least annually and when clinically appropriate. **E** Refer to a trained health care professional for evidence-based intervention. **A**

6.13 Consider an individual's risk for hypoglycemia (see **Table 6.5**) when selecting diabetes medications and glycemic goals. **B**

6.14 Use of CGM is beneficial and recommended for individuals at high risk for hypoglycemia. **A**

6.15 Glucose is the preferred treatment for the conscious individual with glucose <70 mg/dL (<3.9 mmol/L), although any form of carbohydrate that contains glucose may be used. Avoid using foods or beverages high in fat and/or protein for initial treatment of hypoglycemia. Fifteen minutes after initial treatment, repeat the treatment if hypoglycemia persists. **B**

Hypoglycemia Assessment, Prevention, and Treatment (continued)

6.16 Glucagon should be prescribed for all individuals taking insulin or at high risk for hypoglycemia. **A** Family, caregivers, school personnel, and others providing support to these individuals should know its location and be educated on how to administer it. Glucagon preparations that do not have to be reconstituted are preferred. **B**

6.17 First aid kits should include oral glucose for use in treating hypoglycemia. **C**

6.18 All individuals taking insulin **A** or at risk for hypoglycemia **C** should receive structured education for hypoglycemia prevention and treatment, with ongoing education for those who experience hypoglycemic events.

6.19 One or more episodes of level 2 or 3 hypoglycemia should prompt reevaluation of the treatment plan, including deintensifying or switching diabetes medications if appropriate. **B**

6.20 Regularly assess cognitive function; if impaired or declining cognition is found, the clinician, person with diabetes, and caregiver should increase vigilance for hypoglycemia. **B**

Table 6.4—Classification of hypoglycemia

	Glycemic criteria/description
Level 1	Glucose <70 mg/dL (<3.9 mmol/L) and ≥54 mg/dL (≥3.0 mmol/L)
Level 2	Glucose <54 mg/dL (<3.0 mmol/L)
Level 3	A severe event characterized by altered mental and/or physical status requiring assistance for treatment of hypoglycemia, irrespective of glucose level

Adapted from Agiostratidou et al. (71).

Table 6.4
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Table 6.5—Assessment of hypoglycemia risk among individuals treated with insulin, sulfonylureas, or meglitinides

Clinical and biological risk factors	Social, cultural, and economic risk factors
Major risk factors • Recent (within the past 3–6 months) level 2 or 3 hypoglycemia • Intensive insulin therapy* • Impaired hypoglycemia awareness • Kidney failure • Cognitive impairment or dementia • History of metabolic surgery	Major risk factors • Food insecurity • Low-income status§ • Housing insecurity • Fasting for religious or cultural reasons • Underinsurance
Other risk factors • Multiple recent episodes of level 1 hypoglycemia • Basal insulin therapy* • Age ≥75 years† • Female sex • High glycemic variability‡ • Polypharmacy • Cardiovascular disease • Chronic kidney disease (eGFR <60 mL/min/1.73 m^2 or albuminuria) • Neuropathy • Retinopathy • Major depressive disorder • Severe mental illness • Gastroparesis • β-Blocker therapy	Other risk factors • Low health literacy • Alcohol or substance use disorder

Major risk factors are those that have a consistent, independent association with a high risk for level 2 or 3 hypoglycemia. Other risk factors are those with less consistent evidence or a weaker association. These risk factors are identified through observational analyses and are intended to be used for hypoglycemia risk stratification. Individuals considered at high risk for hypoglycemia are those with ≥1 major risk factor or who have multiple other risk factors (determined by the health care professional incorporating clinical judgment) (88, 89, 94, 96–99, 117, 119, 122, 190, 191, 88, 89, 94, 96–99, 117, 120, 191, 192). Proximal causes of hypoglycemic events (e.g., exercise and sleep) are not included. eGFR, estimated glomerular filtration rate. *Rates of hypoglycemia are highest for individuals treated with intensive insulin therapy (including multiple daily injections of insulin, continuous subcutaneous insulin infusion, or automated insulin delivery systems), followed by basal insulin, followed by sulfonylureas or meglitinides. Combining treatment with insulin and sulfonylureas further increases hypoglycemia risk. †Accounting for treatment plan and diabetes subtype, the oldest individuals (aged ≥75 years) have the highest risk for hypoglycemia in type 2 diabetes; younger individuals with type 1 diabetes are also at very high risk. ‡Tight glycemic management in randomized trials increases hypoglycemia rates. In observational studies, both low and high A1C are associated with hypoglycemia in a J-shaped relationship. §Includes factors associated with low income, such as living in a socioeconomically deprived area.

Table 6.5
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Table 6.6—Median monthly (30-day) AWP and NADAC of glucagon formulations in the U.S.

Product	Form	Median AWP* (min, max)	Median NADAC* (min, max)	Dosage
Glucagon	Injection powder with diluent for reconstitution	$303 ($180, $337)	$242 ($207, $242)	1 mg
Glucagon	Nasal powder	$357	$285	3 mg
Glucagon	Prefilled pen, prefilled syringe	$391	$303 ($303, $304)	0.5 mg, 1 mg
Dasiglucagon	Prefilled pen, prefilled syringe	$371	$298	0.6 mg

AWP, average wholesale price; max, maximum; min, minimum; NADAC, National Average Drug Acquisition Cost. AWP and NADAC prices are as of 1 July 2025. *Calculated per unit (AWP [192] or NADAC [193]); median AWP or NADAC is listed alone when only one product and/or price is described).

Table 6.6
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Table 6.7—Components of hypoglycemia prevention for individuals at risk for hypoglycemia at initial, follow-up, and annual visits

Hypoglycemia prevention action	Initial visit	Every follow-up visit	Annual visit
Hypoglycemia history assessment	✓	✓	✓
Hypoglycemia awareness assessment	✓		✓
Cognitive function and other hypoglycemia risk factor assessment	✓		✓
Structured individual education for hypoglycemia prevention and treatment	✓	✓*	✓*
Consideration of diabetes technology needs	✓	✓	✓
Reevaluation of diabetes treatment plan with deintensification, simplification, or agent modification as appropriate	✓	✓†	✓†
Glucagon prescription and training for close contacts for insulin-treated individuals or those at high hypoglycemic risk	✓		✓
Training to improve hypoglycemia awareness	✓‡		✓‡

The listed frequencies are the recommended minimum; actions for hypoglycemia prevention should be taken more often as needed based on clinical judgment. *Indicated with recurrent hypoglycemic events or at initiation of medication with a high risk for hypoglycemia. †Indicated with any level 2 or 3 hypoglycemia, intercurrent illness, or initiating interacting medications. ‡Indicated when impaired hypoglycemia awareness is detected.

Table 6.7
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Hyperglycemic Crises: Diagnosis, Management, and Prevention

6.21 Review history of hyperglycemic crises (i.e., diabetic ketoacidosis and hyperglycemic hyperosmolar state) at every clinical encounter for all individuals with diabetes at risk for these events. **C**

6.22 Provide structured education on the recognition, prevention, and management of hyperglycemic crisis to all individuals with type 1 diabetes, those with type 2 diabetes who have experienced these events, and people at high risk for these events. **B**

Table 6.8—Risk factors for hyperglycemic crises
Type 1 diabetes/absolute insulin deficiency
Younger age
Prior history of hyperglycemic crises
Prior history of hypoglycemic crises
Presence of other diabetes complications
Presence of other chronic health conditions (particularly in people with type 2 diabetes)
Presence of behavioral health conditions (e.g., depression, bipolar disorder, and eating disorders)
Alcohol and/or substance use
High A1C level
Insulin rationing
SGLT2 (SGLT1/2) inhibitor use
Social determinants of health

SGLT, sodium–glucose cotransporter. Data are from McCoy et al. (194), Gibb et al. (195), Randall et al. (196), Thomas et al. (197), and Borden et al. (198).

Table 6.8
Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S132-S149

Section 7.

Diabetes Technology

American Diabetes Association

(https://doi.org/10.2337/dc26-S007)

General Device Principles

7.1 Diabetes devices should be offered to people with diabetes. **A**

7.2 The type(s) and selection of devices should be individualized based on a person's specific needs, circumstances, preferences, and skill level. In the setting of an individual whose diabetes is partially or wholly managed by someone else (e.g., a young child or a person with cognitive impairment or dexterity, psychosocial issues, and/or physical limitations), the caregiver's skills and preferences are integral to the decision-making process. **E**

7.3a When prescribing a continuous glucose monitoring (CGM) device, ensure that people with diabetes and caregivers are offered initial and ongoing training and education as indicated by individual circumstances. Education should include utilization of data, including uploading or sharing data to monitor and adjust therapy. **C**

7.3b When prescribing an automated insulin delivery (AID) system, people with diabetes and their caregivers must receive education on how to use and troubleshoot the system. This education should occur at regular intervals as needed. Education should include utilization of the integrated system and its data, including uploading or sharing data to monitor and adjust therapy. **C**

General Device Principles (continued)

7.4 Health care professionals working with people with diabetes should be aware of available technologies and seek additional support when needed. **E**

7.5 People with diabetes using CGM, continuous subcutaneous insulin infusion (CSII), and/or AID for diabetes management should have continued access to devices across third-party payors, regardless of age or A1C levels. **E**

7.6 Children and adolescents should be supported at school in the use of diabetes technology, such as CGM systems, CSII, connected insulin pens, and AID systems. **E**

7.7 For adults with diabetes using diabetes technology, reasonable accommodations in educational and work settings should include having sufficient time to manage their devices and respond to high and low glucose levels. **E**

General Device Principles (continued)

7.8 Consider early initiation, including at diagnosis, of CGM, CSII, and AID depending on a person's or caregiver's needs and preferences. **C**

7.8a There should be no requirement of C-peptide level, **B** the presence of islet autoantibodies, **B** or duration of insulin treatment **C** before initiation of CSII or AID.

7.9 Standardized reports for all CGM, CSII, AID, and connected insulin devices with a minimum of a single-page report, such as the ambulatory glucose profile and weekly summary, should be available and utilized. Options for daily and weekly reports and raw data should be available. **E**

Blood Glucose Monitoring

7.10 People with diabetes should be provided with blood glucose monitoring (BGM) devices as indicated by their circumstances, preferences, and treatment. People using CGM devices must also have access to BGM at all times. **A**

7.11 People who are taking insulin and using BGM should be encouraged to check their blood glucose levels when appropriate based on their insulin therapy. This may include checking when fasting, prior to meals and snacks, after meals, at bedtime, in the middle of the night, prior to, during, and after exercise, when hypoglycemia is suspected, after treating low blood glucose levels until achieving normoglycemia, when hyperglycemia is suspected, and prior to and while performing critical tasks such as driving. **B**

7.12 Health care professionals should be aware of the differences in accuracy among blood glucose meters. Only meters approved by the U.S. Food and Drug Administration (FDA) (or comparable regulatory agencies for other geographical locations) with proven accuracy should be used, with unexpired test strips purchased from a pharmacy or licensed distributor and properly stored. **E**

Blood Glucose Monitoring (continued)

7.13 Although BGM in people on noninsulin therapies has not consistently shown clinically significant reductions in A1C levels, it may be helpful when modifying meal plans, physical activity plans, and/or medications (particularly medications that can cause hypoglycemia) in conjunction with a treatment adjustment program. **E**

7.14 Consider potential interference of medications and substances on glucose levels measured by blood glucose meters. **B**

Table 7.1—Comparison of ISO 15197:2013 and FDA blood glucose meter accuracy standards

Setting	FDA*	ISO 15197:2013*
Hospital use	95% within 12% for BG ≥75 mg/dL 95% within 12 mg/dL for BG <75 mg/dL 98% within 15% for BG ≥75 mg/dL 98% within 15 mg/dL for BG <75 mg/dL	95% within 15% for BG ≥100 mg/dL 95% within 15 mg/dL for BG <100 mg/dL 99% in A or B region of consensus error grid‡
Home use	95% within 15% for all BG in the usable BG range† 99% within 20% for all BG in the usable BG range†	

BG, blood glucose; FDA, U.S. Food and Drug Administration; ISO, International Organization for Standardization. To convert mg/dL to mmol/L, see endmemo.com/medical/unitconvert/Glucose.php. *Data shown in the FDA column are from the FDA (191). Data shown in the ISO column are from the FDA (192). †The range of blood glucose values for which the meter has been proven accurate and will provide readings (other than low, high, or error). ‡Values outside of the "clinically acceptable" A and B regions are considered "outlier" readings and may be dangerous to use for therapeutic decisions (193).

Table 7.1
Diabetes Technology:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S150-S165

Continuous Glucose Monitoring Devices

7.15 Use of CGM is recommended at diabetes onset and anytime thereafter for children, adolescents, and adults with diabetes who are on insulin therapy, **A** on noninsulin therapies that can cause hypoglycemia, **C** and on any diabetes treatment where CGM helps in management. **C** The specific CGM device and method for use should be made based on the individual's circumstances, preferences, and needs. **E**

7.16 In people with diabetes on insulin therapy, CGM devices should be used as close to daily as possible for maximal benefit. A People with diabetes should have uninterrupted access to their supplies to minimize gaps in CGM. **A**

7.17 During pregnancy for individuals with type 1 diabetes, CGM can help achieve glycemic goals (e.g., time in range and time above range) A and A1C goal **B** and may be beneficial for other types of diabetes in pregnancy. **E** See section 15, "Management of Diabetes in Pregnancy," for more detail regarding use of technology in pregnancy.

Continuous Glucose Monitoring Devices (continued)

7.18 In circumstances when consistent use of CGM is not feasible, consider periodic use of personal or professional CGM to adjust medication and/or lifestyle. **C**

7.19 Skin reactions, either due to irritation or allergy, should be assessed and addressed to aid in successful use of devices. **E**

7.20 People who wear CGM devices should be educated on potential interfering substances and other factors that may affect accuracy. **C**

Table 7.2—Some of the more common interfering substances and/or conditions that affect blood glucose meters (for inpatient and outpatient use)

Substance or condition	Potential effects on glucose readings measured by BGMs*
Maltose†	Falsely higher blood glucose readings
Galactose	Falsely higher blood glucose readings
Xylose	Falsely higher blood glucose readings
N-Acetylcysteine	Falsely higher or lower blood glucose readings (depending on BGM design)
Acetaminophen	Falsely higher or lower blood glucose readings (depending on BGM design)
Dopamine	Falsely higher or lower blood glucose readings (depending on BGM design)
Pralidoxime (2-PAM)	Falsely higher or lower blood glucose readings (depending on BGM design)
Hydroxyurea	Falsely higher or lower blood glucose readings (depending on BGM design)
Vitamin C	Falsely higher or lower blood glucose readings (depending on BGM design)
Hematocrit (high)	Falsely lower blood glucose readings
Hematocrit (low)	Falsely higher blood glucose readings

*These are potential effects. There are blood glucose monitors (BGMs) that behave differently than listed in this table. Refer to product labeling for product-specific information. †Unmodified glucose dehydrogenase pyrroloquinoline quinone (GDH/PQQ) enzyme method only. Modern BGM designs do not incorporate unmodified GDH-PQQ enzyme.

Table 7.2
Diabetes Technology:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S150-S165

Table 7.3—Continuous glucose monitoring devices

Type of device	Brand*	Availability	Alarms
rtCGM	Libre 2 Plus/Libre 3 Plus	Prescription	Yes
	Dexcom G6/G7	Prescription	Yes
	Eversense 365	Prescription	Yes
	Guardian 4	Prescription	Yes
	Simplera	Prescription	Yes
OTC-CGM	Dexcom Stelo	OTC	No
	Abbott Lingo	OTC	No
Professional CGM	Abbott Freestyle Libre Pro	In office	No
	Dexcom G6 Pro	In office	No

CGM, continuous glucose monitoring; isCGM, intermittently scanned CGM; OTC, over the counter; rtCGM, real-time CGM. *Generic names not available.

Table 7.3
Diabetes Technology:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S150-S165

Table 7.4—Continuous glucose monitoring device interfering substances

Medication	Systems affected	Effect
Acetaminophen		
>4 g/day	Dexcom G6, Dexcom G7	Higher sensor readings than actual glucose
Any dose	Medtronic Guardian 4	Higher sensor readings than actual glucose
Ascorbic acid (vitamin C), >500 mg/day	FreeStyle Libre 2, FreeStyle Libre 3	Higher sensor readings than actual glucose
Ascorbic acid (vitamin C), >1,000 mg/day	FreeStyle Libre 2 Plus, FreeStyle Libre 3 Plus	Higher sensor readings than actual glucose
Hydroxyurea	Dexcom G6, Dexcom G7, Medtronic Guardian 4	Higher sensor readings than actual glucose
Mannitol (intravenously or as peritoneal dialysis solution)	Senseonics Eversense365	Higher sensor readings than actual glucose
Sorbitol (intravenously or as peritoneal dialysis solution)	Senseonics Eversense365	Higher sensor readings than actual glucose

Table 7.4
Diabetes Technology:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S150-S165

Insulin Delivery (syringes and pens)

7.21 For people with insulin-requiring diabetes on MDI, insulin pens are preferred in most cases. Still, insulin syringes may be used for insulin delivery considering individual and caregiver preference, insulin type, availability in vials, dosing therapy, cost, and self management capabilities. **C**

7.22 Insulin pens or insulin injection aids are recommended for people with dexterity issues or vision impairment or when decided by shared decision making to facilitate the accurate dosing and administration of insulin. **C**

7.23 Offer connected insulin pens for people with diabetes taking multiple daily insulin injections when appropriate. **B**

7.24 FDA-approved insulin dose calculators/decision support systems may be helpful for calculating insulin doses. **B**

Insulin Delivery (Pumps and Automated Delivery Systems)

7.25a AID systems are the preferred insulin delivery method over multiple daily injections (MDI), CSII, and sensor- augmented pump in people with type 1 diabetes, **A** adults with type 2 diabetes, **A** children and adolescents with type 2 diabetes, **E** and other forms of insulin-deficient diabetes. **B–E** Choice of an AID system should be made based on the individual's circumstances, preferences, and needs. **E**

7.25b Consider AID systems for select people with type 2 diabetes treated with basal insulin not achieving individualized glycemic goals. **B** Choice of an AID system should be made based on the individual's circumstances, preferences, and needs. **E**

7.26 Individuals with diabetes who have been using CSII and/or AID should have continued access across third party payors. **E**

Insulin Delivery (Open-Source Automated Insulin Dosing)

7.27 Support and provide diabetes management advice to people with diabetes who choose to use an opensource AID system. **B**

Digital Health Technology

7.28 Consider combining technology (CGM, insulin pump, and/or diabetes apps) with online or virtual licensed coaching to improve glycemic outcomes in individuals with diabetes or prediabetes. **B**

Inpatient Care

7.29 In people with diabetes wearing personal CGM, the use of CGM should be continued when clinically appropriate during hospitalization, with confirmatory point-of-care glucose measurements for insulin dosing and hypoglycemia assessment and treatment under an institutional protocol. **B**

7.30 Continue use of insulin pump or AID in people with diabetes who are hospitalized when clinically appropriate. This is contingent upon availability of necessary supplies, resources, training, ongoing competency assessments, and implementation of institutional diabetes technology protocols. **C**

Section 8.

Obesity and Weight Management for the Prevention and Treatment of Diabetes

(https://doi.org/10.2337/dc26-S008)

Assessment and Monitoring of the Individual with Overweight or Obesity

8.1 Use person-centered, nonjudgmental language that fosters collaboration between individuals and health care professionals, including person-first language (e.g., “person with obesity” rather than “obese person” and “person with diabetes” rather than “diabetic person”). **E**

8.2a Screen for overweight and obesity using BMI annually. To confirm excess adiposity, additional assessments of body fat using anthropometric assessments (e.g., waist-to-hip ratio) or direct measurements (e.g., dual-energy X-ray absorptiometry, bioelectrical impedance analysis) could be considered where available/feasible. **E**

8.2b Monitor obesity-related anthropometric measurements at least annually to inform treatment considerations. During active weight management treatment, increase monitoring to at least every 3 months. **E**

8.3 Accommodations should be made to provide privacy during anthropometric measurements. **E**

Assessment and Monitoring of the Individual with Overweight or Obesity (continued)

8.4 In people with type 2 diabetes and overweight or obesity, weight management should represent a primary goal of treatment along with glycemic management. **A**

8.5 Provide weight management treatment, aiming for any magnitude of weight loss. Weight loss of 5–7% of baseline weight improves glycemia and other intermediate cardiovascular risk factors. **A** Sustained loss of >10% of body weight usually confers greater benefits, including disease-modifying effects and possible remission of type 2 diabetes **A** and may improve long-term cardiovascular outcomes and mortality. **B**

8.6 Individualize initial treatment approaches for obesity (i.e., lifestyle and nutritional therapy, pharmacologic therapy, or metabolic surgery) **A** based on the person's medical history, life circumstances, and preferences. **C** Consider combining treatment approaches if appropriate. **C**

Nutrition, Physical Activity, and Behavioral Therapy Interventions

8.7 Nutrition, physical activity, and behavioral therapy are recommended for people with type 2 diabetes and overweight or obesity to achieve both weight and health outcome goals. **B**

8.8a Interventions including high frequency of counseling (≥16 sessions in 6 months) with focus on nutrition changes, physical activity, and behavioral strategies to achieve a 500–750 kcal/day energy deficit (irrespective of macronutrient composition) should be recommended for weight loss when available. **A**

8.8b If access to such interventions is limited, consider alternative structured programs delivering nutrition changes, physical activity, and behavioral counseling (e.g., remote, telehealth, mobile app). **E**

8.9 Nutrition recommendations should be individualized to the person's preferences and nutritional needs. Use nutritional plans that create an energy deficit, while still following general nutritional guidance, to achieve weight loss. **A**

Nutrition, Physical Activity, and Behavioral Therapy Interventions (continued)

8.10 When developing a plan of care, consider systemic, structural, cultural, and socioeconomic factors that may impact nutrition patterns and food choices, such as food insecurity and hunger, access to healthful food options, and other social determinants of health. **C**

8.11 For those who achieve weight loss goals, continue to monitor progress, provide ongoing support, and recommend continuing interventions to maintain weight goals long term. **E** Effective long-term (≥1 year) weight maintenance programs provide monthly contact and support, include frequent self-monitoring of body weight (weekly or more frequently) and other self-monitoring strategies (e.g., food diaries or wearables), and encourage regular physical activity (200–300min/week). **A**

8.12 Short-term nutrition intervention using structured, very-low-calorie-meals (800–1,000 kcal/day) should be prescribed only to carefully selected individuals by trained practitioners in medical settings with close monitoring. Long-term, comprehensive weight maintenance strategies and counseling should be integrated to maintain weight loss. **B**

Nutrition, Physical Activity, and Behavioral Therapy (continued)

8.13 Nutritional supplements are not recommended, as they have not been shown to be effective for weight loss. **A**

8.14 Counsel and regularly monitor individuals pursuing intentional weight loss to ensure adequate nutritional intake, with particular attention to preventing protein insufficiency and micronutrient deficiencies. **E**

Pharmacotherapy

8.15 Whenever clinically appropriate, engage other care team members to minimize use of weight-promoting medications for treatment of other conditions among adults with diabetes and obesity. **E**

8.16 When choosing glucose-lowering medications for people with type 2 diabetes and overweight or obesity, prioritize medications with beneficial effect on weight. **B**

8.17 Obesity pharmacotherapy should be considered for people with diabetes and overweight or obesity along with lifestyle changes. Potential benefits and risks must be considered. **A**

8.18 In people with diabetes and overweight or obesity, the preferred pharmacotherapy should be a glucagonlike peptide 1 receptor agonist or dual glucose-dependent insulinotropic polypeptide and glucagon-like peptide 1 receptor agonist with greater weight loss efficacy (i.e., semaglutide or tirzepatide), especially considering their added weight-independent benefits. **A**

Pharmacotherapy (continued)

8.19 Obesity pharmacotherapy indicated for chronic therapy should be continued beyond reaching weight loss goals to maintain the health benefits, as discontinuation often results in recurrence of weight gain and worsening or reemergence of cardiometabolic risk factors. **B**

8.20 Individualize the dose and the dose titration approach of obesity pharmacotherapy to balance effectiveness, health benefits, and tolerability; the optimal treatment dose may not be the maximum approved dose. **B**

8.21 In people with diabetes not reaching weight treatment goals, modify or intensify treatment with additional approaches, including structured lifestyle management programs, metabolic surgery, **A** and additional or alternative pharmacologic agents. **B**

Metabolic Surgery

8.22 Consider metabolic surgery as a weight and glycemic management approach in people with type 2 diabetes with BMI ≥30.0 kg/m^2 (or ≥27.5 kg/m^2 in Asian American individuals) who are otherwise good surgical candidates. **A**

8.23 Metabolic surgery should be performed in high-volume centers with interprofessional teams knowledgeable about and experienced in managing obesity, diabetes, and gastrointestinal surgery. **E**

8.24 People being considered for metabolic surgery should be evaluated for comorbid psychological conditions and social and situational circumstances that have the potential to interfere with surgery outcomes. **B**

8.25 People who undergo metabolic surgery should receive long-term medical and behavioral support and routine micronutrient, nutritional, and metabolic status monitoring. **B**

Metabolic Surgery (continued)

8.26 If post-metabolic surgery hypoglycemia is suspected, clinical evaluation should exclude other potential disorders contributing to hypoglycemia, and management should include education, medical nutrition therapy with a registered dietitian nutritionist experienced in post-metabolic surgery hypoglycemia, and medication treatment, as needed. **A** In individuals with post-metabolic surgery hypoglycemia, use continuous glucose monitoring to improve safety. **C**

8.27 In people who undergo metabolic surgery, routinely screen for psychosocial and behavioral health changes and refer to a qualified behavioral health professional as needed. **C**

8.28 Monitor individuals who have undergone metabolic surgery for insufficient weight loss or weight recurrence at least every 6–12 months. **E** In those who have insufficient weight loss or experience weight recurrence, assess for potential predisposing factors and, if appropriate, consider additional weight loss interventions (e.g., obesity pharmacotherapy). **C**

Treatment of Obesity in Type 1 Diabetes

8.29 Apply obesity management strategies used in the general adult population, including GLP-1 RA-based therapy **B** and metabolic surgery, **C** to adults with type 1 diabetes who have obesity (BMI ≥30.0 kg/m², or ≥27.5 kg/m² in Asian American individuals). Shared decision-making should inform individualized care.

Table 8.1—Obesity pharmacotherapy in individuals with type 2 diabetes

Medication name	Treatment arm; weight loss from baseline	Time frame for weight loss (weeks)*	Common side effects	Possible safety concerns and considerations
Sympathomimetic amine anorectic: approved for short-term use only Phentermine (183,184)†	• 15 mg q.d.; 7.4% • 7.5 mg q.d.; 6.6% • Placebo; 2.3%	28	Dry mouth, insomnia, dizziness, irritability, increased blood pressure, elevated heart rate	• Contraindicated for use in combination with monoamine oxidase inhibitors • Contraindicated with a history of cardiovascular disease • Do not use if at high risk for glaucoma due to risk of acute angle-closure glaucoma
Lipase inhibitor Orlistat (4,185)‡	• 120 mg t.i.d.; 9.6% • Placebo; 5.6%	52	Abdominal pain, flatulence, fecal urgency	• Contraindicated in cholestasis • Potential malabsorption of fat-soluble vitamins (A, D, E, K) and of certain medications (e.g., cyclosporine, thyroid hormone, anticonvulsants) • Rare cases of severe liver injury reported • Cholelithiasis reported • Nephrolithiasis reported. Monitor renal function and discontinue if oxalate nephropathy occurs
Sympathomimetic amine anorectic/ antiepileptic combination Phentermine/topiramate ER (54,116)§	• 15 mg/92 mg q.d.; 9.8% • 7.5 mg/46 mg q.d.; 7.8% • Placebo; 1.2%	56	Constipation, paresthesia, insomnia, nasopharyngitis, xerostomia, increased blood pressure, nephrolithiasis	• Contraindicated for use in combination with monoamine oxidase inhibitors • Contraindicated during pregnancy due to risk of fetal harm with topiramate • Cognitive impairment associated with rapid dose titration or high initial doses • Caution with cardiovascular disease • Do not use if at high risk for glaucoma due to risk of acute angle-closure glaucoma
Opioid antagonist/antidepressant combination Naltrexone/bupropion ER (13,186)	• 16 mg/180 mg b.i.d.; 5% • Placebo; 1.8%	56	Constipation, nausea, headache, xerostomia, insomnia, elevated heart rate and blood pressure	• Contraindicated in people with unmanaged hypertension and/or seizure disorders • Contraindicated for use with chronic opioid therapy • Acute angle-closure glaucoma may occur • Increased blood pressure and heart rate may occur; monitor in people with cardiovascular and cerebrovascular disease **Boxed warning:** • Risk of suicidal behavior/ideation in people younger than 24 years old who have depression
GLP-1 receptor agonist Liraglutide (14,55,187)‖	• 3.0 mg q.d.; 6% • 1.8 mg q.d.; 4.7% • Placebo; 2%	56	Gastrointestinal side effects (nausea, vomiting, diarrhea, esophageal reflux, constipation)	The following apply to both GLP-1 receptor agonists: • Provide guidance on discontinuation prior to surgical procedures to mitigate potential for pulmonary aspiration with general anesthesia or deep sedation

Table 8.1
Obesity and Weight Management for the Prevention and Treatment of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S166-S182

Table 8.1—Continued

Medication name	Treatment arm; weight loss from baseline	Time frame for weight loss (weeks)*	Common side effects	Possible safety concerns and considerations
Semaglutide (54,117,188)	• 2.4 mg weekly; 9.6% • 1.0 mg weekly; 7% • Placebo; 3.4%			• Pancreatitis: acute pancreatitis has been reported, but causality has not been established. Do not initiate if at high risk for pancreatitis and discontinue if pancreatitis is suspected • Biliary disease: evaluate for gallbladder disease if cholelithiasis or cholecystitis is suspected; avoid use in at-risk individuals • Gastrointestinal disorders (severe constipation and small-bowel obstruction/ileus progression) • Diabetic retinopathy: close monitoring of retinopathy in those at high risk (older individuals and those with longer duration of type 2 diabetes [≥10 years]) • Nonarteritic anterior ischemic optic neuropathy reported; rare incidence. Monitor for this during eye examinations • Impact on drug absorption: orally administered drug absorption may be impaired during dose titration (including oral contraceptives) • Gastrointestinal side effects: counsel on potential for gastrointestinal side effects; provide guidance on dietary modifications to mitigate gastrointestinal side effects (reduction in meal size, mindful eating practices [e.g., stop eating once full], decreasing intake of high-fat or spicy food); consider slower dose titration for those experiencing gastrointestinal challenges. Not recommended for individuals with gastroparesis • Hypoglycemia (with concomitant use of insulin or sulfonylurea) **Boxed warning:** • Risk of thyroid C-cell tumors in rodents; human relevance not determined; do not use in individuals with personal or family history of medullary thyroid cancer or multiple endocrine neoplasia type 2
Dual GIP and GLP-1 receptor agonist				
Tirzepatide (109,189)	• 15 mg weekly; 14.7% • 10 mg weekly; 12.8% • Placebo; 3.2%	72	Gastrointestinal side effects (nausea, vomiting, diarrhea, esophageal reflux, constipation)	Same as for GLP-1 receptor agonists, with addition of the following: • Monitor effects of oral medications with narrow therapeutic index (warfarin) or whose efficacy is dependent on threshold concentration • Advise individuals using oral contraceptives to switch to a nonoral contraceptive method or add a barrier method of contraception for 4 weeks after initiation and for 4 weeks after each dose escalation

Select safety and side effect information is provided; for a comprehensive discussion of safety considerations, please refer to the prescribing information for each agent, b.i.d., twice daily; ER, extended release; GIP, glucose-dependent insulinotropic polypeptide; GLP-1, glucagon-like peptide 1; q.d., every day; Rx, prescription; t.i.d., three times daily, p.o., by mouth. *Time frames used in clinical trials. Medications approved for long-term use should be continued as indicated beyond reaching weight loss goals. †Phentermine was evaluated in a general adult population with obesity. As monotherapy, phentermine is only approved for short-term use. Use lowest effective dose; maximum appropriate dose is 37.5 mg. ‡Enrolled participants had normal (79%) or impaired (21%) glucose tolerance. §Maximum dose, depending on response, is 15 mg/92 mg q.d. Approximately 68% of enrolled participants had type 2 diabetes or impaired glucose tolerance. ||Agent has indication for reduction of cardiovascular events (55,117).

Table 8.1 (continued)
Obesity and Weight Management for the Prevention and Treatment of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S166-S182

Table 8.2—Median monthly (30-day) AWP and NADAC of maximum or maintenance dose of obesity pharmacotherapies

Medication name	Typical adult maintenance dose	AWP (median and range for 30-day supply)	NADAC (median and range for 30-day supply)
Sympathomimetic amine anorectic: approved for short-term use only			
Phentermine	8–37.5 mg daily	$43 ($3–$58)*	$2 ($2–$3)*
Lipase inhibitor			
Orlistat	60 mg t.i.d. (OTC)	$58 ($41–$90)	NA
	120 mg t.i.d. (Rx)	$675 ($520–$781)	$514 ($416–$611)
Sympathomimetic amine anorectic/ antiepileptic combination			
Phentermine/topiramate ER	7.5 mg/46 mg daily	$238 ($238–$251)	NA
Opioid antagonist/antidepressant combination			
Naltrexone/bupropion ER	16 mg/180 mg b.i.d.	$750	NA
GLP-1 receptor agonist			
Liraglutide	3 mg daily	$1,619	$1,303
Semaglutide	2.4 mg once weekly	$1,619	$1,302
Dual GIP and GLP-1 receptor agonist			
Tirzepatide	5, 10, or 15 mg once weekly	$1,304**	$1,022

The costs listed in this table are representative of costs at a national level. These costs may not be representative of an individual's cost and do not account for medication coverage or available discounts. AWP, average wholesale price; b.i.d., twice daily; ER, extended release; GIP, glucose-dependent insulinotropic polypeptide; GLP-1, glucagon-like peptide 1; NA, data not available; NADAC, National Average Drug Acquisition Cost; OTC, over the counter; Rx, prescription; t.i.d., three times daily. AWP and NADAC prices are for a 30-day supply of maximum or maintenance dose as of 15 July 2025 (190,191). *Data are for 37.5 mg q.d. dose. **Pricing listed for tirzepatide is for the pens only, not the cost of vials available from the manufacturer.

Table 8.2
Obesity and Weight Management for the Prevention and Treatment of Diabetes:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S166-S182

Section 9.

Pharmacologic Approaches to Glycemic Treatment

(https://doi.org/10.2337/dc26-S009)

Pharmacologic Therapy for Adults with Type 1 Diabetes

9.1 Treat most adults with type 1 diabetes with continuous subcutaneous insulin infusion or multiple daily doses of prandial (injected or inhaled) and basal insulin. **A**

9.2 For most adults with type 1 diabetes, insulin analogs (or inhaled insulin) are preferred over injectable human insulins to minimize hypoglycemia risk. **A**

9.3 To improve glycemic outcomes and quality of life and to minimize hypoglycemia risk, most adults with type 1 diabetes should receive education on how to match mealtime insulin doses to carbohydrate intake and fat and protein intake depending on the person's or caregiver's needs or preferences. They should also be taught how to modify the insulin dose (correction dose) based on concurrent glycemia, glycemic trends (if available), sick-day management, and anticipated physical activity. **B**

9.4 Insulin treatment plans and insulin-taking behaviors should be reevaluated at regular intervals (e.g., every 3–6 months) and adjusted to incorporate specific factors that affect choice of treatment and ensure achievement of individualized glycemic goals. **E**

Representative relative attributes of insulin delivery approaches in people with type 1 diabetes

Insulin plans	Greater flexibility	Lower risk of hypoglycemia	Higher costs
MDI with LAA + RAA or URAA	+++	+++	$$$
Less-preferred, alternative injected insulin plans			
MDI with NPH + RAA or URAA	++	++	$$
MDI with NPH + short-acting (regular) insulin	++	+	$
Two daily injections with NPH + short-acting (regular) insulin or premixed	+	+	$

Continuous insulin infusion plans	Greater flexibility	Lower risk of hypoglycemia	Higher costs
Automated insulin delivery systems	+++++	+++++	$$$$$
Insulin pump with threshold/ predictive low-glucose suspend	++++	++++	$$$$$
Insulin pump therapy without automation	+++	+++	$$$$

Figure 9.1—Choices of insulin plans in people with type 1 diabetes. Continuous glucose monitoring improves outcomes with injected or infused insulin and is superior to blood glucose monitoring. Inhaled insulin may be used in place of injectable prandial insulin in the U.S. The number of plus or dollar signs is an estimate of relative association of the plan with greater flexibility, lower risk of hypoglycemia, and higher costs between the different plans. Cost symbols are reflective of general costs, which may vary for individuals based on various circumstances: insurance coverage, discounts, rebates, and other price adjustments involved in prescription sales. LAA, long-acting insulin analog; MDI, multiple daily injections; RAA, rapid-acting insulin analog; URAA, injectable ultra-rapid-acting insulin analog or inhaled insulin. Adapted from Holt et al. (4).

Figure 9.1
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

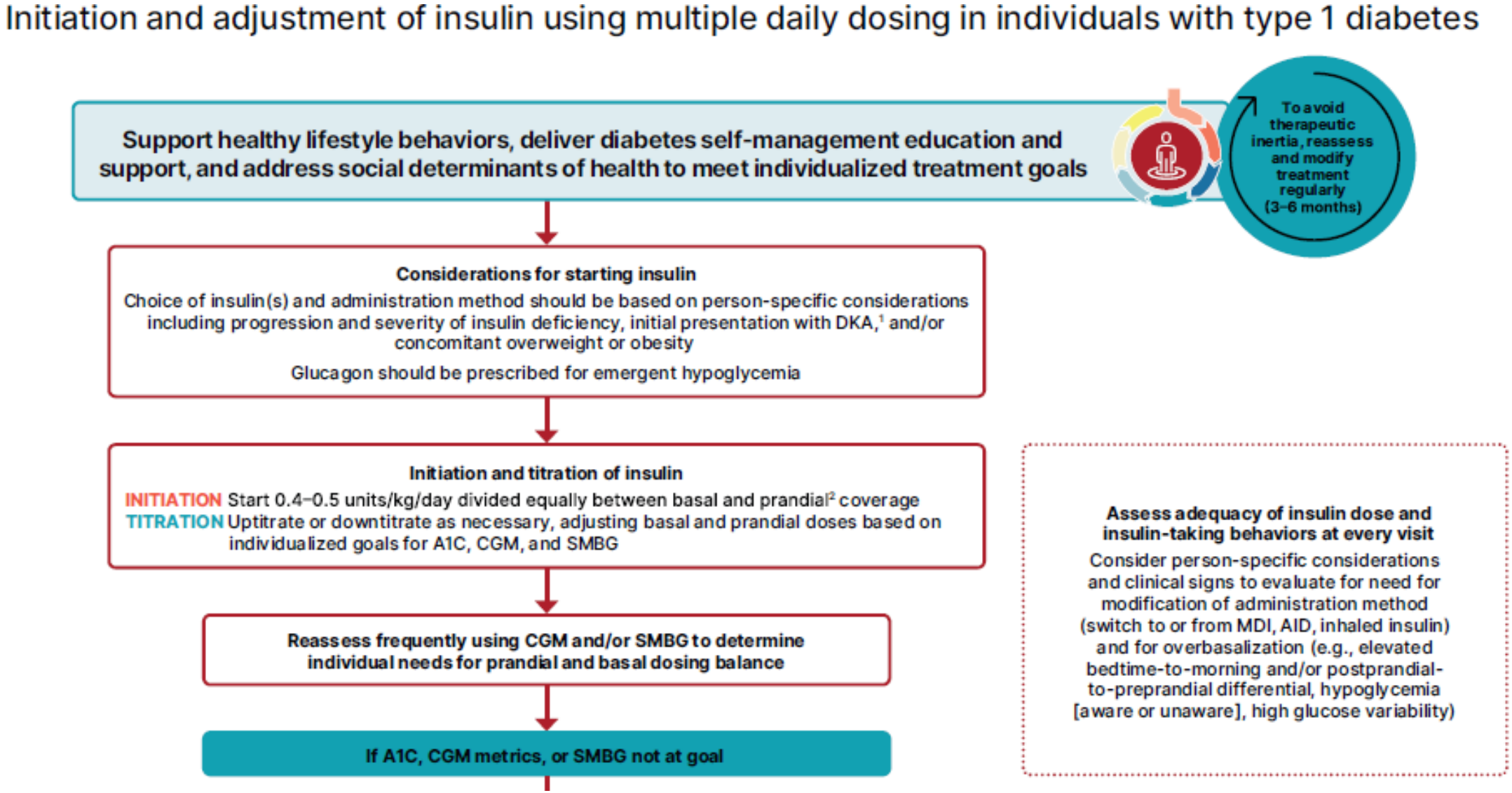
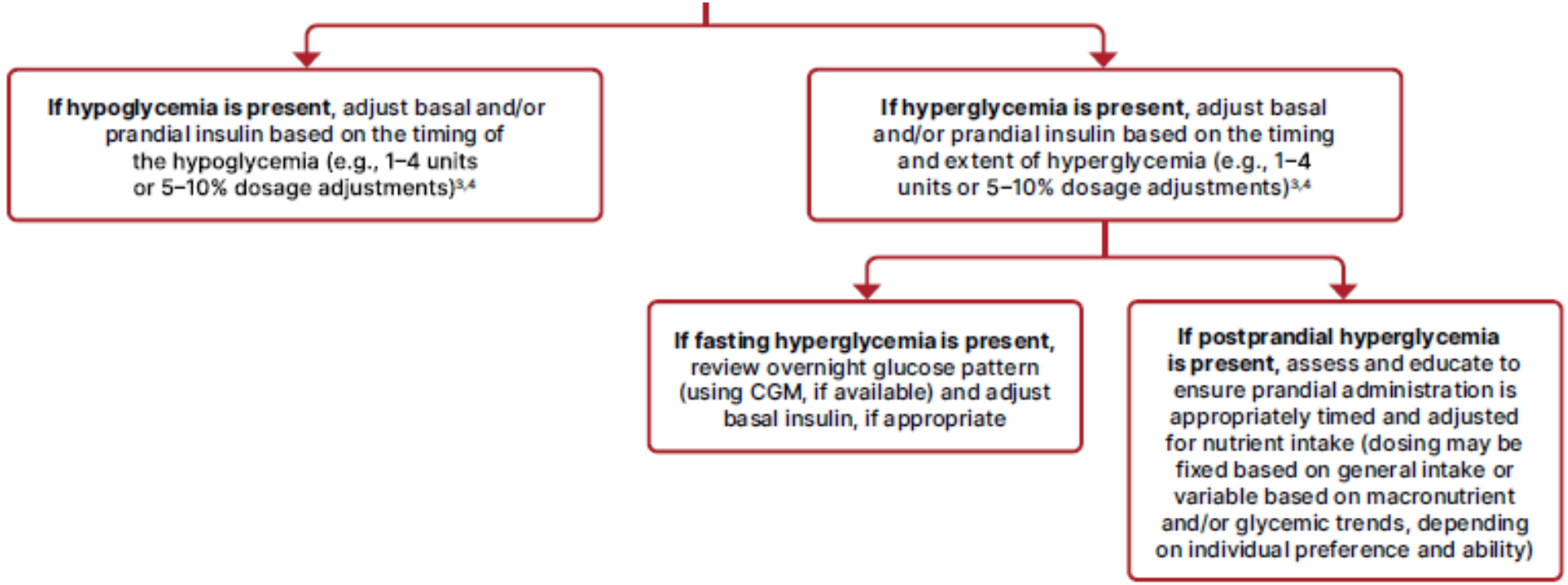


Figure 9.2
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215



1. Refer to section 16, "Diabetes Care in the Hospital," for information on care during hospitalizations.
2. Prandial insulin options include: injectable rapid-acting and ultra-rapid-acting analog insulins, injectable short-acting human insulin, and inhaled human insulin.
3. Amount of dosage adjustment may vary between individuals depending on their severity of insulin deficiency and/or insulin resistance. Some individuals may require adjustments of 10–20%.
4. Adjustment may be done by their diabetes care team or by individuals, with guidance provided by their diabetes care team, as frequently as once to twice weekly using the lowest levels or average of the previous 3–4 days.

Figure 9.2—Insulin initiation and adjustment for people with type 1 diabetes using multiple daily dosing. AID, automated insulin delivery; CGM, continuous glucose monitoring; DKA, diabetic ketoacidosis; MDI, multiple daily injections; SMBG, self-monitoring of blood glucose.

Figure 9.2 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

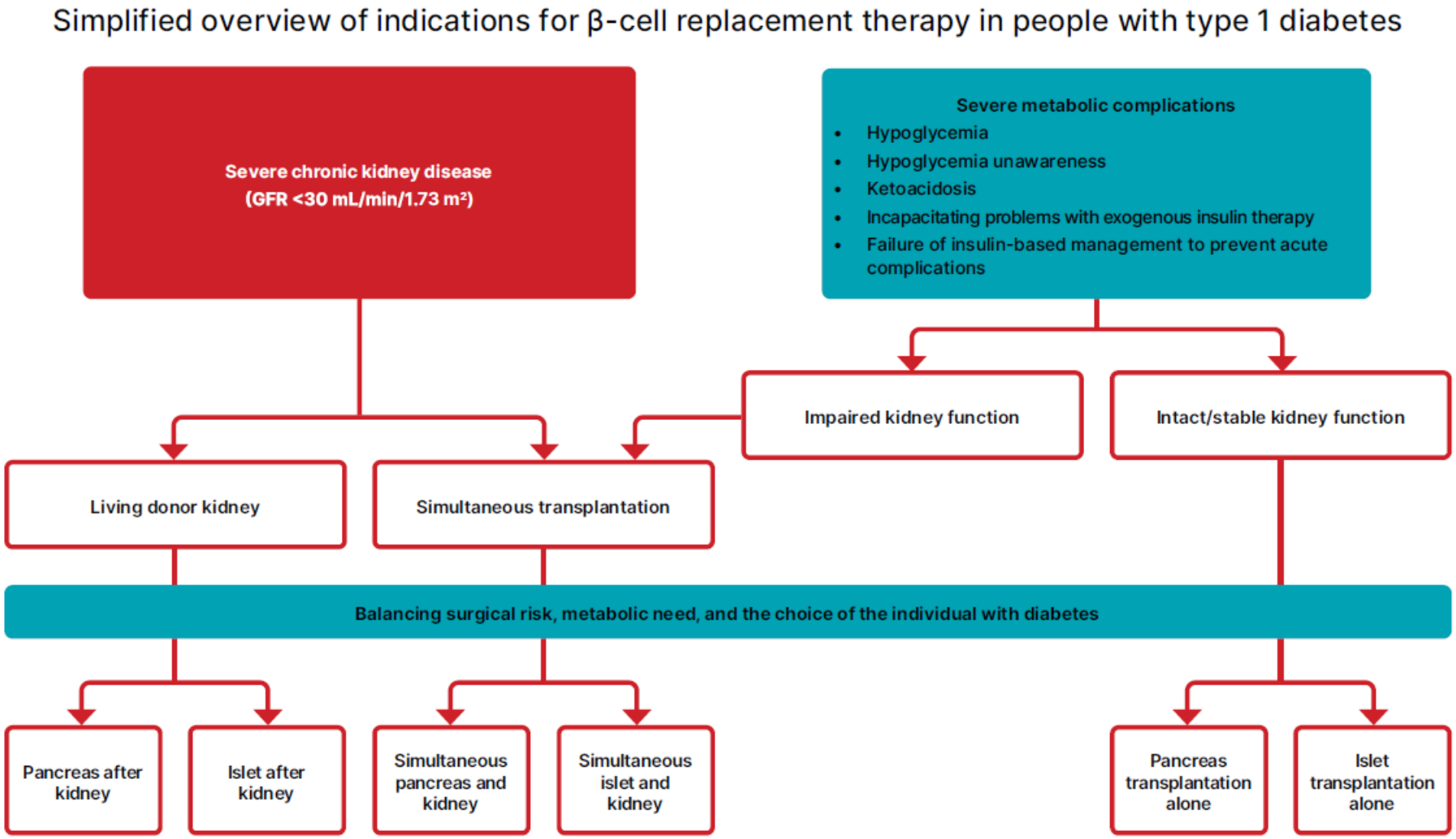


Figure 9.3—Simplified overview of indications for β-cell replacement therapy in people with type 1 diabetes. The two main forms of β-cell replacement therapy are whole-pancreas transplantation and islet cell transplantation. β-Cell replacement therapy can be combined with kidney transplantation if the individual has kidney failure, which may be performed simultaneously or after kidney transplantation. All decisions about transplantation must consider the surgical risk, metabolic need, and the choices of the individual with diabetes. GFR, glomerular filtration rate. Adapted from Holt et al. (4).

Figure 9.3
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.1—Examples of subcutaneous insulin treatment plans

Plans	Timing and distribution	Advantages	Disadvantages	Adjusting doses
Plans that more closely mimic normal insulin secretion				
Insulin pump therapy, including conventional CSII, sensor-augmented pump, low-glucose suspend; AID systems such as hybrid closed loop	Basal delivery of URAA or RAA; generally 30–50% of TDD. Mealtime and correction: URAA or RAA by bolus based on ICR and/or ISF and target glucose, with premeal insulin ~15 min before eating.	Can adjust basal rates for varying insulin sensitivity by time of day, for exercise, and for sick days. Flexibility in meal timing and content. Pump can deliver insulin in increments of fractions of units. Potential for integration with CGM for AID systems. TIR % highest and TBR % lowest with: hybrid closed-loop > low-glucose suspend > sensor-augmented open-loop > conventional CSII.	Most expensive plan. Must continuously wear one or more devices. Risk of rapid development of ketosis or DKA with interruption of insulin delivery. Potential reactions to adhesives and site infections. Most technically complex approach (harder for people with lower numeracy or literacy skills).	Mealtime insulin: if carbohydrate counting is accurate, change ICR if glucose after meal consistently out of target. Correction insulin: adjust ISF and/or target glucose if correction does not consistently bring glucose into range. Basal rates: adjust based on overnight, fasting or daytime glucose outside of activity of URAA/RAA bolus. AID systems: carbohydrate ratio, insulin on board, targets, and/or ISF may be adjusted, depending on the system; make sure to review and adjust manual mode settings, if available.

Table 9.1
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

MDI: LAA + flexible doses of URAA or RAA at meals	LAA once daily (insulin detemir or insulin glargine may require twice-daily dosing); generally 30–50% of TDD. Mealtime and correction: URAA or RAA based on ICR and/or ISF and target glucose.	Can use pens for all components. Flexibility in meal timing and content. Insulin analogs cause less hypoglycemia than human insulins.	At least four daily injections. Most costly insulins. Smallest increment of insulin is 1 unit (0.5 unit with some pens). LAAs may not cover strong dawn phenomenon (rise in glucose in early morning hours) as well as pump therapy.	Mealtime insulin: if carbohydrate counting is accurate, change ICR if glucose after meal consistently out of target. Correction insulin: adjust ISF and/or target glucose if correction does not consistently bring glucose into range. LAA: based on overnight or fasting glucose or daytime glucose outside of activity time course, or URAA or RAA injections.
MDI plans with less flexibility				
Four injections daily with fixed doses of N and RAA	Pre-breakfast: RAA ~20% of TDD. Pre-lunch: RAA ~10% of TDD. Pre-dinner: RAA ~10% of TDD. Bedtime: N ~50% of TDD.	May be feasible if unable to carbohydrate count. All meals have RAA coverage. N is less expensive than LAAs.	Shorter duration RAA may lead to basal deficit during day; may need twice-daily N. Greater risk of nocturnal hypoglycemia with N. Requires relatively consistent mealtimes and carbohydrate intake.	Pre-breakfast RAA: based on BGM after breakfast or before lunch. Pre-lunch RAA: based on BGM after lunch or before dinner. Pre-dinner RAA: based on BGM after dinner or at bedtime. Evening N: based on fasting or overnight BGM.

Continued on p. S186

Table 9.1 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.1—Continued

Plans	Timing and distribution	Advantages	Disadvantages	Adjusting doses
Four injections daily with fixed doses of N and R	Pre-breakfast: R ~20% of TDD. Pre-lunch: R ~10% of TDD. Pre-dinner: R ~10% of TDD. Bedtime: N ~50% of TDD.	May be feasible if unable to carbohydrate count. R can be dosed based on ICR and correction. All meals have R coverage. Least expensive insulins.	Greater risk of nocturnal hypoglycemia with N. Greater risk of delayed post-meal hypoglycemia with R. Requires relatively consistent mealtimes and carbohydrate intake. R must be injected at least 30 min before meal for better effect.	Pre-breakfast R: based on BGM after breakfast or before lunch. Pre-lunch R: based on BGM after lunch or before dinner. Pre-dinner R: based on BGM after dinner or at bedtime. Evening N: based on fasting or overnight BGM.
Plans with fewer daily injections				
Three injections daily: N + R or N + RAA	Pre-breakfast: N ~40% TDD + R or RAA ~15% TDD. Pre-dinner: R or RAA ~15% TDD. Bedtime: N ~30% TDD.	Morning insulins can be mixed in one syringe. May be appropriate for those who cannot take injection in middle of day. Morning N covers lunch to some extent. Same advantages of RAAs over R. Least (N + R) or less expensive insulins than MDI with analogs.	Greater risk of nocturnal hypoglycemia with N than LAAs. Greater risk of delayed post-meal hypoglycemia with R than RAAs. Requires relatively consistent mealtimes and carbohydrate intake. Coverage of post-lunch glucose often suboptimal. R must be injected at least 30 min before meal for better effect.	Morning N: based on pre-dinner BGM. Morning R: based on pre-lunch BGM. Morning RAA: based on post-breakfast or pre-lunch BGM. Pre-dinner R: based on bedtime BGM. Pre-dinner RAA: based on post-dinner or bedtime BGM. Evening N: based on fasting BGM.
Twice-daily "split-mixed": N + R or N + RAA	Pre-breakfast: N ~40% TDD + R or RAA ~15% TDD. Pre-dinner: N ~30% TDD + R or RAA ~15% TDD.	Least number of injections for people with strong preference for this. Insulins can be mixed in one syringe. Least (N + R) or less (N + RAA) expensive insulins vs. analogs. Eliminates need for doses during the day.	Risk of hypoglycemia in afternoon or middle of night from N. Fixed mealtimes and meal content. Coverage of post-lunch glucose often suboptimal. Difficult to reach targets for blood glucose without hypoglycemia.	Morning N: based on pre-dinner BGM. Morning R: based on pre-lunch BGM. Morning RAA: based on post-breakfast or pre-lunch BGM. Evening R: based on bedtime BGM. Evening RAA: based on post-dinner or bedtime BGM. Evening N: based on fasting BGM.

AID, automated insulin delivery; BGM, blood glucose monitoring; CGM, continuous glucose monitoring; CSII, continuous subcutaneous insulin infusion; ICR, insulin-to-carbohydrate ratio; ISF, insulin sensitivity factor; LAA, long-acting analog; MDI, multiple daily injections; N, NPH insulin; R, short-acting (regular) insulin; RAA, rapid-acting analog; TBR, time below range; TDD, total daily insulin dose; TIR, time in range; URAA, ultra-rapid-acting analog (inhaled insulin may be considered if appropriate). Adapted from Holt et al. (4).

Table 9.1 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Pharmacologic Therapy for Adults with Type 2 Diabetes

9.5 A person-centered shared decision-making approach should guide the choice of glucose-lowering medications for adults with type 2 diabetes. Use medications that provide sufficient effectiveness to achieve and maintain intended treatment goals with consideration of the effects on cardiovascular, kidney, weight, and other relevant comorbidities; hypoglycemia risk; cost and access; risk for adverse reactions and tolerability; and individual preferences (**Fig. 9.4** and **Table 9.2**). **E**

9.6 Consider combination therapy in adults with type 2 diabetes for initial treatment to shorten time to attainment of individualized glycemic goals. **A**

9.7 In adults with type 2 diabetes and established or high risk of atherosclerotic cardiovascular disease, the treatment plan should include medications with demonstrated benefits to reduce cardiovascular events (e.g., glucagon-like peptide 1 receptor agonist [GLP-1 RA] and/or sodium–glucose cotransporter 2 [SGLT2] inhibitor) for glycemic management and comprehensive cardiovascular risk reduction (irrespective of A1C) (**Fig. 9.4** and **Table 9.2**). **A**

Pharmacologic Therapy for Adults with Type 2 Diabetes (continued)

9.8 In adults with type 2 diabetes who have heart failure (HF) (with either reduced or preserved ejection fraction), an SGLT2 inhibitor is recommended for both glycemic management and prevention of HF hospitalizations (irrespective of A1C) (**Fig. 9.4**). **A**

9.9a In adults with type 2 diabetes, obesity, and symptomatic heart failure with preserved ejection fraction (HFpEF), the glucose-lowering treatment plan should include a dual glucose-dependent insulinotropic polypeptide (GIP) and GLP-1 RA with demonstrated benefits for HF-related symptoms and reduction in HF events (irrespective of A1C). **A**

9.9b In adults with type 2 diabetes, obesity, and symptomatic HFpEF, the glucose-lowering treatment plan should include a GLP-1 RA with demonstrated benefits for HF-related symptoms **A** and/or reduction in HF events (irrespective of A1C). **B**

9.10 In adults with type 2 diabetes who have chronic kidney disease (CKD) (with confirmed estimated glomerular filtration rate [eGFR] 20–60 mL/min/1.73 m^2 and/or albuminuria), an SGLT2 inhibitor or GLP-1 RA with demonstrated benefit in this population should be used for both glycemic management and for slowing progression of CKD and reduction in cardiovascular events (irrespective of A1C) (**Fig. 9.4**). The glycemic benefits of SGLT2 inhibitors are reduced at eGFR <45 mL/min/1.73 m^2. **A**

Pharmacologic Therapy for Adults with Type 2 Diabetes (continued)

9.11 In adults with type 2 diabetes and advanced CKD (eGFR <30 mL/min/1.73 m^2), a GLP-1 RA is preferred for glycemic management due to lower risk of hypoglycemia and for cardiovascular event reduction. **B** Individuals on dialysis can be safely initiated or continued on GLP-1–based therapy (that is not dependent on kidney clearance) to reduce cardiovascular risk and mortality. **C**

9.12 In adults with type 2 diabetes, metabolic dysfunction–associated steatotic liver disease (MASLD), and overweight or obesity, consider using a GLP-1 RA with demonstrated benefits in metabolic dysfunction–associated steatohepatitis (MASH) **A** or a dual GIP and GLP-1 RA with potential benefits in MASH **B** for glycemic management and as an adjunctive therapy to interventions for weight loss.

9.13a In adults with type 2 diabetes and biopsy-proven MASH or those at high risk for liver fibrosis (based on noninvasive tests), a GLP-1 RA is preferred for glycemic management due to beneficial effects on MASH. **A** Pioglitazone or a dual GIP and GLP-1 RA **B** can be considered for glycemic management due to potential beneficial effects on MASH.

9.13b Combination therapy with pioglitazone plus a GLP-1 RA can be considered for the treatment of hyperglycemia in adults with type 2 diabetes with biopsy-proven MASH or those at high risk of liver fibrosis (identified with noninvasive tests) due to potential beneficial effects on MASH. **B**

Pharmacologic Therapy for Adults with Type 2 Diabetes (continued)

9.14 Medication plan and medication-taking behavior should be reevaluated at regular intervals (e.g., every 3–6 months) and adjusted as needed to incorporate specific factors that affect choice of treatment and ensure achievement of individualized glycemic goals (**Fig. 4.1** and **Table 9.2**). **E**

9.15 Treatment modification (including intensification or deintensification) for adults not meeting individualized treatment goals should not be delayed. **A**

9.16 Choice of glucose-lowering therapy modification should take into consideration individualized glycemic and weight goals, presence of comorbidities (cardiovascular, kidney, liver, and other metabolic comorbidities), and the risk of hypoglycemia. **A**

9.17 When initiating a new glucose-lowering medication, reassess the need for and/or dose of medications with higher hypoglycemia risk (i.e., sulfonylureas, meglitinides, and insulin) to minimize the risk of hypoglycemia and treatment burden. **A**

Pharmacologic Therapy for Adults with Type 2 Diabetes (continued)

9.18 Concurrent use of dipeptidyl peptidase 4 (DPP-4) inhibitors with a GLP-1 RA or a dual GIP and GLP-1 RA is not recommended due to lack of additional glucose lowering beyond that of a GLP-1–based therapy. **B**

9.19 In adults with type 2 diabetes who have not achieved their individualized weight goals, additional weight management interventions (e.g., intensification of lifestyle modifications, structured weight management programs, pharmacologic agents, or metabolic surgery, as appropriate) are recommended. **A**

9.20 In adults with type 2 diabetes, initiation of insulin should be considered regardless of background glucose-lowering therapy or disease duration if symptoms of hyperglycemia are present or when A1C or blood glucose levels are very high (i.e., A1C >10% [>86 mmol/mol] or blood glucose ≥300 mg/dL [≥16.7 mmol/L]). **E**

9.21 In adults with type 2 diabetes without severe hyperglycemia or hyperglycemic crisis, GLP-1–based therapy is preferred to insulin for initial or add-on glucose-lowering therapy (**Fig. 9.4**). **B**

Pharmacologic Therapy for Adults with Type 2 Diabetes (continued)

9.22 If insulin is used, combination therapy with a GLP-1 RA, including a dual GIP and GLP-1 RA, is recommended for greater glycemic effectiveness as well as beneficial effects on weight and hypoglycemia risk for adults with type 2 diabetes. Insulin dosing should be reassessed upon addition or dose escalation of a GLP-1 RA or dual GIP and GLP-1 RA. **A**

9.23 In adults with type 2 diabetes who are initiating insulin therapy, continue glucose-lowering agents (unless contraindicated or not tolerated) for ongoing glycemic and metabolic benefits (i.e., weight, cardiometabolic, or kidney benefits). **A**

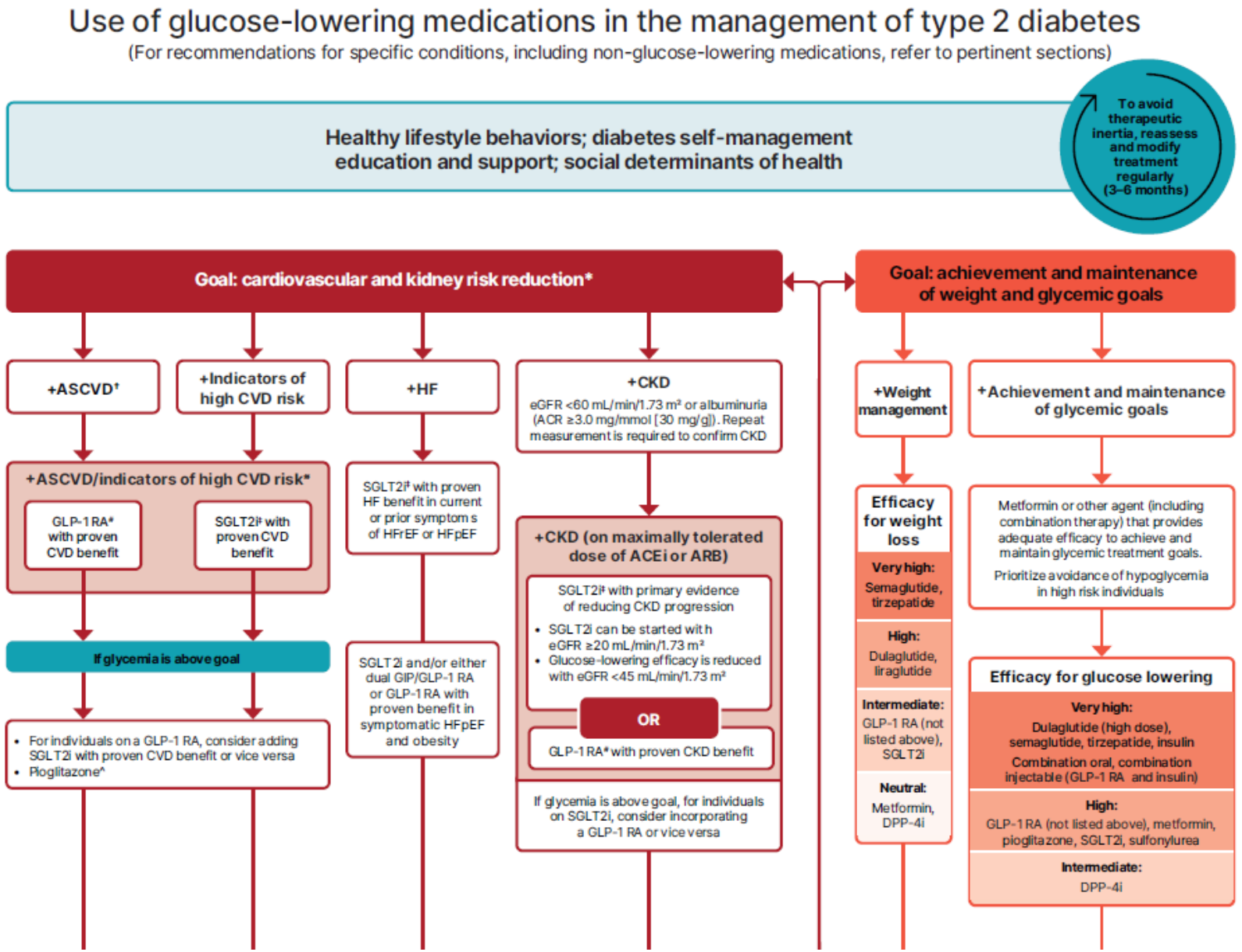
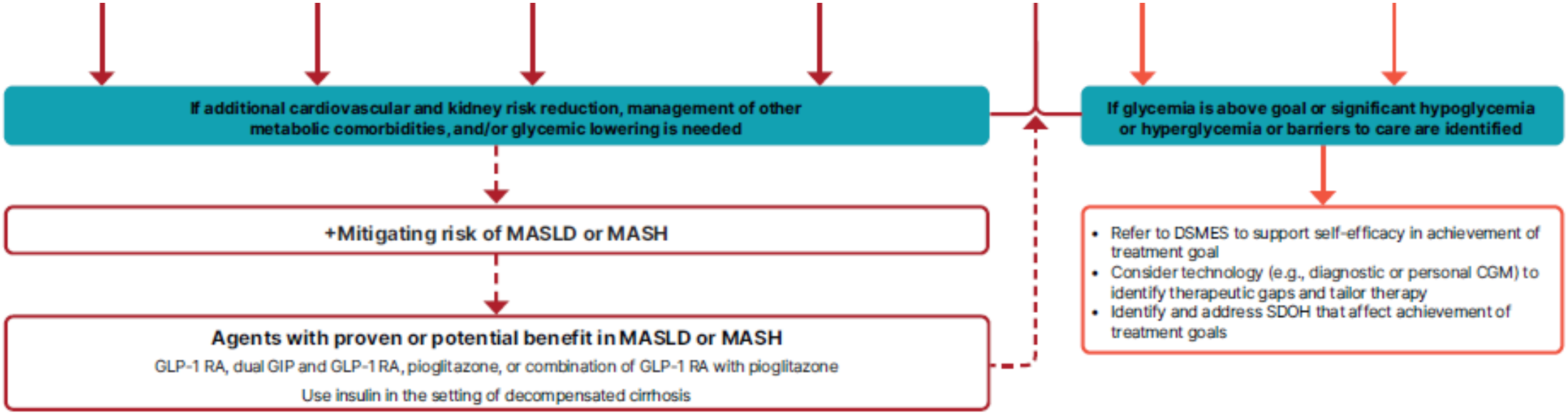


Figure 9.4
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215



* In people with HF, CKD, established CVD, or multiple risk factors for CVD, the decision to use a GLP-1 RA or SGLT2i with proven benefit should be made irrespective of attainment of glycemic goal.

† ASCVD: Defined differently across CVOTs but all included individuals with established CVD (e.g., MI, stroke, and arterial revascularization procedure) and variably included conditions such as transient ischemic attack, unstable angina, amputation, and symptomatic or asymptomatic coronary artery disease. Indicators of high risk: While definitions vary, most comprise ≥55 years of age with two or more additional risk factors (including obesity, hypertension, smoking, dyslipidemia, or albuminuria).

≈ A strong recommendation is warranted for people with CVD and a weaker recommendation for those with indicators of high risk CVD. Moreover, a higher absolute risk reduction and thus lower numbers needed to treat are seen at higher levels of baseline risk and should be factored into the shared decision-making process. See text for details.

For GLP-1 RAs, CVOTs demonstrate their efficacy in reducing composite MACE, CV death, all-cause mortality, MI, stroke, and kidney end points in individuals with T2D with established or high risk of CVD. One kidney outcome trial demonstrated benefit in reducing persistent eGFR reduction and CV death for a GLP-1 RA in individuals with CKD and T2D.

‡ For SGLT2is, CV and kidney outcomes trials demonstrate their efficacy in reducing the risks of composite MACE, CV death, all-cause mortality, MI, HHF, and kidney outcomes in individuals with T2D and established or high risk of CVD.

^ Low-dose pioglitazone may be better tolerated and similarly effective as higher doses.

Figure 9.4 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Figure 9.4—Use of glucose-lowering medications in the management of type 2 diabetes. The left side of the algorithm prioritizes mitigation of diabetes-related complications and end-organ effects, while the right side addresses weight and glucose management goals. ACEi, angiotensin-converting enzyme inhibitor; ACR, albumin-to-creatinine ratio; ARB, angiotensin receptor blocker; ASCVD, atherosclerotic cardiovascular disease; CGM, continuous glucose monitoring; CKD, chronic kidney disease; CV, cardiovascular; CVD, cardiovascular disease; CVOT, cardiovascular outcomes trial; DPP-4i, dipeptidyl peptidase 4 inhibitor; DSMES, diabetes self-management education and support; eGFR, estimated glomerular filtration rate; GLP-1 RA, glucagon-like peptide 1 receptor agonist; HF, heart failure; HFpEF, heart failure with preserved ejection fraction; HFrEF, heart failure with reduced ejection fraction; HHF, hospitalization for heart failure; MACE, major adverse cardiovascular events; MASH, metabolic dysfunction-associated steatohepatitis; MASLD, metabolic dysfunction-associated steatotic liver disease; MI, myocardial infarction; SDOH, social determinants of health; SGLT2i, sodium–glucose cotransporter 2 inhibitor; T2D, type 2 diabetes. Adapted from Davies et al. (90).

Figure 9.4 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.2—Features of medications for lowering glucose in type 2 diabetes

Medication (route of administration)	Glucose-lowering efficacy[1]	Hypoglycemia risk	Weight effects[2]	CV effects		Kidney effects		MASH effects	Clinical considerations and adverse effects
				Effect on MACE	Effect on HF	Progression of CKD	Dosing/use considerations*		
Metformin (oral)	High	No	Neutral (potential for modest loss)	Potential benefit	Neutral	Neutral	• Contraindicated with eGFR <30 mL/min/1.73 m^2	Neutral	• GI side effects: mitigate with slow dose titration, extended-release formulations, and administration with food • Potential for vitamin B12 deficiency: monitor and replete as appropriate
SGLT2 inhibitors (oral)	Intermediate to high	No	Loss (intermediate)	Benefit: canagliflozin, empagliflozin	Benefit: canagliflozin, dapagliflozin, empagliflozin, ertugliflozin	Benefit: canagliflozin, empagliflozin, dapagliflozin	• See labels of individual agents for dosage considerations for kidney function • Glucose-lowering effect is minimal at eGFR <45 mL/min/1.73 m^2 and lower; continue or start for cardiovascular and kidney benefit if eGFR >20 mL/min/1.73 m^2. May continue until dialysis or transplantation	Unknown	• DKA risk in individuals with insulin deficiency (rare in T2D): discontinue, evaluate, and treat promptly if suspected; be aware of predisposing risk factors and clinical presentations (including euglycemic DKA); mitigate risk with sick-day planning; discontinue before scheduled surgery (e.g., 3–4 days), during critical illness, or during prolonged fasting • Genital mycotic infections: mitigate risk with genital hygiene and avoid use in high-risk individuals • Urosepsis and pyelonephritis: evaluate individuals for signs and symptoms of urinary tract infections and treat promptly • Necrotizing fasciitis in the perineum (Fournier gangrene): rare; prompt treatment if suspected • Intravascular volume depletion: attention to volume status and blood pressure, particularly when ill or fasting; adjust other volume-contracting agents as applicable; monitor kidney function upon initiation

Continued on p. S194

Table 9.2
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.2—Continued

Medication (route of administration)	Glucose-lowering efficacy[1]	Hypoglycemia risk	Weight effects[2]	CV effects		Kidney effects		MASH effects	Clinical considerations and adverse effects
				Effect on MACE	Effect on HF	Progression of CKD	Dosing/use considerations*		
GLP-1 RAs (SQ; semaglutide also available in oral formulation)	High to very high	No	Loss (intermediate to very high)	Benefit: dulaglutide, liraglutide, semaglutide (SQ and oral) Neutral: exenatide once weekly, lixisenatide	Benefit: semaglutide (SQ)	Benefit for kidney end points in CVOTs, driven by albuminuria outcomes: dulaglutide, liraglutide, semaglutide (SQ) Demonstrated benefit for progression of CKD for semaglutide (SQ)	• See labels of individual agents for dosage considerations for kidney function • No dose adjustment for dulaglutide, liraglutide, or semaglutide • Monitor kidney function when initiating or escalating doses in individuals with kidney impairment reporting severe adverse GI reactions	Benefit: semaglutide (SQ)	• Thyroid C-cell tumors identified in rodents; human relevance not determined • Provide guidance on discontinuation prior to surgical procedures to mitigate potential for pulmonary aspiration with general anesthesia or deep sedation • Pancreatitis: acute pancreatitis has been reported, but causality has not been established; do not initiate if at high risk for pancreatitis, and discontinue if pancreatitis is suspected • Biliary disease: evaluate for gallbladder disease if cholelithiasis or cholecystitis is suspected; avoid use in at-risk individuals • Ileus: reported, but risk level is not well established • Diabetic retinopathy: close monitoring of retinopathy in those at high risk (older individuals and those with longer duration of T2D [≥10 years]) • Nonarteritic anterior ischemic optic neuropathy (NAION) reported (rare incidence); monitor for NAION during eye examinations • Impact on drug absorption: orally administered drug absorption may be impaired during dose titration (including oral contraceptives) • GI side effects: counsel on potential for GI side effects; provide guidance on dietary modifications to mitigate GI side effects (reduction in meal size, mindful eating practices [e.g., stop eating
Dual GIP and GLP-1 RA (SQ)	Very high	No	Loss (very high)	Under investigation	Benefit: tirzepatide	Potential benefit	• See labels of individual agents for dosage considerations for kidney function • No dose adjustment • Monitor kidney function when initiating or escalating doses in individuals with kidney impairment reporting severe adverse GI reactions	Potential benefit	

Table 9.2 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.2—Continued

Medication (route of administration)	Glucose-lowering efficacy[1]	Hypoglycemia risk	Weight effects[2]	CV effects		Kidney effects		MASH effects	Clinical considerations and adverse effects
				Effect on MACE	Effect on HF	Progression of CKD	Dosing/use considerations*		
									once full], decreasing intake of high-fat or spicy food); consider slower dose titration for those experiencing GI challenges; not recommended for individuals with gastroparesis
DPP-4 inhibitors (oral)	Intermediate	No	Neutral	Neutral	Neutral (potential risk: saxagliptin)	Neutral	• Dose adjustment required based on kidney function (sitagliptin, saxagliptin, alogliptin) • No dose adjustment required for linagliptin	Unknown	• Pancreatitis has been reported but causality has not been established; discontinue if pancreatitis is suspected • Postmarketing concerns about joint pain (consider discontinuing if debilitating and other treatment options are feasible) and bullous pemphigoid (discontinue if suspected) reversible upon discontinuation
Pioglitazone (oral)	High	No	Gain	Potential benefit	Increased risk	Neutral	• No dose adjustment required • Generally not recommended in kidney impairment due to potential for fluid retention	Potential benefit	• Increased risk of HF and fluid retention; do not use in setting of HF • Risk of bone fractures • Bladder cancer: do not use in individuals with active bladder cancer, and use caution in those with prior history of bladder cancer; association observed with higher cumulative exposure (e.g., longer duration, higher doses)
Sulfonylureas (2nd generation) (oral)	High	Yes	Gain	Neutral	Neutral	Neutral	• Glyburide: generally not recommended in CKD • Glipizide and glimepiride: initiate conservatively to avoid hypoglycemia	Unknown	• FDA Special Warning on increased risk of CV mortality based on studies of an older sulfonylurea (tolbutamide); glimepiride shown to be CV safe (see text) • Use with caution in individuals at risk for hypoglycemia, particularly if in combination with insulin

Table 9.2 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.2—Continued

Medication (route of administration)	Glucose-lowering efficacy[1]	Hypoglycemia risk	Weight effects[2]	CV effects		Kidney effects		MASH effects	Clinical considerations and adverse effects
				Effect on MACE	Effect on HF	Progression of CKD	Dosing/use considerations*		
Insulin (human) (SQ; regular insulin also available as inhaled formulation) Insulin (analogs) (SQ)	High to very high	Yes	Gain	Neutral	Neutral	Neutral	• Lower insulin doses required with a decrease in eGFR; titrate per clinical response	Unknown	• Injection site reactions • Higher risk of hypoglycemia with human insulin (NPH or premixed formulations) vs. analogs • Risk of hypoglycemia and duration of activity increases with severity of impaired kidney function • Refer to device-specific instructions for insulins compatible with different delivery systems (i.e., pumps, connected insulin pens, insulin patches)

CKD, chronic kidney disease; CV, cardiovascular; DKA, diabetic ketoacidosis; DPP-4, dipeptidyl peptidase 4; eGFR, estimated glomerular filtration rate; FDA, U.S. Food and Drug Administration; GI, gastrointestinal; GIP, glucose-dependent insulinotropic polypeptide; GLP-1 RA, glucagon-like peptide 1 receptor agonist; HF, heart failure; MACE, major adverse cardiovascular events; MASH, metabolic dysfunction-associated steatohepatitis; SGLT2, sodium–glucose cotransporter 2; SQ, subcutaneous; T2D, type 2 diabetes. *For agent-specific dosing recommendations, please refer to manufacturers' prescribing information. [1]Tsapas et al. (107). [2]Tsapas et al. (317). Adapted from Davies et al. (90).

Table 9.2 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

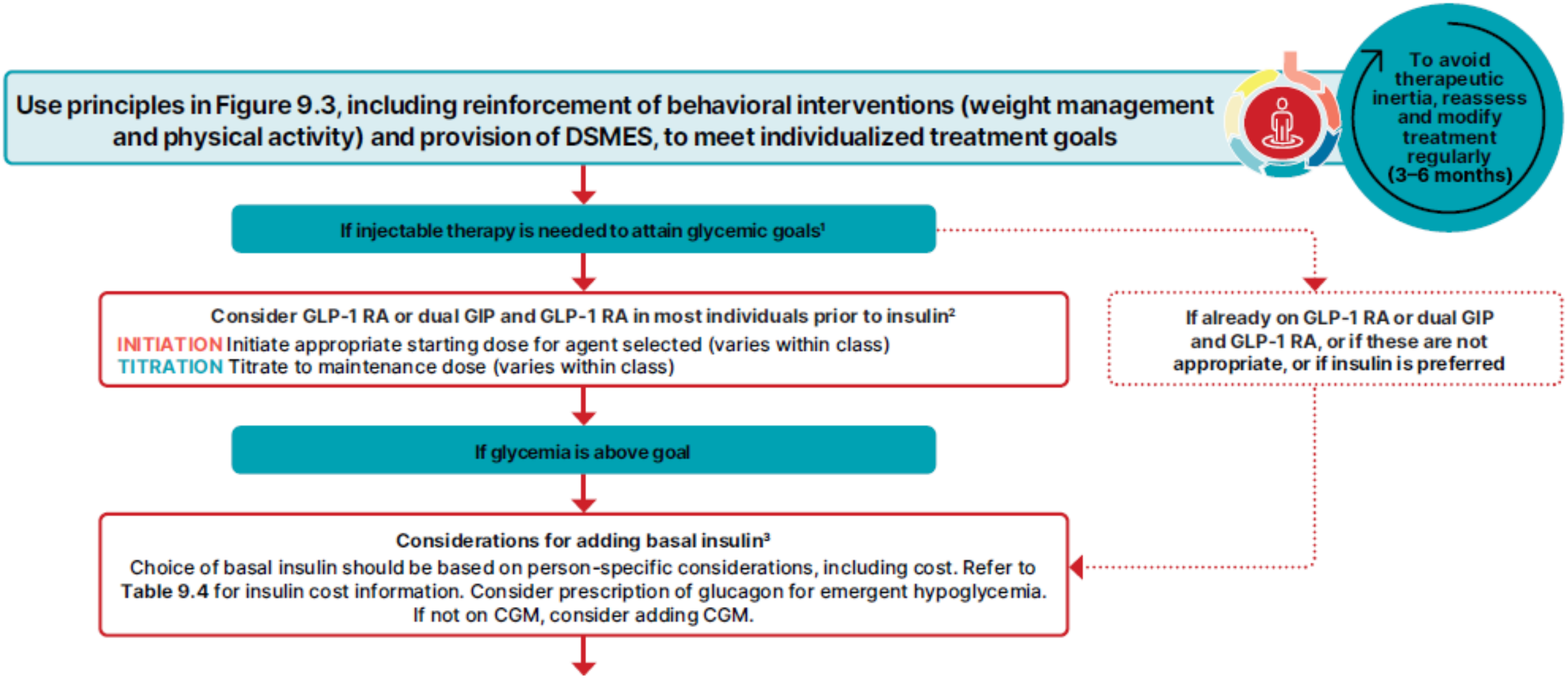


Figure 9.5
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

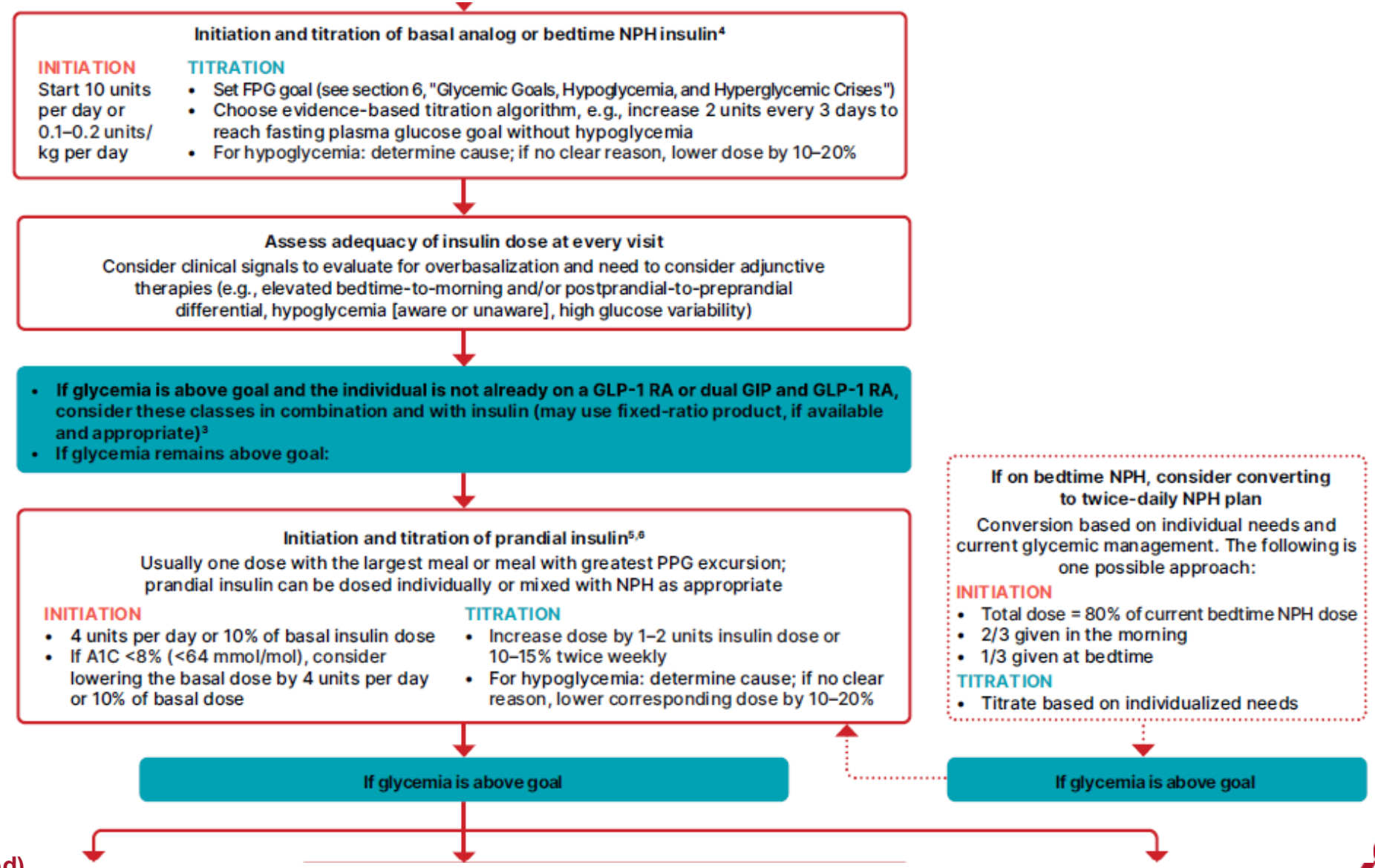
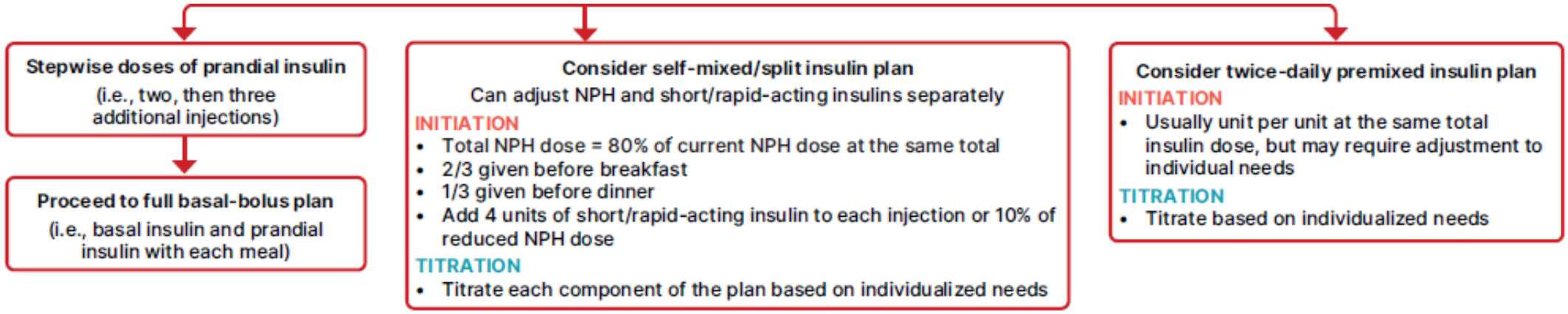


Figure 9.5 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215



1. Consider insulin as the first injectable if symptoms of hyperglycemia are present, when A1C or blood glucose levels are very high (i.e., A1C >10% [>86 mmol/mol] or blood glucose ≥300 mg/dL [≥16.7 mmol/L]), or when a diagnosis of type 1 diabetes is a possibility.
2. When selecting GLP-1 RAs, consider individual preference, glycemic lowering, weight-lowering effect, and frequency of injection. If CVD is present, consider GLP-1 RA with proven CVD benefit; oral or injectable GLP-1 RAs are appropriate.
3. For people on GLP-1 RA and basal insulin combination, consider use of a fixed-ratio combination product (IDegLira or iGlarLixi).
4. Consider switching from evening NPH to a basal analog if the individual develops hypoglycemia and/or frequently forgets to administer NPH in the evening and would be better managed with a morning dose of a long-acting basal Insulin. Consider dosing NPH in the morning for steroid-induced hyperglycemia.
5. Prandial insulin options include injectable rapid- and ultra-rapid-acting analog insulins, injectable short-acting human insulin, or inhaled human insulin.
6. If adding prandial insulin to NPH, consider initiation of a self-mixed or premixed insulin plan to decrease the number of injections required.

Figure 9.5—Intensifying to injectable therapies in type 2 diabetes. CGM, continuous glucose monitoring; DSMES, diabetes self-management education and support; FPG, fasting plasma glucose; GLP-1 RA, glucagon-like peptide 1 receptor agonist; GIP, glucose-dependent insulinotropic polypeptide; PPG, postprandial glucose. Adapted from Davies et al. (318).

Figure 9.5 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.3—Median monthly (30-day) AWP and NADAC of maximum approved daily dose of noninsulin glucose-lowering agents in the U.S.

Class	Compound	Dosage strength/ product (if applicable)	Maximum approved daily dose†	Median AWP (min, max)*	Median NADAC (min, max)*
Biguanides	• Metformin	500 mg (IR)	2,000 mg	$85 ($3, $216)	$2
		500 mg (ER)	2,000 mg	$89 ($5, $6,719)	$3
		850 mg (IR)	2,550 mg	$108 ($3, $189)	$2
		1,000 mg (IR)	2,000 mg	$87 ($2, $144)	$1
		1,000 mg (ER)	2,000 mg	$1,884 ($242, $7,214)	$26 ($24, $28)
		500 mg (Sol)	2,000 mg	$810 ($810, $1,478)	$417
Sulfonylureas (2nd generation)	• Glimepiride	4 mg	8 mg	$73 ($71, $198)	$2
	• Glipizide	10 mg (IR)	40 mg	$72 ($67, $91)	$6
		10 mg (XL/ER)	20 mg	$48	$9
	• Glyburide	6 mg (micronized)	12 mg	$54 ($48, $71)	$13
		5 mg	20 mg	$88 ($63, $432)	$7
Thiazolidinedione	• Pioglitazone	45 mg	45 mg	$348 ($7, $349)	$4
α-Glucosidase inhibitors	• Acarbose	100 mg	300 mg	$106 ($104, $378)	$22
	• Miglitol	100 mg	300 mg	$294 ($241, $346)	$183
Meglitinides	• Nateglinide	120 mg	360 mg	$104	$16
	• Repaglinide	2 mg	16 mg	$878 ($799, $1,728)	$28
DPP-4 inhibitors	• Alogliptin	25 mg	25 mg	$234	$143
	• Linagliptin	5 mg	5 mg	$630	$504
	• Saxagliptin	5 mg	5 mg	$524 ($523, $524)	$179
	• Sitagliptin	100 mg	100 mg	$341	$303
		25 mg/mL	100 mg	$354	NA
SGLT2 inhibitors	• Bexagliflozin	20 mg	20 mg	$47	NA
	• Canagliflozin	300 mg	300 mg	$718	$575
	• Dapagliflozin	10 mg	10 mg	$664	$345
	• Empagliflozin	25 mg	25 mg	$629	$604
	• Ertugliflozin	15 mg	15 mg	$428	$343

Table 9.3
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

GLP-1 RAs	• Dulaglutide	4.5 mg pen	4.5 mg‡	$1,185	$953
	• Liraglutide	18 mg/3 mL pen	1.8 mg	$929 ($845, $929)	$577
	• Semaglutide	2 mg pen	2 mg‡	$1,197	$966
		14 mg (tablet)	14 mg	$1,197	$965
Dual GIP and GLP-1 RA	• Tirzepatide	15 mg pen	15 mg‡	$1,296	$1,041
Bile acid sequestrant	• Colesevelam	625 mg tabs	3.75 g	$692 ($674, $712)	$56
		3.75 g suspension	3.75 g	$674 ($673, $675)	$93
Dopamine-2 agonist	• Bromocriptine	0.8 mg	4.8 mg	$1,188	$957

AWP, average wholesale price; DPP-4, dipeptidyl peptidase 4; ER and XL, extended release; GIP, glucose-dependent insulinotropic polypeptide; GLP-1 RA, glucagon-like peptide 1 receptor agonist; IR, immediate release; max, maximum; min, minimum; NA, data not available; NADAC, National Average Drug Acquisition Cost; SGLT2, sodium–glucose cotransporter 2; Sol, solution. AWP and NADAC prices as of 15 July 2025. *Calculated for 30-day supply (AWP [222] or NADAC [223] unit price × number of doses required to provide maximum approved daily dose × 30 days); median AWP or NADAC listed alone when only one product and/or price. †Used to calculate median AWP and NADAC (min, max); generic prices used, if available commercially. ‡Administered once weekly.

Table 9.3 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Table 9.4—Median cost of insulin products in the U.S. calculated as AWP and NADAC per 1,000 units of specified dosage form/product

Insulins	Compounds	Dosage form/product	Median AWP (min, max)*	Median NADAC (min, max)*
Rapid-acting	• Aspart	U-100 vial	$87†	$70†
		U-100 cartridge	$107†	$86†
		U-100 prefilled pen	$112†	$90†
	• Aspart biosimilars#	U-100 vial	$83	NA
		U-100 prefilled pen	$107	NA
	• Aspart ("faster acting product")	U-100 vial	$347	$278
		U-100 cartridge	$430	$344
		U-100 prefilled pen	$447	$358
	• Glulisine	U-100 vial	$102	$82
		U-100 prefilled pen	$132	$105
	• Inhaled insulin	Inhalation cartridges	$1,578	$1,265
	• Lispro	U-100 vial	$30†	$24†
		U-100 cartridge	$123	$98
		U-100 prefilled pen	$127†	$102†
		U-200 prefilled pen	$424	$339
	• Lispro-aabc	U-100 vial	$330	$263
		U-100 prefilled pen	$424	$339
		U-200 prefilled pen	$424	$339
	• Lispro follow-on product	U-100 vial	$118	$110
		U-100 prefilled pen	$151	$121
Short-acting	• Human regular	U-100 vial	$56 ($54, $58)‡	$44 ($43, $46)‡
		U-100 prefilled pen	$73	$58
Intermediate-acting	• Human NPH	U-100 vial	$58 ($54, $58)‡	$45 ($43, $46)‡
		U-100 prefilled pen	$93 ($73, $113)	$74 ($58, $91)

Table 9.4
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Concentrated human regular insulin	• U-500 human regular insulin	U-500 vial	$178	$143
		U-500 prefilled pen	$230	$183
Long-acting	• Degludec	U-100 vial	$142†	$114†
		U-100 prefilled pen	$142†	$114†
		U-200 prefilled pen	$142†	$114†
	• Glargine	U-100 vial	$77	$62
		U-100 prefilled pen	$77	$62
		U-300 prefilled pen	$152	$122†
	• Glargine biosimilar/follow-on products	U-100 vial	$76†	$61†
		U-100 prefilled pen	$74 ($74,† $261)	$59 ($59,† $209)
Premixed insulin products	• Aspart 70/30	U-100 vial	$87†‡	$69†‡
		U-100 prefilled pen	$112†‡	$90†‡
	• Lispro 50/50	U-100 vial	$102	NA
		U-100 prefilled pen	$127	$102
	• Lispro 75/25	U-100 vial	$102	$82
		U-100 prefilled pen	$127†	$102†
	• NPH/regular 70/30	U-100 vial	$56 ($54, $58)	$45 ($43, $46)
		U-100 prefilled pen	$93 ($73, $113)‡	$74 ($58, $90)‡
Premixed insulin/GLP-1 RA products	• Degludec/liraglutide	100/3.6 mg prefilled pen	$1,073	$859
	• Glargine/lixisenatide	100/33 mg prefilled pen	$713	$571

AWP, average wholesale price; GLP-1 RA, glucagon-like peptide 1 receptor agonist; NA, data not available; NADAC, National Average Drug Acquisition Cost. AWP (222) and NADAC (223) prices as of 15 July 2025. *AWP or NADAC calculated as in **Table 9.3**. †Unbranded product prices used when available. ‡AWP and NADAC data presented do not include human insulins (approximately $25/vial or $43/box of 5 pens) or select analog insulins (approximately $73/vial or $86/box of 5 pens) available at Walmart; median listed alone when only one product and/or price. #Pricing for aspart-xjhz not available on 15 July 2025.

Table 9.4 (continued)
Pharmacologic Approaches to Glycemic Treatment:
Standards of Care in Diabetes - 2026. Diabetes Care 2026;49(Suppl. 1):S183-215

Additional Recommendations For All Individuals with Diabetes

9.24 Include healthy behaviors, diabetes self-management education and support, avoidance of therapeutic inertia, and social determinants of health as essential components of the glucose-lowering management of diabetes. **A**

9.25 Use of continuous glucose monitoring (CGM) is recommended at diabetes onset and anytime thereafter for adults with diabetes who are on insulin therapy, **A** on noninsulin therapies that can cause hypoglycemia, **B** and on any diabetes treatment where CGM aids in management. **B** The choice of CGM device and method for use should be made based on the individual's circumstances, preferences, and needs. **B**

9.26 Monitor for signs of overbasalization during insulin therapy, such as significant bedtime-to-morning or postprandial-to-preprandial glucose differential, occurrences of hypoglycemia (aware or unaware), and high glycemic variability. When overbasalization is suspected, a thorough reevaluation should occur promptly to further tailor therapy to the individual's needs. **E**

9.27 Automated insulin delivery systems should be offered to all adults with type 1 and 2 diabetes on insulin depending on the person's or caregiver's needs and preferences. **A**

Additional Recommendations For All Individuals with Diabetes

9.28 Glucagon should be prescribed for all individuals taking insulin or at high risk for hypoglycemia. **A** Family, caregivers, school personnel, and others providing support to these individuals should know its location and be educated on how to administer it. Glucagon preparations that do not require reconstitution are preferred. **B**

9.29 Routinely assess all people with diabetes for financial obstacles that could impede their diabetes management. Clinicians, members of the diabetes care team, and social services professionals should work collaboratively, as appropriate and feasible, to support these individuals by implementing strategies to reduce costs, thereby improving their access to evidence-based care. **E**

9.30 In adults with diabetes and cost-related barriers, consider use of lower-cost medications for glycemic management (i.e., metformin, sulfonylureas, thiazolidinediones, and human insulin) within the context of their risks for hypoglycemia, weight gain, cardiovascular and kidney events, and other adverse effects. **E**

Special Circumstances and Populations

9.31a Use of compounded products that are not approved by the U.S. Food and Drug Administration (FDA) is not recommended due to uncertainty about their content and resulting concerns about safety, quality, and effectiveness. **C**

9.31b If a glucose-lowering medication is unavailable (e.g., in shortage), it is recommended to switch to a different FDA-approved medication with similar efficacy, as clinically appropriate. **E**

9.31c Upon resolution of the unavailability (e.g., shortage), reassess the appropriateness of resuming the original FDA-approved medication. **E**

Special Circumstances and Populations (continued)

9.32a Individuals of childbearing potential with diabetes should be counseled on contraception options **A** and the impact of some glucose-lowering medications on contraception efficacy. **C**

9.32b A person-centered shared decision-making approach to preconception planning is essential for all individuals of childbearing potential with diabetes. **A** Preconception planning should address attainment of glycemic goals, **A** the time frame for discontinuing noninsulin glucose-lowering medications, **E** and optimal glycemic management in preparation for pregnancy. **A**

9.33 Individuals who develop hyperglycemia during treatment with immunotherapy (including anti-PD-1 or anti-PD-L1 therapy, e.g., nivolumab, pembrolizumab, or avelumab) should be assessed for the immediate need for initiation of insulin therapy due to the potential risk of diabetic ketoacidosis while additional testing is completed to determine if the hyperglycemia is related to immunotherapy-associated diabetes. Close monitoring, education, and dose adjustment are needed if insulin is started. **C**

Special Circumstances and Populations (continued)

9.34 Consider metformin as the first-line treatment of hyperglycemia due to mTOR inhibitors. **E**

9.35a Consider metformin as the firstline treatment of hyperglycemia due to phosphoinositide 3-kinase (PI3K) inhibitors that affect the α isoform (e.g., alpelisib and inavolisib). **E**

9.35b Use of insulin should be reserved for severe hyperglycemia and hyperglycemic crises due to its potential impact on the efficacy of PI3K inhibitors. **E**

9.36 Adjust or initiate additional glucose-lowering therapies to maintain individualized glycemic goals based on the specific glucocorticoid treatment plan, with frequent reassessment of glucose levels and glucocorticoid treatment plans. **C**

Special Circumstances and Populations (continued)

9.37 In adults with posttransplantation diabetes mellitus (PTDM) or preexisting type 2 diabetes, insulin is preferred for the management of hyperglycemia in the postoperative setting. **A** A DPP-4 inhibitor can be considered for mild hyperglycemia. **E**

9.38a In adults with PTDM or preexisting type 2 diabetes, noninsulin pharmacotherapy can be used for long-term glycemic management, **C** and medication selection may differ depending on the transplanted organ(s). **E**

9.38b In adults with PTDM or preexisting type 2 diabetes, a GLP-1 RA can be considered for long-term glycemic management due to additional cardiometabolic benefits (e.g., cardiovascular, kidney, weight, and liver benefits). **C**

9.38c If long-term individualized glycemic goals cannot be achieved or maintained with noninsulin pharmacotherapy in adults with PTDM or preexisting type 2 diabetes, consider adding insulin. **C**

9.39 Educate individuals with diabetes who are at risk for developing diabetic ketoacidosis and who are treated with SGLT inhibition on the risks and signs of ketoacidosis and methods of risk mitigation management, provide them with appropriate tools for ketone measurement (i.e., serum β-hydroxybutyrate), and discourage a ketogenic eating pattern. **E**

Section 10.

Cardiovascular Disease and Risk Management

(https://doi.org/10.2337/dc26-S010)

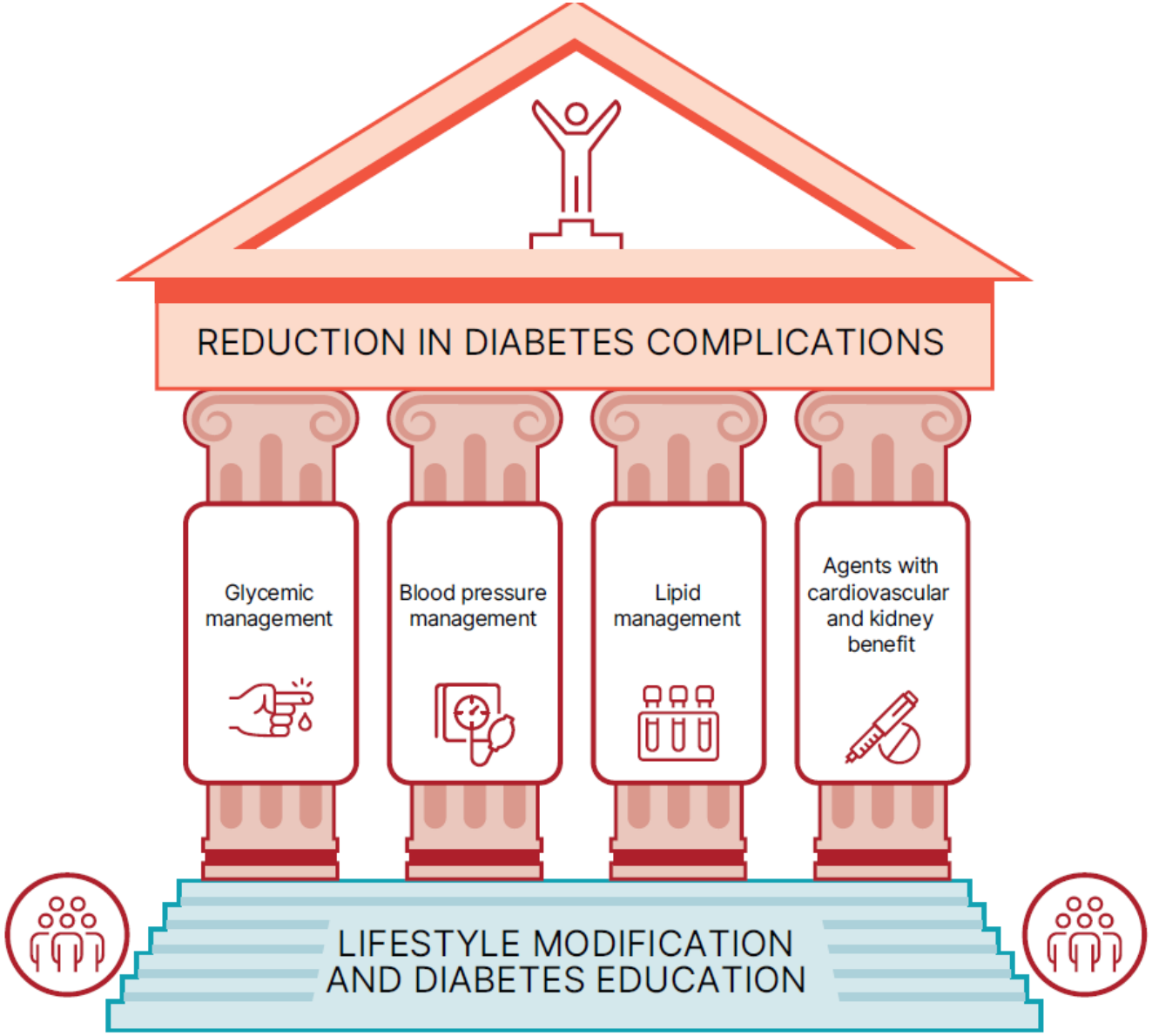


Figure 10.1—Multifactorial approach to reduction in risk of diabetes complications.

Figure 10.1
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

Hypertension and Blood Pressure Management – Screening and Diagnosis

10.1 Blood pressure should be measured at every routine clinical visit, or at least every 6 months. Individuals found to have elevated blood pressure without a diagnosis of hypertension (systolic blood pressure 120–129 mmHg and diastolic blood pressure <80 mmHg) should have blood pressure confirmed using multiple readings, including measurements on a separate day, to diagnose hypertension. **A** Hypertension is defined as a systolic blood pressure ≥130 mmHg or a diastolic blood pressure ≥80 mmHg based on an average of two or more measurements obtained on two or more occasions. **A** Individuals with blood pressure ≥180/110 mmHg and cardiovascular disease could be diagnosed with hypertension at a single visit. **E**

10.2 Counsel all people with hypertension and diabetes to monitor their blood pressure at home after appropriate education. **A**

Hypertension and Blood Pressure Management – Treatment Goals

10.3 For people with diabetes and hypertension, blood pressure goals should be individualized through a shared decision-making process that addresses cardiovascular risk, potential adverse effects of antihypertensive medications, and individual preferences. **B**

10.4 If it can be safely attained, the on-treatment blood pressure goal is <130/80 mmHg; a systolic blood pressure goal <120 mmHg should be encouraged in individuals with high cardiovascular or kidney risk. **A**

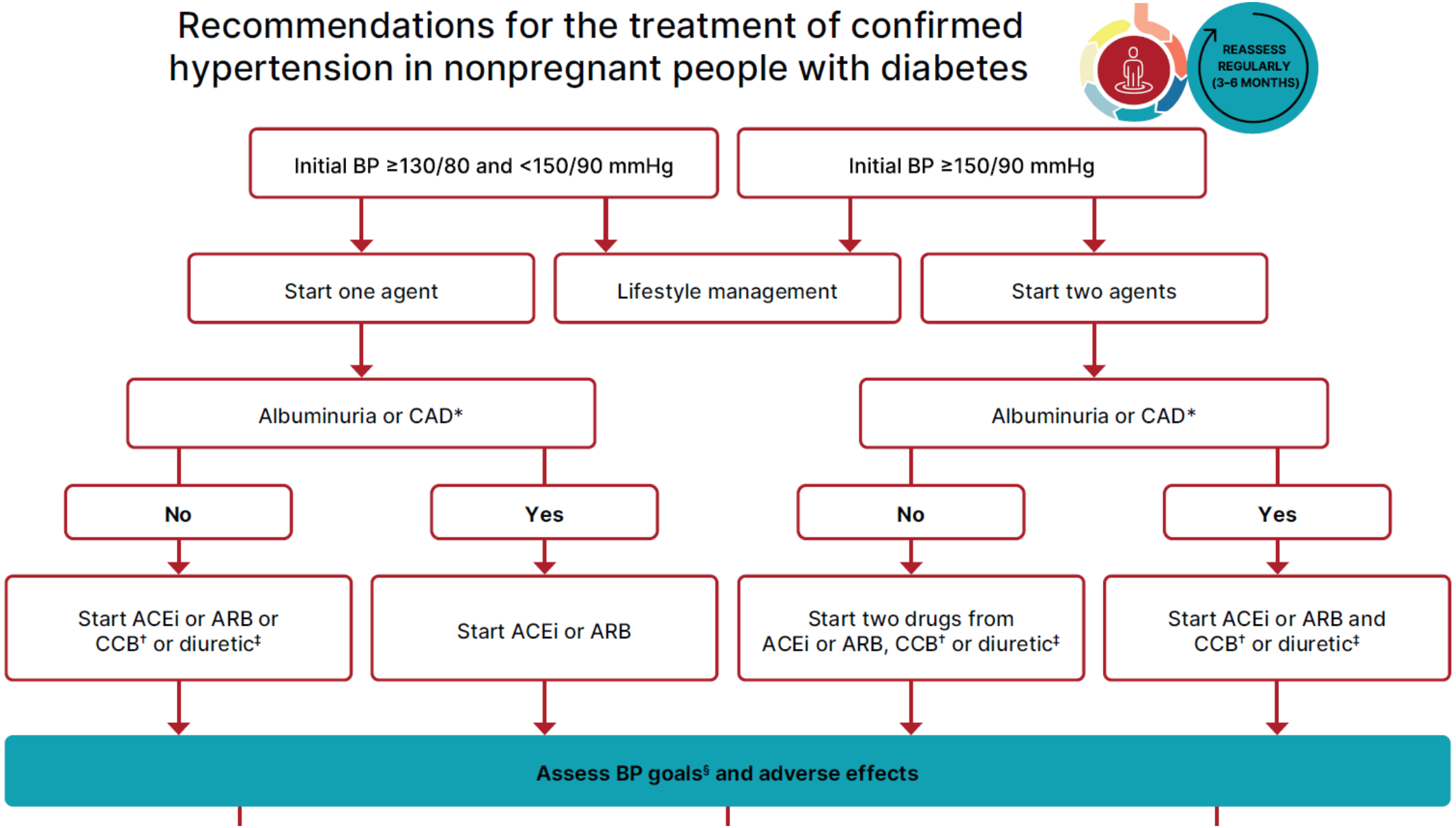


Figure 10.2
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

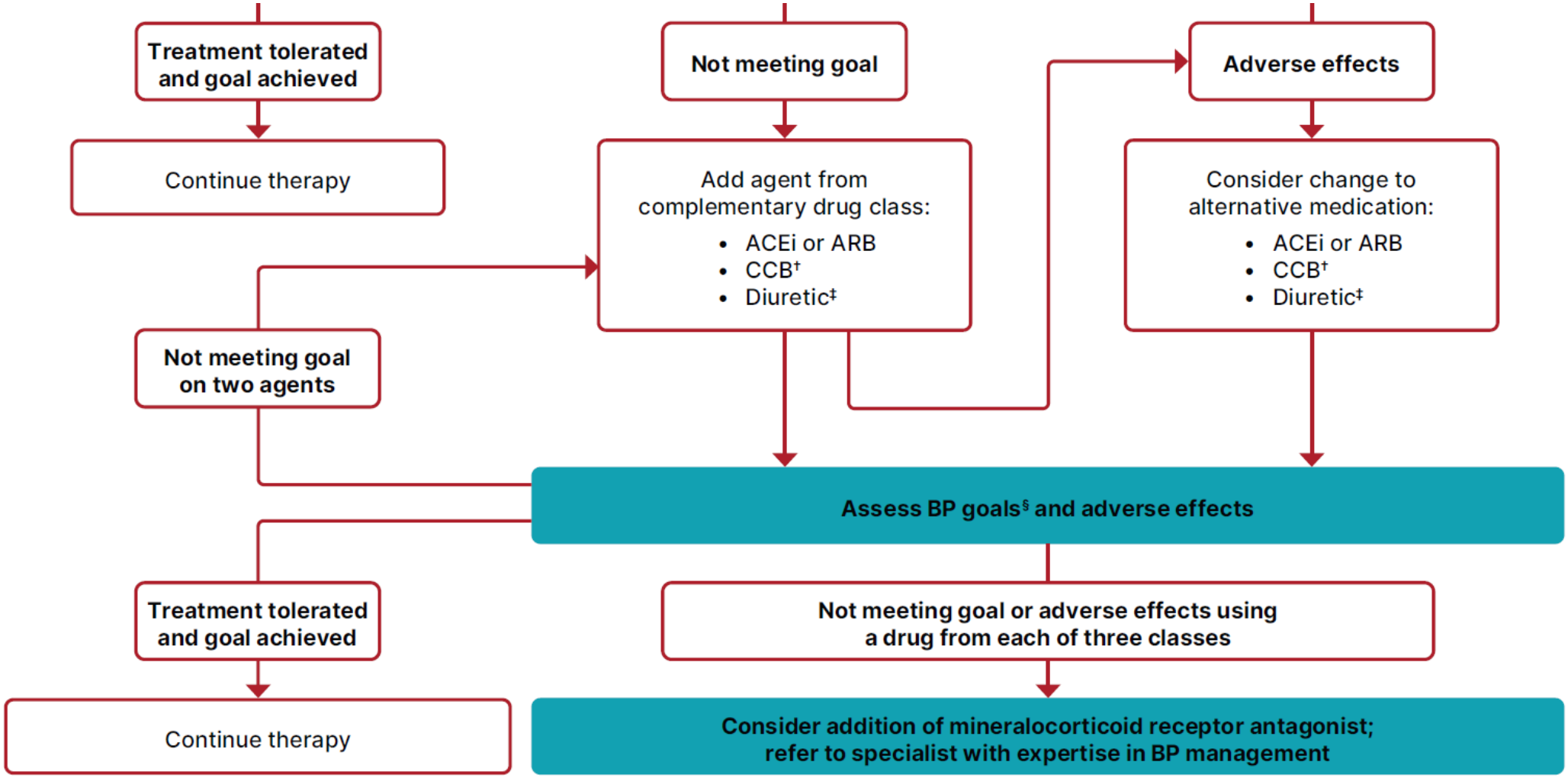


Figure 10.2 (continued)
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

Figure 10.2—Recommendations for the treatment of confirmed hypertension in nonpregnant people with diabetes. *An ACE inhibitor (ACEi) or angiotensin receptor blocker (ARB) is suggested for the treatment of hypertension in people with coronary artery disease (CAD) or urine albumin-to-creatinine ratio 30–299 mg/g creatinine and is strongly recommended for individuals with urine albumin-to-creatinine ratio ≥300 mg/g creatinine. †Dihydropyridine calcium channel blocker (CCB). ‡Thiazide-like diuretic; long-acting agents shown to reduce cardiovascular events, such as chlorthalidone and indapamide, are preferred. §If it can be safely attained, the on-treatment blood pressure goal is <130/80 mmHg; a systolic blood pressure goal <120 mmHg should be encouraged in individuals with high cardiovascular or kidney risk. BP, blood pressure. Adapted from de Boer et al. (24).

Hypertension and Blood Pressure Management – Treatment Strategies (Lifestyle Intervention)

10.5 For people with diabetes and blood pressure >120/80 mmHg, advise lifestyle behaviors including weight loss when indicated, a Dietary Approaches to Stop Hypertension (DASH)–style eating pattern including reducing sodium, limiting or avoiding alcohol consumption, increased physical activity, and smoking cessation. **A**

Hypertension and Blood Pressure Management – Treatment Strategies (Pharmacologic Interventions)

10.6 In individuals with confirmed office-based blood pressure ≥130/80 mmHg, pharmacologic therapy should be initiated and titrated to achieve their individualized blood pressure goal. **A**

10.7 Individuals with confirmed office based blood pressure ≥150/90 mmHg should, in addition to lifestyle therapy, have prompt initiation and timely titration of two drugs or a single-pill combination of drugs for hypertension that have also been demonstrated to reduce cardiovascular events in people with diabetes. **A**

10.8 Treatment for hypertension should include drug classes demonstrated to reduce cardiovascular events in people with diabetes. **A** ACE inhibitors or angiotensin receptor blockers (ARBs) are recommended first-line therapy for hypertension in people with diabetes and albuminuria or coronary artery disease. **A**

10.9 Multiple-drug therapy is generally required to achieve blood pressure goals. However, avoid any combination of ACE inhibitors, ARBs (including ARBs and neprilysin inhibitors), and direct renin inhibitors. **A**

Hypertension and Blood Pressure Management – Treatment Strategies (Pharmacologic Interventions, continued)

10.10 In nonpregnant people with diabetes and hypertension, either an ACE inhibitor or ARB is recommended for those with moderately increased albuminuria (UACR 30–299 mg/g creatinine) **B** and is strongly recommended for those with severely increased albuminuria (UACR ≥300 mg/g creatinine) and/or estimated glomerular filtration rate (eGFR) <60 mL/min/1.73 m^2 to maximally tolerated dose to prevent the progression of kidney disease and reduce cardiovascular events. **A** If one class is not tolerated, the other should be substituted. **B**

10.11 Monitor for drop in eGFR and for increase in serum potassium levels at initiation and periodically as clinically appropriate when ACE inhibitors, ARBs, and mineralocorticoid receptor antagonists (MRAs) are used. **B** Monitor for hypokalemia when diuretics are used at routine visits and 7–14 days after initiation or after a dose change and periodically as clinically appropriate. **B**

10.12 In sexually active individuals of childbearing potential who are not using reliable contraception, avoid ACE inhibitors, ARBs, MRAs, direct renin inhibitors, and neprilysin inhibitors, as they are contraindicated in pregnancy. **A**

Hypertension and Blood Pressure Management – Treatment Strategies (Resistant Hypertension)

10.13 Individuals with hypertension who are not meeting blood pressure goals on three classes of antihypertensive medications (including a diuretic) should be considered for MRA therapy. **A**

Lipid Management (Lifestyle Intervention)

10.14 Lifestyle modification focusing on weight loss (if indicated); application of a Mediterranean or DASH eating pattern; reduction of saturated fat and trans fat; increase of dietary n-3 fatty acids, viscous fiber, and plant stanol and sterol intake; and increased physical activity should be recommended to improve the lipid profile and reduce the risk of developing atherosclerotic cardiovascular disease (ASCVD) in people with diabetes. **A**

10.15 Intensify lifestyle therapy and optimize glycemic management for people with diabetes with elevated triglyceride levels (≥150 mg/dL [≥1.7 mmol/L]) and/or low HDL cholesterol (<40 mg/dL [<1.0 mmol/L] for men and <50 mg/dL [<1.3 mmol/L] for women). **C**

Lipid Management (Ongoing Therapy and Monitoring with Lipid Panel)

10.16 In adults with prediabetes or diabetes not taking statins or other lipid-lowering therapy, it is reasonable to obtain a lipid profile at the time of diagnosis, at an initial medical evaluation, annually thereafter, or more frequently if indicated. **E**

10.17 Obtain a lipid profile at initiation of statins or other lipid-lowering therapy, 4–12 weeks after initiation or a change in dose, and annually thereafter, as it facilitates monitoring the response to therapy and informs medication-taking behavior. **A**

Statin Treatment (Primary Prevention)

10.18 For people with diabetes aged 40–75 years without ASCVD, use moderate-intensity statin therapy in addition to lifestyle therapy. **A**

10.19 For people with diabetes aged 20–39 years with additional ASCVD risk factors, it may be reasonable to initiate statin therapy in addition to lifestyle therapy. **C**

10.20 For people with diabetes aged 40–75 years at higher cardiovascular risk, including those with one or more additional ASCVD risk factors, high-intensity statin therapy is recommended to reduce LDL cholesterol by ≥50% of baseline and to obtain an LDL cholesterol goal of <70 mg/dL (<1.8 mmol/L). **A**

10.21 For people with diabetes aged 40–75 years at higher cardiovascular risk, especially those with multiple additional ASCVD risk factors and an LDL cholesterol ≥70 mg/dL (≥1.8 mmol/L), it may be reasonable to add ezetimibe or a proprotein convertase subtilisin/kexin type 9 (PCSK9) inhibitor to maximum tolerated statin therapy. **B**

Statin Treatment (Primary Prevention, continued)

10.22 In adults with diabetes aged >75 years already on statin therapy, it is reasonable to continue statin treatment. **B**

10.23 In adults with diabetes aged >75 years, it may be reasonable to initiate moderate-intensity statin therapy after discussion of potential benefits and risks. **C**

10.24 In people with diabetes intolerant to statin therapy, treatment with bempedoic acid is recommended to reduce cardiovascular event rates as an alternative cholesterol-lowering plan. **A**

10.25 In most circumstances, lipid-lowering agents should be stopped prior to conception and avoided in sexually active individuals of childbearing potential who are not using reliable contraception. **B** In some circumstances (e.g., familial hypercholesterolemia, familial hypertriglyceridemia, prior ASCVD event, or history of pancreatitis), lipid-lowering therapy may be continued when the benefits outweigh risks. **E**

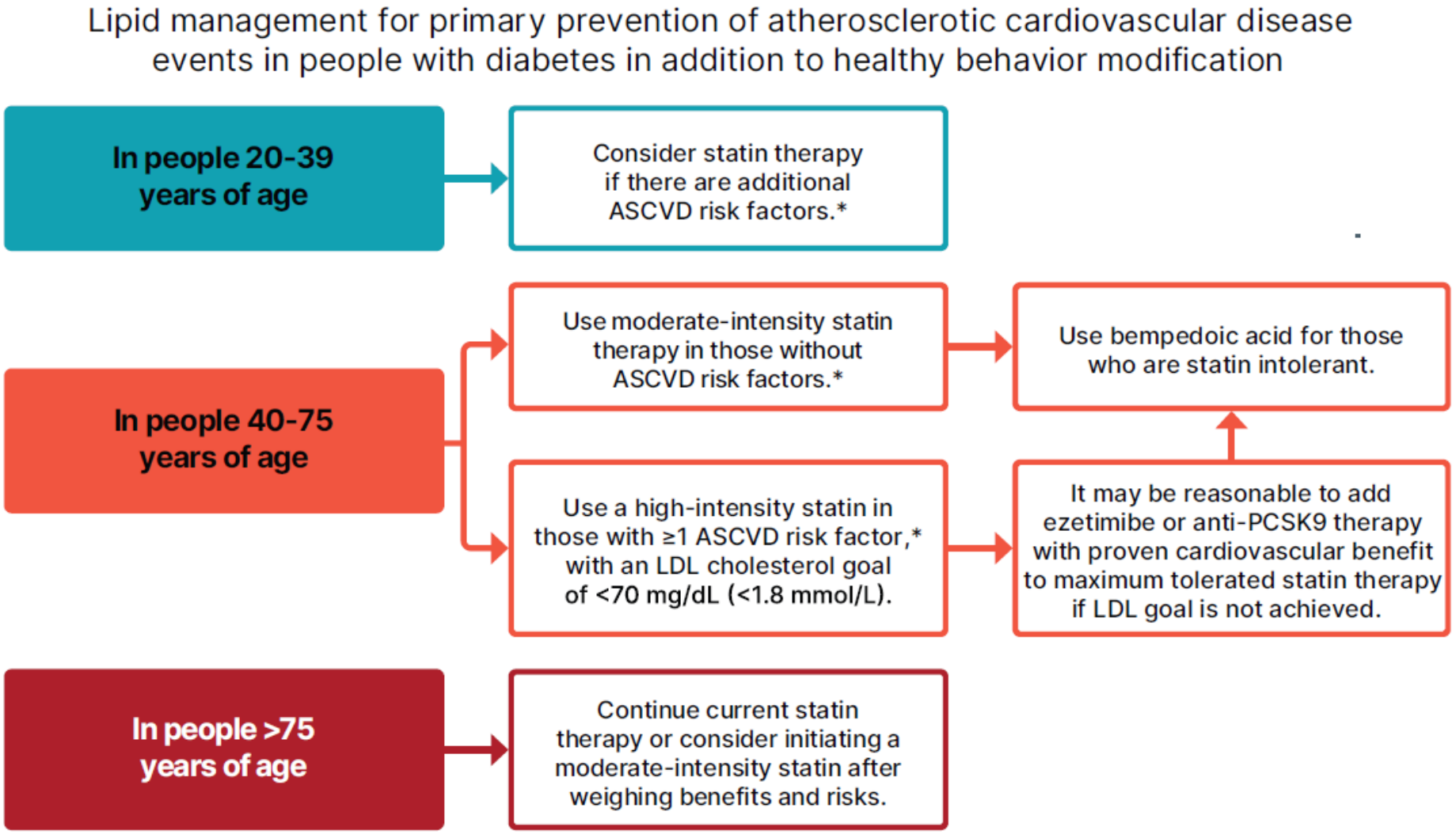


Figure 10.3—Recommendations for primary prevention of atherosclerotic cardiovascular disease (ASCVD) in people with diabetes using cholesterol-lowering therapy. *ASCVD risk factors include older age, hypertension, dyslipidemia, smoking, chronic kidney disease, or obesity. Adapted from "Standards of Care in Diabetes—2024 Abridged for Primary Care Professionals" (315).

Figure 10.3
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

Statin Treatment (Secondary Prevention)

10.26 For people of all ages with diabetes and ASCVD, high-intensity statin therapy should be added to lifestyle therapy. **A**

10.27 For people with diabetes and ASCVD, treatment with high-intensity statin therapy is recommended to obtain an LDL cholesterol reduction of ≥50% from baseline and an LDL cholesterol goal of <55 mg/dL (<1.4 mmol/L). Addition of ezetimibe or a PCSK9 inhibitor with proven benefit in this population is recommended if this goal is not achieved on maximum tolerated statin therapy. **B**

10.28a For individuals who do not tolerate the intended statin intensity, the maximum tolerated statin dose should be used. **E**

10.28b For people with diabetes and ASCVD intolerant to statin therapy, PCSK9 monoclonal antibody therapy, **A** bempedoic acid therapy, **A** or PCSK9 inhibitor therapy with inclisiran siRNA **E** should be considered as an alternative cholesterol-lowering therapy.

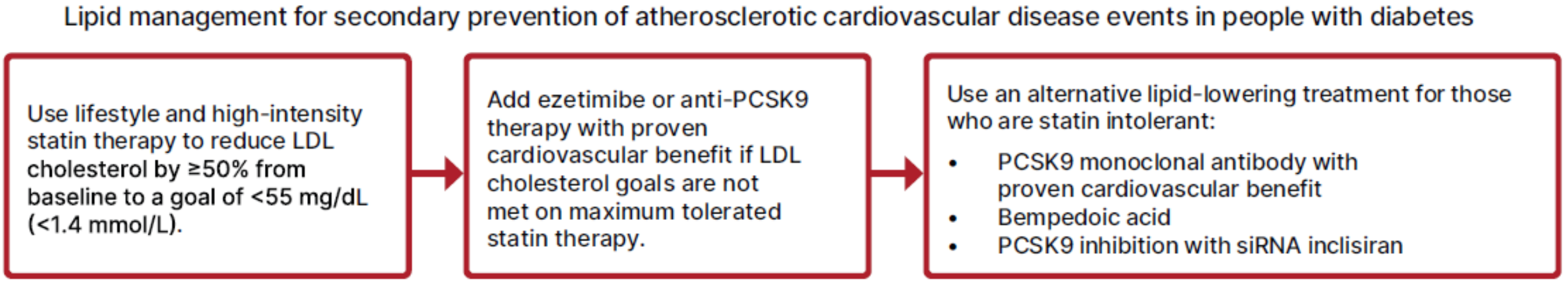


Figure 10.4—Recommendations for secondary prevention of atherosclerotic cardiovascular disease (ASCVD) in people with diabetes using cholesterol-lowering therapy. Adapted from "Standards of Care in Diabetes—2024 Abridged for Primary Care Professionals" (315).

Table 10.1—High-intensity and moderate-intensity statin therapy

High-intensity statin therapy (lowers LDL cholesterol by ≥50%)	Moderate-intensity statin therapy (lowers LDL cholesterol by 30–49%)
Atorvastatin 40–80 mg	Atorvastatin 10–20 mg
Rosuvastatin 20–40 mg	Rosuvastatin 5–10 mg
	Simvastatin 20–40 mg
	Pravastatin 40–80 mg
	Lovastatin 40 mg
	Fluvastatin XL 80 mg
	Pitavastatin 1–4 mg

Once-daily dosing. XL, extended release.

Table 10.1
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

Treatment of Other Lipoprotein Fractions or Goals

10.29 For individuals with fasting triglyceride levels ≥500 mg/dL (≥5.7 mmol/L), evaluate for secondary causes of hypertriglyceridemia and consider medical therapy to reduce the risk of pancreatitis. **C**

10.30 In adults with hypertriglyceridemia (fasting triglycerides >150 mg/dL [>1.7 mmol/L] or nonfasting triglycerides >175 mg/dL [>2.0 mmol/L]), clinicians should address and treat lifestyle factors (obesity and metabolic syndrome), secondary factors (diabetes, chronic liver or kidney disease and/or nephrotic syndrome, and hypothyroidism), and medications that raise triglycerides. **C**

10.31 In individuals with ASCVD or other cardiovascular risk factors on a statin with managed LDL cholesterol but elevated triglycerides (150–499 mg/dL [1.7–5.6 mmol/L]), the addition of icosapent ethyl can be considered to reduce cardiovascular risk. **B**

Other Combination Therapy

10.33 In individuals receiving statin therapy, the addition of fibrate, niacin, or dietary supplements containing n-3 fatty acids is not recommended as they do not provide additional cardiovascular risk reduction. **A**

Antiplatelet Agents

10.33 Use aspirin therapy (75–162mg/day) as a secondary prevention strategy in those with diabetes and a history of ASCVD. **A**

10.34a For individuals with ASCVD and documented aspirin allergy, clopidogrel (75 mg/day) should be used. **B**

10.34b The length of treatment with dual antiplatelet therapy using low-dose aspirin and a P2Y12 inhibitor in individuals with diabetes after an acute coronary syndrome, acute ischemic stroke, or transient ischemic attack should be determined by an interprofessional team approach that includes a cardiovascular or neurological specialist, respectively. **E**

10.35 Combination therapy with 81 mg aspirin daily plus 2.5 mg rivaroxaban twice daily should be considered for individuals with stable coronary and/or peripheral artery disease (PAD) and low bleeding risk to prevent major adverse limb and cardiovascular events. **A**

10.36 Aspirin therapy (75–162 mg/day) may be considered as a primary prevention strategy in those with diabetes who are at increased cardiovascular risk after a comprehensive discussion with the individual on the benefits versus the comparable increased risk of bleeding. **A**

Cardiovascular Disease – Screening

10.37a In asymptomatic individuals, routine screening for coronary artery disease is not recommended, as it does not improve outcomes as long as ASCVD risk factors are treated. **A**

10.37b Consider investigations for coronary artery disease in the presence of any of the following: signs or symptoms of cardiac or associated vascular disease, including carotid bruits, transient ischemic attack, stroke, claudication, or PAD; or electrocardiographic abnormalities (e.g., pathological Q waves). **E**

Cardiovascular Disease – Screening (continued)

10.38a Adults with diabetes are at increased risk for the development of asymptomatic cardiac structural or functional abnormalities (stage B heart failure) or symptomatic (stage C) heart failure. Consider screening adults with diabetes by measuring a natriuretic peptide (B-type natriuretic peptide [BNP] or N-terminal pro-BNP [NTproBNP]) to facilitate prevention of stage C heart failure. **B**

10.38b In asymptomatic individuals with diabetes and abnormal natriuretic peptide levels, echocardiography is recommended to identify stage B heart failure. **A**

10.39 In asymptomatic individuals with diabetes and age ≥65 years, microvascular disease in any location, or foot complications or any end-organ damage from diabetes, screening for PAD with ankle-brachial index testing is recommended if a PAD diagnosis would change management. **B**

Cardiovascular Disease – Treatment

10.40a Among people with type 2 diabetes who have established ASCVD or chronic kidney disease (CKD), a sodium–glucose cotransporter 2 (SGLT2) inhibitor or glucagon-like peptide 1 receptor agonist (GLP-1 RA) with demonstrated cardiovascular disease benefit is recommended as part of the comprehensive cardiovascular risk reduction and/or glucose-lowering treatment plans. **A**

10.40b In people with type 2 diabetes and established ASCVD or multiple ASCVD risk factors, or CKD, an SGLT2 inhibitor with demonstrated cardiovascular benefit is recommended to reduce the risk of cardiovascular events. **A**

10.40c In people with type 2 diabetes and established ASCVD or multiple risk factors for ASCVD, or CKD, a GLP-1 RA with demonstrated cardiovascular benefit is recommended to reduce the risk of cardiovascular events. **A**

10.40d In people with type 2 diabetes and established ASCVD or multiple risk factors for ASCVD, combined therapy with an SGLT2 inhibitor with demonstrated cardiovascular benefit and a GLP-1 RA with demonstrated cardiovascular benefit may be considered for additive reduction of the risk of adverse cardiovascular and kidney events. **B**

Cardiovascular Disease – Treatment (continued)

10.41a In people with type 2 diabetes and established heart failure with either preserved or reduced ejection fraction, an SGLT2 inhibitor (including SGLT1/2 inhibitor) with proven benefit in this population is recommended to reduce the risk of worsening heart failure and cardiovascular death. **A**

10.41b In people with type 2 diabetes and established heart failure with either preserved or reduced ejection fraction, an SGLT2 inhibitor with proven benefit in this population is recommended to improve quality of life. **A**

10.42 For individuals with type 2 diabetes and CKD with albuminuria treated with maximum tolerated doses of ACE inhibitor or ARB, recommend treatment with a nonsteroidal MRA with demonstrated benefit to improve cardiovascular outcomes and reduce the risk of CKD progression. **A**

Cardiovascular Disease – Treatment (continued)

10.43 In individuals with diabetes aged ≥55 years with established ASCVD or multiple ASCVD risk factors, ACE inhibitor or ARB therapy is recommended to reduce the risk of cardiovascular events. **A**

10.44a In individuals with diabetes and asymptomatic (stage B) heart failure, an interprofessional approach to optimize guideline-directed medical therapy, which should include a cardiovascular disease specialist, is recommended to reduce the risk for progression to symptomatic (stage C) heart failure. **A**

10.44b In individuals with diabetes and asymptomatic (stage B) heart failure, ACE inhibitors or ARBs and β-blockers are recommended to reduce the risk for progression to symptomatic (stage C) heart failure. **A**

Cardiovascular Disease – Treatment (continued)

10.44c In individuals with type 2 diabetes and asymptomatic (stage B) heart failure or with high risk of or established cardiovascular disease, treatment with an SGLT inhibitor with proven heart failure prevention benefit **A** or a GLP-1 RA with heart failure prevention benefit **B** is recommended to reduce the risk of hospitalization for heart failure.

10.44d In adults with type 2 diabetes, obesity, and symptomatic heart failure with preserved ejection fraction (HFpEF), the treatment plan should include a dual GIP/GLP-1 RA **A** or a GLP-1 RA **B** with demonstrated benefit for reduction in heart failure events.

10.44e In adults with type 2 diabetes, obesity, and symptomatic heart failure with preserved ejection fraction (HFpEF), the treatment plan should include a dual GIP/GLP-1 RA or a GLP-1 RA with demonstrated benefit for reduction in heart failure symptoms. **A**

Cardiovascular Disease – Treatment (continued)

10.44f In individuals with type 2 diabetes and CKD, recommend treatment with a nonsteroidal MRA with demonstrated benefit to reduce the risk of hospitalization for heart failure. **A**

10.44g In individuals with diabetes, guideline-directed medical therapy for myocardial infarction and symptomatic stage C heart failure is recommended with ACE inhibitors or ARBs (including ARBs and neprilysin inhibitors), MRAs, β-blockers, and SGLT2 inhibitors. **A**

10.44h In individuals with diabetes and symptomatic stage C heart failure with ejection fraction >40%, a nonsteroidal MRA with proven benefit in reducing worsening heart failure events is recommended. **A** A nonsteroidal MRA should not be used with an MRA.

Cardiovascular Disease – Treatment (continued)

10.45 In people with type 2 diabetes with stable heart failure, metformin may be continued for glucose lowering if eGFR remains >30 mL/min/1.73 m^2 but should be avoided in unstable or hospitalized individuals with heart failure. **B**

10.46 Educate individuals with diabetes who are at risk for developing diabetic ketoacidosis and who are treated with SGLT inhibition on the risks and signs of ketoacidosis and methods of risk mitigation management, provide them with appropriate tools for ketone measurement (i.e., serum β-hydroxybutyrate), and discourage a ketogenic eating pattern. **E**

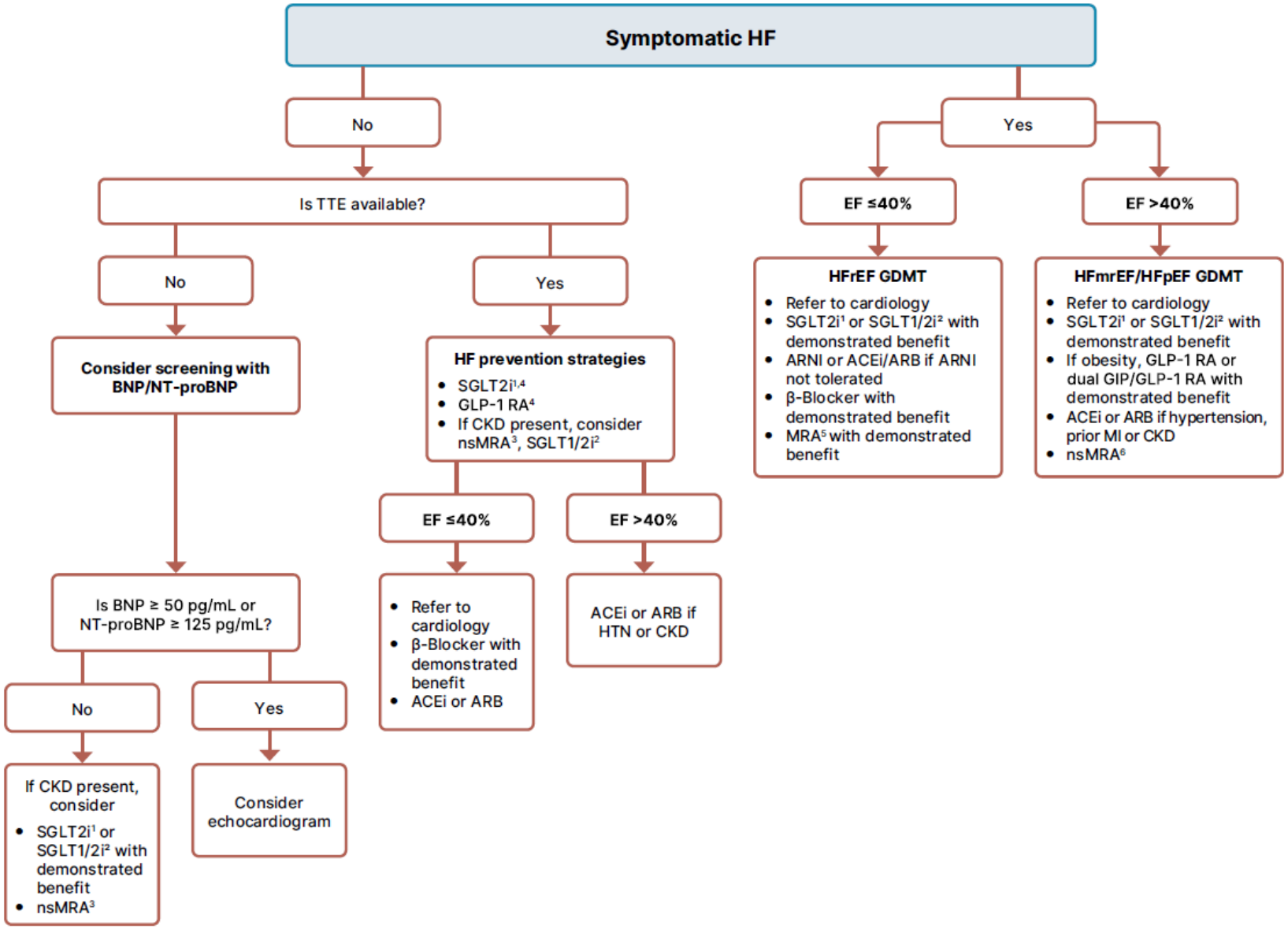


Figure 10.5
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

1. SGLT2i parameters: if eGFR ≥ 20 mL/min/1.73 m²
2. SGLT1/2i can be used as an alternative to SGLT2i if eGFR ≥ 30 mL/min/1.73 m²
3. If UACR ≥ 30mg/g, K+<5.0 mEq/L (< 5.0 mmol/L)
4. The absolute benefit of SGLT2i and GLP-1RA in preventing symptomatic HF will be greatest among those at the highest absolute risk.
5. If eGFR ≥30 mL/min/1.73 m², K+<5.0 mEq/L (< 5.0 mmol/L)
6. If eGFR ≥25 mL/min/1.73 m², K+<5.0 mEq/L (< 5.0 mmol/L)

Figure 10.5—Overview of recommendations for the prevention and treatment of symptomatic heart failure in people with diabetes. ACEi, ACE inhibitor; ARB, angiotensin receptor blocker; ARNI, angiotensin receptor neprilysin inhibitor; BNP, B-type natriuretic peptide; NT-proBNP, N-terminal pro-B–type natriuretic peptide; CKD, chronic kidney disease; CVD, cardiovascular disease; EF, ejection fraction; eGFR, estimated glomerular filtration rate; GDMT, guideline-directed medical therapy; GIP/GLP-1 RA, glucose-dependent insulinotropic polypeptide and glucagon-like peptide-1 receptor agonist; GLP-1 RA, glucagon-like peptide-1 receptor agonist; HF, heart failure; HFmrEF, heart failure with mildly reduced ejection fraction; HFpEF, heart failure with preserved ejection fraction; HFrEF, heart failure with reduced ejection fraction; HTN, hypertension; MI, myocardial infarction; MRA, mineralocorticoid receptor antagonist; nsMRA, nonsteroidal mineralocorticoid receptor antagonist; SGLT1/2i, sodium–glucose cotransporter 1 and 2 inhibitor; SGLT2i, sodium–glucose cotransporter 2 inhibitor; TTE, transthoracic echocardiography; UACR, urine albumin-to-creatinine ratio.

Figure 10.5 (continued)
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

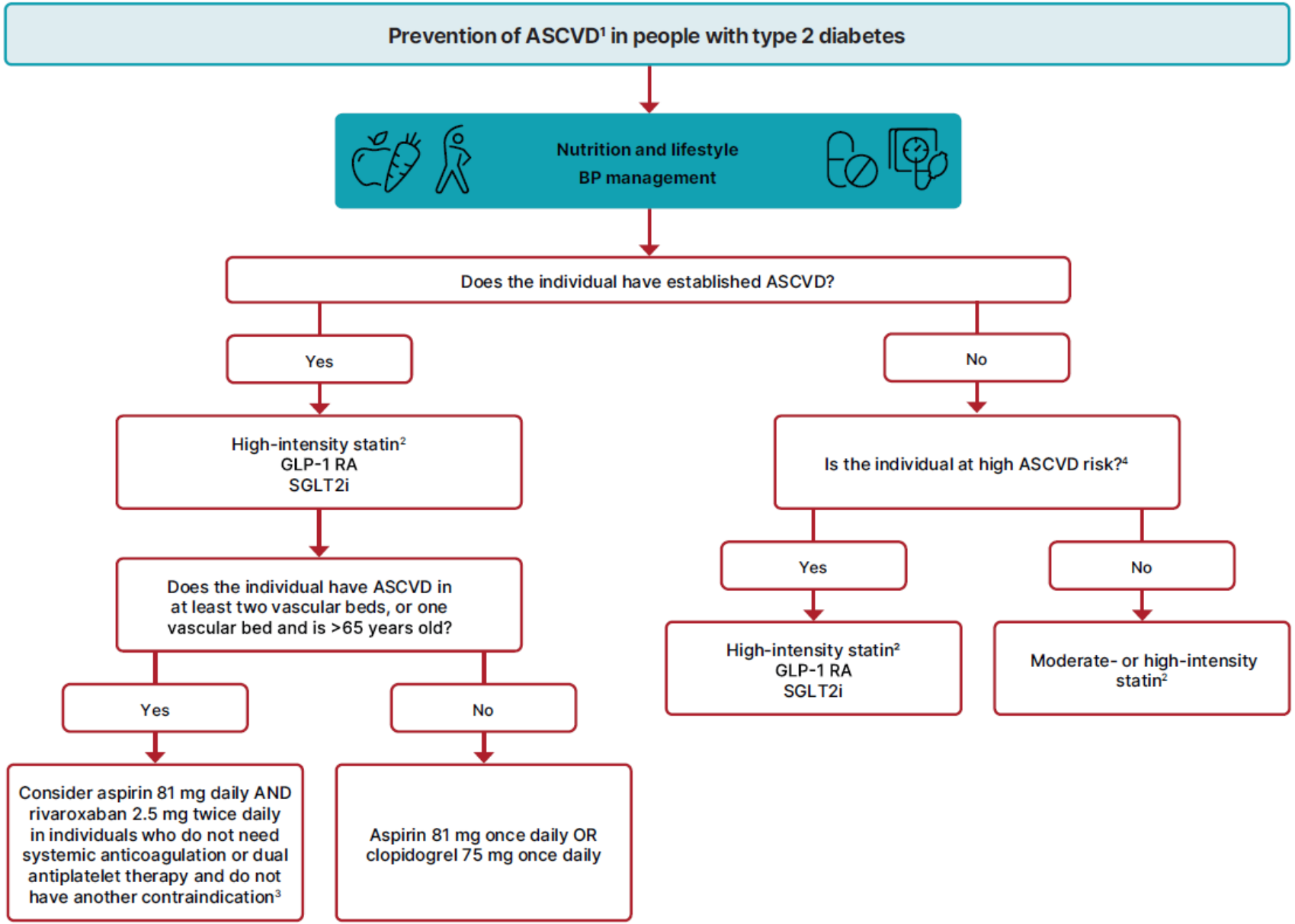


Figure 10.6
Cardiovascular Disease and Risk Management:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S216-S245

Fig 10.6 (continued)

1. ASCVD is defined as a history of an acute coronary syndrome or myocardial infarction, angina, coronary heart disease with or without revascularization, other arterial revascularization, stroke, or peripheral artery disease assumed to be atherosclerotic in origin.
2. Non-statins with demonstrated benefit should be considered in those with statin intolerance.
3. Consider low-dose rivaroxaban in people with atherosclerotic disease in at least two vascular beds, or in one bed and are older than 65 years, who do NOT have an increased risk of bleeding, recent (<1 year) stroke, renal failure, LVEF <30%, or need for dual antiplatelet therapy or systemic anticoagulation.
4. Individuals at high risk for ASCVD include those with end-organ damage such as left ventricular hypertrophy or retinopathy or with multiple CV risk factors (e.g., older age, hypertension, smoking, dyslipidemia, CKD, and obesity).

Figure 10.6—Approach to prevent ASCVD in people with type 2 diabetes. ASCVD, atherosclerotic cardiovascular disease; BP, blood pressure; CKD, chronic kidney disease; CV, cardiovascular; GLP-1 RA, glucagon-like peptide 1 receptor agonist; LVEF, left ventricle ejection fraction; SGLT2i, sodium–glucose cotransporter 2 inhibitor.

Section 11.

Chronic Kidney Disease and Risk Management

(https://doi.org/10.2337/dc26-S011)

Chronic Kidney Disease - Assessment

11.1a Assess kidney function with random urine albumin-to-creatinine ratio (UACR) and estimated glomerular filtration rate (eGFR) at least annually in people with type 1 diabetes with duration of ≥5 years and in all people with type 2 diabetes regardless of treatment. **B**

11.1b In people with chronic kidney disease (CKD), monitor urinary albumin (e.g., spot UACR) and eGFR 1–4 times per year depending on the stage of F1 kidney disease (**Fig. 11.1**). **B**

Chronic Kidney Disease – Surveillance

11.2 Aim to reduce urinary albumin by ≥30% in people with CKD and albuminuria ≥300 mg/g to slow CKD progression. **B**

Chronic Kidney Disease – Nutrition

11.3 For people with CKD stage G3 or higher, protein intake should be 0.8 g/kg body weight per day, as for the general population. A For individuals on dialysis, protein intake of 1.0–1.2 g/kg/day should be considered, since protein energy wasting is a major problem for some individuals on dialysis. **B**

Chronic Kidney Disease – Glycemic Goals

11.4 Optimize glucose management to reduce the risk or slow the progression of CKD (**Fig. 9.4**). **A**

Chronic Kidney Disease – Blood Pressure and Use of ACE Inhibitors and Angiotensin Receptor Blockers

11.5 If it can be safely attained, the on treatment blood pressure goal for people with CKD is <130/80 mmHg; a systolic blood pressure goal <120 mmHg and/or reduction in blood pressure variability should be encouraged. **A**

11.6a In nonpregnant people with diabetes and hypertension, either an ACE inhibitor or an angiotensin receptor blocker (ARB) is recommended for those with moderately increased albuminuria (UACR 30–299 mg/g creatinine) **B** and is strongly recommended for those with severely increased albuminuria (UACR ≥300 mg/g creatinine) and/or eGFR <60 mL/min/1.73 m^2 to maximally tolerated dose to prevent the progression of kidney disease and reduce cardiovascular events. **A** If one class is not tolerated, the other should be substituted. **B**

11.6b Monitor for drop in eGFR and increase in serum potassium levels at initiation and periodically as clinically appropriate when ACE inhibitors, ARBs, and mineralocorticoid receptor antagonists (MRAs) are used. **B** Monitor for hypokalemia when diuretics are used at routine visits and 7–14 days after initiation or after a dose change and periodically as clinically appropriate. **B**

Chronic Kidney Disease – Blood Pressure and Use of ACE Inhibitors and Angiotensin Receptor Blockers (continued)

11.6c An ACE inhibitor or an ARB is not recommended for the primary prevention of CKD in people with diabetes who have normal blood pressure, normal UACR (<30 mg/g creatinine), and normal eGFR. **A**

11.6d Continue renin-angiotensin system blockade for mild to moderate increases in serum creatinine (≤30%) in individuals who have no signs of extracellular fluid volume depletion. **A**

Chronic Kidney Disease – Glucose Lowering Medications

11.7a For people with type 2 diabetes and CKD, use of a sodium–glucose cotransporter 2 (SGLT2) inhibitor with demonstrated benefit to reduce CKD progression and cardiovascular events is recommended. SGLT2 inhibitors should be initiated in individuals with eGFR ≥20 mL/min/1.73 m^2 but can safely continue until kidney failure. **A**

11.7b To reduce kidney disease progression and cardiovascular risk in people with type 2 diabetes and CKD, a glucagon-like peptide 1 agonist with demonstrated benefit in this population is recommended. **A**

Chronic Kidney Disease – Mineralocorticoid Receptor Antagonists

11.8 To reduce CKD progression and cardiovascular events in people with CKD and albuminuria, a nonsteroidal MRA that has been shown to be effective in clinical trials is recommended (if eGFR is ≥25 mL/min/1.73 m^2). Potassium levels should be monitored 1 month after initiation. **A**

Chronic Kidney Disease – Combination therapy

11.9 Simultaneous initiation of an SGLT2 inhibitor and a nonsteroidal MRA (finerenone) can be considered in adults with type 2 diabetes and UACR ≥100 mg/g with eGFR 30-90 mL/min/1.73m^2 on RAS inhibitor due to evidence of safety and beneficial effects on albuminuria. **B**

Chronic Kidney Disease – Kidney-Protective Medications in Pregnancy

11.10 Kidney-protective medications that are potentially harmful in pregnancy should be avoided in sexually active individuals of childbearing potential who are not using reliable contraception and, if used, should be switched prior to conception to kidney-protective medications considered safer during pregnancy. **B**

Chronic Kidney Disease – Severe Chronic Kidney Disease and Kidney Failure

11.11a Individuals with eGFR <20 mL/min/1.73 m^2 and not on dialysis can be safely continued on SGLT2 inhibitors to reduce the risk of CKD progression **B** and for cardiovascular benefits. **C**

11.11b Individuals on dialysis can be safely initiated or continued on GLP-1–based therapy that is not dependent on kidney clearance to reduce cardiovascular risk and mortality. **C**

Chronic Kidney Disease – Referral

11.12a Individuals should be referred for evaluation by a nephrologist if they have rapidly increasing urinary albumin levels and/or rapidly decreasing eGFR and/or if the eGFR is <30 mL/min/1.73 m^2. **A**

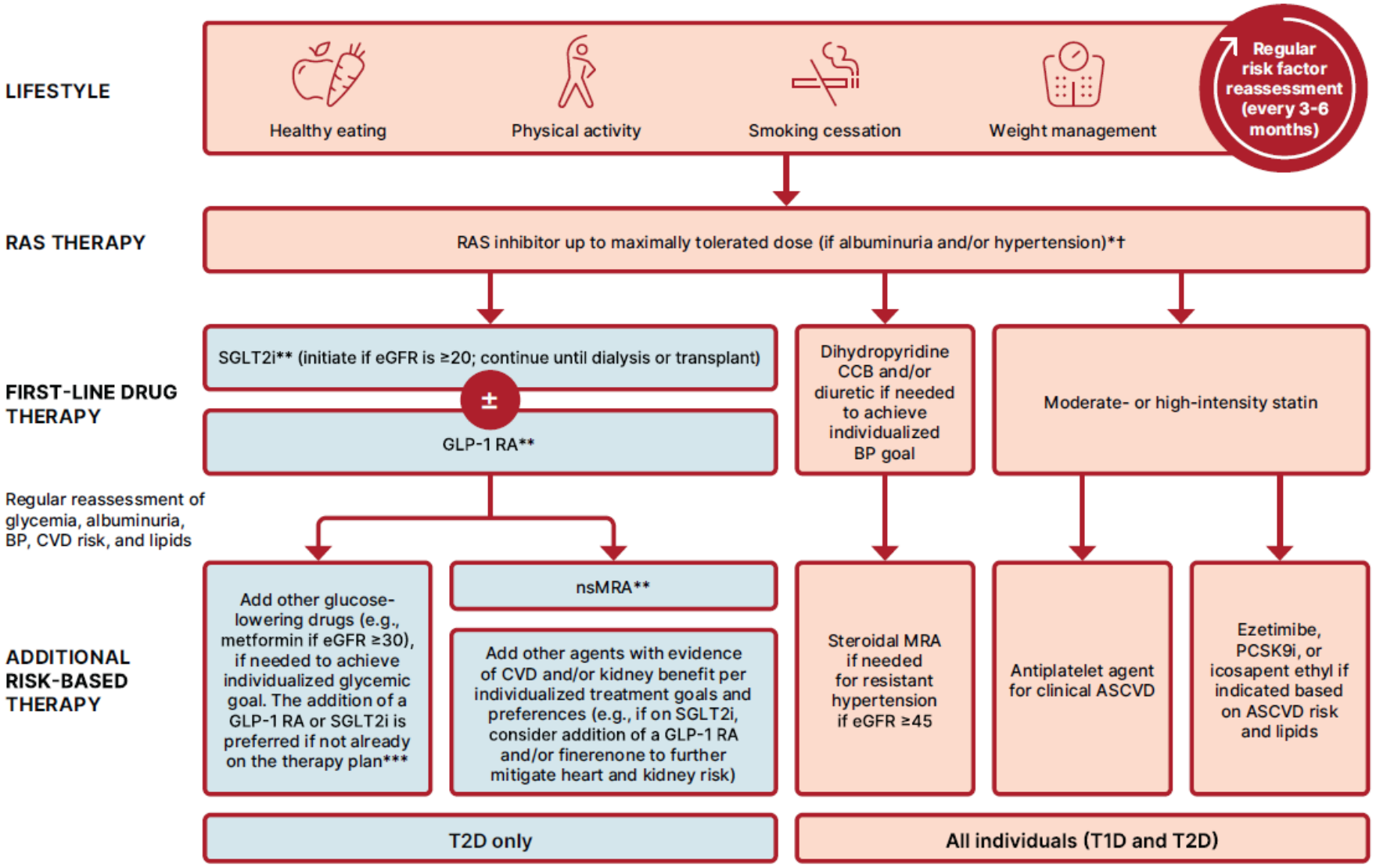
11.12b Refer to a nephrologist for uncertainty about the etiology of kidney disease or difficult management issues. **B**

CKD is classified based on: • GFR (G) • Albuminuria (A)				Albuminuria categories Description and range		
				A1	A2	A3
				Normal to mildly increased	Moderately increased	Severely increased
				<30 mg/g <3 mg/mmol	30-299 mg/g 3-29 mg/mmol	≥300 mg/g ≥30 mg/mmol
GFR categories (mL/min/1.73 m²) Description and range	G1	Normal or high	≥90	Screen 1	Treat 1	Treat and refer 2
	G2	Mildly decreased	60-89	Screen 1	Treat 1	Treat and refer 2
	G3a	Mildly to moderately decreased	45-59	Treat 1	Treat 2	Treat and refer 3
	G3b	Moderately to severely decreased	30-44	Treat 2	Treat and refer 3	Treat and refer 3
	G4	Severely decreased	15-29	Treat and refer 3	Treat and refer 3	**Treat and refer 4+**
	G5	Kidney failure	<15	**Treat and refer 4+**	**Treat and refer 4+**	**Treat and refer 4+**

Low risk (if no other markers of kidney disease, no CKD)
Moderately increased risk
High risk
Very high risk

Figure 11.1—Risk of CKD progression, cardiovascular disease risk, and mortality; frequency of visits; and referral to nephrology according to GFR and albuminuria. The numbers in the boxes are a guide to the frequency of screening or monitoring (number of times per year). Green reflects no evidence of CKD by estimated GFR or albuminuria, with screening indicated once per year. For monitoring of prevalent CKD, suggested monitoring varies from once per year (yellow) to four times or more per year (i.e., every 1–3 months [deep red]) according to risks of CKD progression and CKD complications (e.g., cardiovascular disease, anemia, and hyperparathyroidism). These are general parameters based only on expert opinion and underlying comorbid conditions, and disease state must be taken into account, as should the likelihood of impacting a change in management for any individual. CKD, chronic kidney disease; GFR, glomerular filtration rate. Adapted from de Boer et al. (1).

Figure 11.1
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260



*The majority of participants in SGLT2i, GLP-1 RA and nsMRA kidney outcome trials were receiving background optimized RAS inhibitor therapy.
**With demonstrated benefit in this population
***Glucose-lowering efficacy of GLP-1 RAs is preserved at low eGFR; glucose-lowering efficacy of SGLT2i is diminished at lower eGFR.

Figure 11.2
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260

Figure 11.2—Holistic approach for improving outcomes in people with diabetes and CKD. Icons presented indicate the following benefits: BP cuff, BP lowering; glucose meter, glucose lowering; heart, cardioprotection; kidney, kidney protection; scale, weight management. eGFR is presented in units of mL/min/1.73 m^2. †ACEi or ARB (at maximal tolerated doses) should be first-line therapy for hypertension when albuminuria is present. Otherwise, dihydropyridine calcium channel blocker or diuretic can also be considered; all three classes are often needed to attain BP targets. ‡Finerenone is currently the only nsMRA with proven clinical kidney and cardiovascular benefits. ACEi, angiotensin-converting enzyme inhibitor; ACR, albumin-to-creatinine ratio; ARB, angiotensin receptor blocker; ASCVD, atherosclerotic cardiovascular disease; BP, blood pressure; CCB, calcium channel blocker; CVD, cardiovascular disease; eGFR, estimated glomerular filtration rate; GLP-1 RA, glucagon-like peptide 1 receptor agonist; HTN, hypertension; MRA, mineralocorticoid receptor antagonist; nsMRA, nonsteroidal mineralocorticoid receptor antagonist; PCSK9i, proprotein convertase subtilisin/kexin type 9 inhibitor; RAS, renin-angiotensin system; SGLT2i, sodium–glucose cotransporter 2 inhibitor; T1D, type 1 diabetes; T2D, type 2 diabetes.

Figure 11.2 (continued)
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260

Table 11.1—Reasons to consider nondiabetic kidney diseases in a person with chronic kidney disease and diabetes

- Type 1 diabetes duration <5 years or no retinopathy
- Active urine sediment (e.g., containing red blood cells or cellular casts)
- Chronically well-managed blood glucose
- Rapidly declining eGFR
- Rapidly increasing or very high UACR (>300 mg/g) or urine protein/creatinine level (>500 mg/g)

Information adapted from Liang et al. (133). eGFR, estimated glomerular filtration rate; UACR, urine albumin-to-creatinine ratio.

Table 11.1
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260

Table 11.2—Screening for selected complications of chronic kidney disease

Complication	Physical and laboratory evaluation
Blood pressure >130/80 mmHg	Blood pressure, weight, BMI
Volume overload	History, physical examination, weight
Electrolyte abnormalities	Serum electrolytes
Metabolic acidosis	Serum electrolytes
Anemia	Hemoglobin; iron, iron saturation, ferritin testing if indicated
Metabolic bone disease	Serum calcium, phosphate, PTH, vitamin 25(OH)D

Complications of chronic kidney disease (CKD) generally become prevalent when estimated glomerular filtration rate falls below 60 mL/min/1.73 m^2 (stage G3 CKD or greater) and become more common and severe as CKD progresses. Evaluation of elevated blood pressure and volume overload should occur at every clinical contact possible; laboratory evaluations are generally indicated every 6–12 months for stage G3 CKD, every 3–5 months for stage G4 CKD, and every 1–3 months for stage G5 CKD, or as indicated to evaluate symptoms or changes in therapy. 25(OH)D, 25-hydroxyvitamin D; PTH, parathyroid hormone.

Table 11.2
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260

Table 11.3—Interventions that lower albuminuria

- Blood glucose management
- Blood pressure management
- Treatment with ACE inhibitors or ARBs
- Smoking cessation
- Weight loss
- Changes in eating patterns (decreased salt intake and/or protein intake)
- Physical activity
- Treatment with SGLT2 inhibitors, MRAs, or GLP-1 RAs

ARB, angiotensin receptor blocker; GLP-1 RA, glucagon-like peptide 1 receptor agonist; MRA, mineralocorticoid receptor antagonist; SGLT2, sodium–glucose cotransporter 2.

Table 11.3
Chronic Kidney Disease and Risk Management:
Standards of Care in Diabetes – 2026 Diabetes Care 2026;49(Suppl. 1):S246-S260

Section 12.

Retinopathy, Neuropathy, and Foot Care

(https://doi.org/10.2337/dc26-S012)

Diabetic Retinopathy

12.1 Implement strategies to help people with diabetes reach glycemic goals to reduce the risk or slow the progression of diabetic retinopathy. **A**

12.2 Implement strategies to help people with diabetes reach blood pressure and lipid goals to reduce the risk or slow the progression of diabetic retinopathy. **A**

Diabetic Retinopathy - Screening

12.3 Adults with type 1 diabetes should have an initial dilated and comprehensive eye examination by an ophthalmologist or optometrist 5 years after the onset of diabetes. **B**

12.4 People with type 2 diabetes should have an initial dilated and comprehensive eye examination by an ophthalmologist or optometrist at the time of the diabetes diagnosis. **B**

12.5 If there is no evidence of retinopathy from one or more annual eye exams and glycemic indicators are within the goal range, then screening every 1–2 years may be considered. If any level of diabetic retinopathy is present, subsequent dilated retinal examinations should be repeated at least annually by an ophthalmologist or optometrist. If retinopathy is progressing or sight threatening, then examinations by an ophthalmologist will be required more frequently. **B**

Diabetic Retinopathy – Screening (continued)

12.6 Programs that use retinal photography with remote reading or the use of U.S. Food and Drug Administration– approved artificial intelligence algorithms to improve access to diabetic retinopathy screening are appropriate screening strategies for diabetic retinopathy. Such programs need to provide pathways for timely referral for a comprehensive eye examination when indicated. **B**

12.7 Counsel individuals of childbearing potential with preexisting type 1 or type 2 diabetes who are planning pregnancy or who are pregnant on the risk of development and/or progression of diabetic retinopathy. **B**

12.8 Individuals with preexisting type 1 or type 2 diabetes should receive an eye exam before pregnancy as well as in the first trimester and may need to be monitored every trimester and for 1 year postpartum as indicated by the degree of retinopathy. **B**

Diabetic Retinopathy – Treatment

12.9 Promptly refer individuals with any level of diabetic macular edema, moderate or worse nonproliferative diabetic retinopathy (a precursor of proliferative diabetic retinopathy [PDR]), or any PDR to an ophthalmologist who is knowledgeable and experienced in the management of diabetic retinopathy. **A**

12.10 Panretinal laser photocoagulation therapy is indicated to reduce the risk of vision loss in individuals with high-risk PDR and, in some cases, severe nonproliferative diabetic retinopathy. **A**

12.11 Intravitreous injections of anti-vascular endothelial growth factor (anti-VEGF) are a reasonable alternative to traditional panretinal laser photocoagulation for some individuals with PDR and also reduce the risk of vision loss in these individuals. **A**

Diabetic Retinopathy – Treatment (continued)

12.12 Intravitreous injections of anti-VEGF are indicated as first-line treatment for most eyes with diabetic macular edema that involves the foveal center and impairs visual acuity. **A**

12.13 Macular focal/grid photocoagulation and intravitreal injections of corticosteroid are reasonable treatments in eyes with persistent diabetic macular edema despite previous anti-VEGF therapy or eyes that are not candidates for this first-line approach. **A**

12.14 The presence of retinopathy is not a contraindication to aspirin therapy for cardioprotection, as aspirin does not increase the risk of retinal hemorrhage. **A**

Visual Rehabilitation

12.15 People who experience diabetes-related vision loss should be counseled on the availability and scope of vision rehabilitation care and provided, or referred for, a comprehensive evaluation of their visual impairment by a practitioner experienced in vision rehabilitation. **E**

12.16 People with diabetes-related vision loss should receive educational materials and resources for eye care support in addition to self-management education (e.g., glycemic management and hypoglycemia awareness). **E**

Neuropathy – Screening

12.17 All people with diabetes should be assessed for diabetic peripheral neuropathy starting at diagnosis of type 2 diabetes and 5 years after the diagnosis of type 1 diabetes and at least annually thereafter. **B**

12.18 Assessment for distal symmetric polyneuropathy should include a careful history and assessment of either temperature or pinprick sensation (small-fiber function) and vibration sensation using a 128-Hz tuning fork (large-fiber function). All people with diabetes should have annual 10-g monofilament testing to identify feet at risk for ulceration and amputation. **B**

12.19 Symptoms and signs of autonomic neuropathy should be assessed in people with diabetes starting at diagnosis of type 2 diabetes and 5 years after the diagnosis of type 1 diabetes, and at least annually thereafter, and with evidence of other microvascular complications, particularly kidney disease and diabetic peripheral neuropathy. Screening can include asking about orthostatic dizziness, syncope, early satiety, erectile dysfunction, changes in sweating patterns, or dry cracked skin in the extremities. Signs of autonomic neuropathy include orthostatic hypotension, a resting tachycardia, or evidence of peripheral dryness or cracking of skin. **E**

Neuropathy – Treatment

12.20 Optimize glucose management to prevent or delay the development of neuropathy in people with type 1 diabetes **A** and to slow the progression of neuropathy in people with type 2 diabetes. **C** Optimize weight, blood pressure, and serum lipid management to reduce the risk or slow the progression of diabetic neuropathy. **B**

12.21 Assess and treat pain related to diabetic peripheral neuropathy **B** and symptoms of autonomic neuropathy to improve quality of life. **E**

12.22 Gabapentinoids, serotonin - norepinephrine reuptake inhibitors, tricyclic antidepressants, and sodium channel blockers are recommended as initial pharmacologic treatments for neuropathic pain in diabetes. **A** Combinations of these medications can provide additional relief of neuropathic pain. **A** Opioids, including tramadol and tapentadol, should not be used for neuropathic pain treatment in diabetes given the potential for adverse events except in rare circumstances. **B**

Foot Care

12.23 Perform a comprehensive foot evaluation at least annually to identify risk factors for ulcers and amputations. **A**

12.24 The examination should include inspection of the skin, assessment of foot deformities, neurological assessment (10-g monofilament testing or Ipswich touch test with at least one additional assessment: pinprick, temperature, or vibration), and vascular assessment, including pulses in the legs and feet. **B**

12.25 Individuals with evidence of sensory loss or prior ulceration or amputation should have their feet inspected at every visit. **A**

12.26 Obtain a prior history of ulceration, amputation, Charcot foot, angioplasty or vascular surgery, cigarette smoking, retinopathy, and renal disease and assess current symptoms of neuropathy (e.g., pain, burning, numbness) and vascular disease (e.g., leg fatigue, claudication). **B**

Foot Care (continued)

12.27 Initial screening for peripheral artery disease (PAD) should include assessment of lower-extremity pulses, capillary refill time, rubor on dependency, pallor on elevation, and venous filling time. Individuals with a history of leg fatigue, claudication, and rest pain relieved with dependency or decreased or absent pedal pulses should be referred for ankle-brachial index with toe pressures and for further vascular assessment as appropriate. **B**

12.28 An interprofessional approach facilitated by a podiatrist in conjunction with other appropriate team members is recommended for individuals with foot ulcers and high-risk feet (e.g., those on dialysis, those with Charcot foot, those with a history of prior ulcers or amputation, and those with PAD). **B**

12.29 Refer individuals who smoke and have a history of prior lower-extremity complications, loss of protective sensation, structural abnormalities, or PAD to foot care specialists for ongoing preventive care and lifelong surveillance. **B** These individuals should also be provided with information on the importance of smoking cessation and referred for counseling on smoking cessation. **A**

Foot Care (continued)

12.30 Provide general preventive foot self-care education to all people with diabetes, including those with loss of protective sensation, on appropriate ways to examine their feet (palpation or visual inspection with an unbreakable mirror) for daily surveillance of early foot problems. **B**

12.31 The use of specialized therapeutic footwear is recommended for people with diabetes at high risk for ulceration, including those with loss of protective sensation, foot deformities, ulcers, callous formation, poor peripheral circulation, or history of amputation. **B**

12.32 For chronic diabetic foot ulcers that have failed to heal with optimal standard care alone, adjunctive treatment with randomized controlled trial–proven advanced agents should be considered (e.g., negative-pressure wound therapy, several skin substitutes, or topical oxygen therapy). **A**

Factors that are associated with the at-risk foot include the following:

- Chronic hyperglycemia
- Peripheral neuropathy/LOPS
- PAD
- Foot deformities (bunions, hammertoes, Charcot joint, etc.)
- Preulcerative corns or calluses
- Prior ulceration
- Prior amputation
- Smoking
- Retinopathy
- Nephropathy (particularly individuals on dialysis or posttransplant)
- Social determinants of health such as socioeconomic status, and access-to-care factors

Table 12.1—International Working Group on Diabetic Foot risk stratification system and corresponding foot screening frequency

Category	Ulcer risk	Characteristics	Examination frequency*
0	Very low	No LOPS and no PAD	Annually
1	Low	LOPS or PAD	Every 6–12 months
2	Moderate	LOPS + PAD, or LOPS + foot deformity, or PAD + foot deformity	Every 3–6 months
3	High	LOPS or PAD and one or more of the following: • History of foot ulcer • Amputation (minor or major) • Kidney failure	Every 1–3 months

Adapted with permission from Schaper et al. (123). LOPS, loss of protective sensation; PAD, peripheral artery disease. *Examination frequency suggestions are based on expert opinion and person-centered requirements.

Table 12.1
Retinopathy, Neuropathy, and Foot Care:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S261-S276

Section 13.

Older Adults

(https://doi.org/10.2337/dc26-S013)

Overall recommendations

13.1 Assess the medical, psychological, functional (self-management abilities), and social domains in older adults with diabetes using a comprehensive approach to determine goals and therapeutic approaches for diabetes management. **B**

13.2 Screen at least annually for geriatric syndromes (e.g., cognitive impairment, depression, urinary incontinence, falls, persistent pain, and frailty), hypoglycemia, and polypharmacy in older adults with diabetes, as they may affect diabetes management and diminish quality of life. **B**

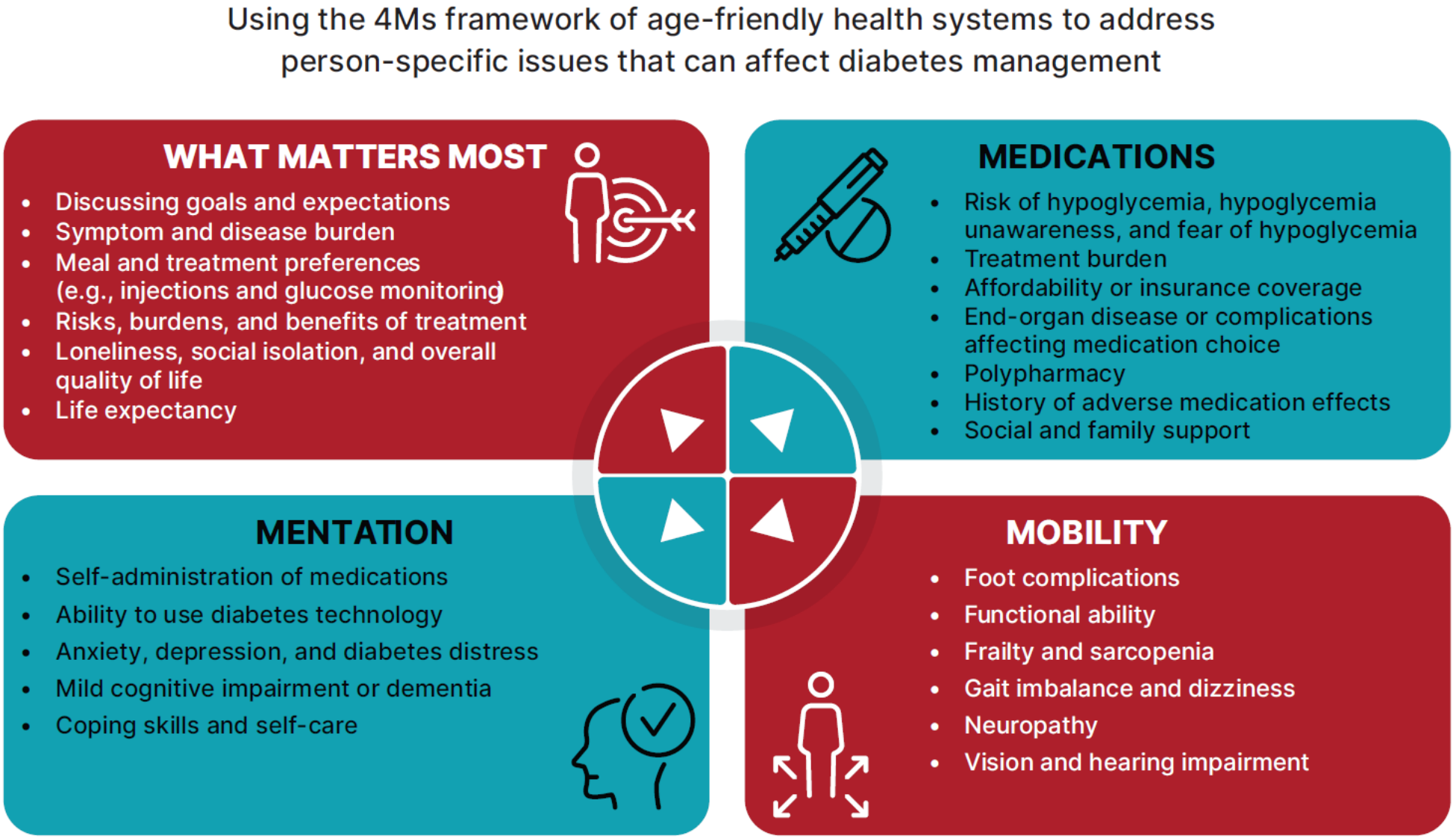


Figure 13.1—Using the 4Ms framework of age-friendly health systems to address person-specific issues that can affect diabetes management.

Figure 13.1
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Table 13.1—Geriatric syndromes and other functional impairments: key symptoms and suggested screening approaches

Domain	Sample screening questions	Suggested screening measure	No. of items	Score range	Cut points	Time to complete	Measure translated languages
Geriatric syndromes							
Cognitive impairment or dementia	Have you noticed any changes in your memory or ability to think clearly?	Mini-Cog (31)	5	0–5	<3 validated for dementia screening	3 min	Multiple
Delirium	What day of the week is it?	4AT (182)	4	0–12	≥4 possible delirium	2 min	Multiple
Depression	During the past month, have you often been bothered by feeling down, depressed, or hopeless?	Geriatric Depression Scale 5 (GDS-5) (183,184)	5	0–5	≥2 possible depression	2 min	Multiple
Falls and risk of falling	Have you fallen in the past year or are you afraid of falling due to a balance or walking problem?	STEADI tool (185)	3	0–3	A single "yes" response to any question indicates an increased risk of falling	1–2 min	Multiple
Frailty	Have you felt unusually tired recently or had difficulty walking up steps without resting?	Clinical Frailty Scale (CFS) (186)	1	1–9	≥5 clinically meaningful frailty	1–2 min	Multiple
Malnutrition	Have you unintentionally lost weight in the last 6 months?	Mini Nutritional Assessment Short Form (MNA-SF) (187)	6	0–14	12–14, normal nutritional status; 8–11, at risk for malnutrition; 0–7, malnourished	5 min	Multiple

Table 13.1
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Pain	Are you currently experiencing any pain?	Numeric Pain Rating Scale	1	0–10	1–3, mild; 4–6, moderate; 7–10, severe	1–2 min	NA
Polypharmacy	Are you taking more medications than you think you should, or are you experiencing side effects?	Beers Criteria or review all prescription and nonprescription medications (188)	List review	NA	The presence of any medication on the list warrants review, especially if the risk outweighs the benefit	Varies	Multiple
Sarcopenia	Have you had difficulty walking across a room or climbing a flight of stairs in the past year?	SARC-F Questionnaire (189)	5	0–10	≥4 sarcopenia risk	2–3 min	Multiple
Sensory impairment: hearing	Do you have difficulty hearing, even when using a hearing aid?	Whisper test (190)	6	NA	Possible hearing loss: unable to repeat at least 3 of the random letters and numbers	2–3 min	NA

Continued on p. S280

Table 13.1 (continued)
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Table 13.1—Continued

Domain	Sample screening questions	Suggested screening measure	No. of items	Score range	Cut points	Time to complete	Measure translated languages
Sensory impairment: vision	Do you have difficulty seeing, even when wearing your glasses?	Snellen chart	1	NA	20/40 or worse indicates vision impairment	2–3 min	NA
Sleep disorders: insomnia	Do you have trouble falling asleep, staying asleep, or waking up too early and not being able to fall back asleep?	Insomnia Severity Index 7 (ISI-7) (191)	7	0–28	≥8 insomnia	3 min	Multiple
Urinary incontinence	Do you have trouble with bladder control or accidental leakage?	3 Incontinence Questions (3IQ) (192)	3	NA	The tool classifies type of incontinence based on responses	1–2 min	Multiple
Functional impairments							
Dexterity	Do you have trouble using your hands for everyday tasks like buttoning clothes?	Button and coin test: button and unbutton a shirt button and pick up a small coin from a flat surface	2	0–2	0, significant impairment; 1, mild to moderate impairment; 2, limited to no impairment	1–2 min	NA
Dizziness	Do you experience dizziness, feeling lightheaded, or a feeling of spinning (vertigo)? Have you noticed any changes in your balance or steadiness?	Orthostatic vital signs; neurological examination (e.g., assessment of gait, balance, reflexes, sensory loss, vestibular function); medical evaluation	NA	NA	NA	NA	NA
Executive functioning	Do you find it hard to plan or organize your daily activities?	Trail Making Test B	25	Time to finish	≥180 s indicates impairment in executive function	3–5 min	Multiple

This table summarizes key symptoms and suggested screening approaches for common geriatric syndromes and other functional impairments that can be present in older adults with diabetes. NA, not applicable.

Table 13.1 (continued)
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Neurocognitive Function

13.3 Screening for early detection of mild cognitive impairment or dementia should be performed for adults 65 years of age or older at the initial visit, annually, and as appropriate. **B**

Hypoglycemia

13.4 Ascertain and address episodes of hypoglycemia at routine visits because older adults with diabetes have a greater risk of hypoglycemia, especially when treated with hypoglycemic agents (e.g., sulfonylureas, meglitinides, and insulin). **B**

13.5 Recommend continuous glucose monitoring (CGM) for older adults with type 1 diabetes **A** and type 2 diabetes on insulin therapy **B** to improve glycemic outcomes, reduce hypoglycemia, and reduce treatment burden.

13.6 Consider the use of automated insulin delivery systems **B** and other advanced insulin delivery devices such as connected pens **E** to reduce risk of hypoglycemia for older adults, based on individual ability and support system.

Treatment Goals

13.7a Older adults with diabetes with few and stable coexisting chronic illnesses, and intact cognitive and functional status, should have lower glycemic goals (such as A1C <7.0–7.5% [<53–58 mmol/mol]) and/or time in range [TIR] 70–180 mg/dL [3.9–10.0 mmol/L] of ≥70% and time below range ≤70 mg/dL [3.9 mmol/L] of ≤4%) if CGM is used. **C**

13.7b Older adults with diabetes and intermediate or complex health are clinically heterogeneous with variable life expectancy. Selection of glycemic goals should be individualized and should prioritize avoidance of hypoglycemia, with less stringent goals (such as A1C <8.0% [<64 mmol/mol] and/or TIR 70–180 mg/dL [3.9–10.0 mmol/L] of ≥50% and time below range <70 mg/dL [≤3.9 mmol/L] of <1%) for those with significant cognitive and/or functional limitations, frailty, severe comorbidities, and a less favorable risk-to-benefit ratio of diabetes medications. **C**

13.7c Older adults with very complex or poor health receive minimal benefit from stringent glycemic goals. Clinicians should focus on avoiding hypoglycemia and symptomatic hyperglycemia. **C**

Treatment Goals (continued)

13.8 Screening for diabetes complications should be individualized in older adults with diabetes. Prioritize screening for complications that would lead to impairment of functional status or quality of life. **C**

13.9 The on-treatment blood pressure goal for most older adults with diabetes is <130/80 mmHg when it can be achieved safely, **A** and more a relaxed blood pressure goal (e.g., <140/90 mmHg) may be used for people with poor health, limited life expectancy, or high risk for adverse effects of hypertensive therapy. **E**

13.10 Treatment of other cardiovascular risk factors should be individualized in older adults with diabetes, considering the time frame of benefit. Lipid-lowering therapy and antiplatelet agents may benefit those with life expectancies at least equal to the time frame of primary prevention or secondary intervention trials. **A**

Table 13.2—Framework for considering treatment goals for glycemia, blood pressure, and dyslipidemia in older adults with diabetes

Characteristics and health status of person with diabetes	Rationale	Reasonable A1C goal*	Reasonable CGM goals	Fasting or preprandial glucose	Bedtime glucose	Blood pressure	Lipids
Healthy (few coexisting chronic illnesses, intact cognitive and functional status)	Longer remaining life expectancy	<7.0–7.5% (<53–58 mmol/mol)	TIR 70–180 mg/dL (3.9–10.0 mmol) of ≥70%, and TBR ≤70 mg/dL (≤3.9 mmol/L) of ≤4%	80–130 mg/dL (4.4–7.2 mmol/L)	80–180 mg/dL (4.4–10.0 mmol/L)	<130/80 mmHg	Statin, unless contraindicated or not tolerated
Complex/intermediate (multiple coexisting chronic illnesses† or two or more ADL impairments or mild to moderate cognitive impairment)	Variable life expectancy. Individualize goals, considering: • Severity of comorbidities • Cognitive and functional limitations • Frailty • Risk-to-benefit ratio of diabetes medications • Individual preference	<8.0% (<64 mmol/mol)	TIR 70–180 mg/dL (3.9–10.0 mmol) of ≥50% and TBR <70 mg/dL (<3.9 mmol/L) of <1%	90–150 mg/dL (5.0–8.3 mmol/L)	100–180 mg/dL (5.6–10.0 mmol/L)	<130/80 mmHg	Statin, unless contraindicated or not tolerated

Table 13.2
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Very complex/poor health (PALTC or end-stage chronic illnesses‡ or moderate to severe cognitive impairment or two or more ADL impairments)	Limited remaining life expectancy makes benefit minimal	Avoid reliance on A1C; glucose management decisions should be based on avoiding hypoglycemia and symptomatic hyperglycemia	100–180 mg/dL (5.6–10.0 mmol/L)	110–200 mg/dL (6.1–11.1 mmol/L)	<140/90 mmHg	Consider likelihood of benefit with statin

This table represents a consensus framework for considering treatment goals for glycemia, blood pressure, and dyslipidemia in older adults with diabetes. The characteristic categories are general concepts. Not every individual will clearly fall into a particular category. Consideration of individual and care partner preferences, care partner engagement, abilities, and resources is an important aspect of treatment individualization. Additionally, an individual's health status and preferences may change over time. ADL, activities of daily living; CGM, continuous glucose monitoring; PALTC, post-acute and long-term care; TBR, time below range; TIR, time in range. *A lower A1C goal may be set for an individual if achievable without recurrent or severe hypoglycemia or undue treatment burden. †Coexisting chronic illnesses are conditions serious enough to require medications or lifestyle management and may include arthritis, cancer, heart failure, depression, emphysema, falls, hypertension, incontinence, stage 3 or worse chronic kidney disease, myocardial infarction, and stroke. "Multiple" means at least three, but many individuals may have five or more (193). ‡The presence of a single end-stage chronic illness, such as stage 3–4 heart failure or oxygen-dependent lung disease, chronic kidney disease requiring dialysis, or uncontrolled metastatic cancer, may cause significant symptoms or impairment of functional status and significantly reduce life expectancy. Adapted from Kirkman et al. (5).

Table 13.2 (continued)
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Lifestyle Management

13.11a Recommend healthful eating with adequate protein intake (at least 0.8 g/kg body weight/day) for older adults with diabetes to maintain and potentially higher, individualized amounts to regain lean body mass and function. **B**

13.11b Recommend regular physical activity, including aerobic activity, weight-bearing exercise, and resistance training as tolerated in those who can safely engage in such activities, particularly to maintain lean body mass, especially in those pursuing weight loss. **B**

13.12 For older adults with type 2 diabetes, overweight or obesity, and the capacity to exercise safely, an intensive lifestyle intervention focused on dietary changes, physical activity, and weight loss (e.g., 5–7%) should be considered for its benefits on quality of life, mobility and physical functioning, and cardiometabolic risk. **A**

Pharmacologic Therapy

13.13 Select medications with low risk of hypoglycemia in older adults with type 2 diabetes, specifically for those with hypoglycemia risk factors. **B**

13.14a Deintensify hypoglycemia-causing medications (e.g., insulin, sulfonylureas, or meglitinides) or switch to a medication class with low hypoglycemia risk for individuals who are at high risk for hypoglycemia, using individualized glycemic goals. **B**

13.14b In older adults with diabetes, deintensify diabetes medications for individuals for whom the harms and/or burdens of treatment may be greater than the benefits, within individualized glycemic goals. **E**

Pharmacologic Therapy (continued)

13.14c Simplify complex treatment plans (especially insulin) to reduce the risk of hypoglycemia and polypharmacy and to decrease treatment burden. **B**

13.14d In older adults with type 2 diabetes and established or high risk of atherosclerotic cardiovascular disease, heart failure, and/or chronic kidney disease, the treatment plan should include agents that reduce cardiovascular and kidney disease risk, irrespective of glycemia. **A**

13.15 Consider costs of care and coverage when developing treatment plans to reduce risk of cost-related barriers to taking medication and performing self-management behaviors. **B**

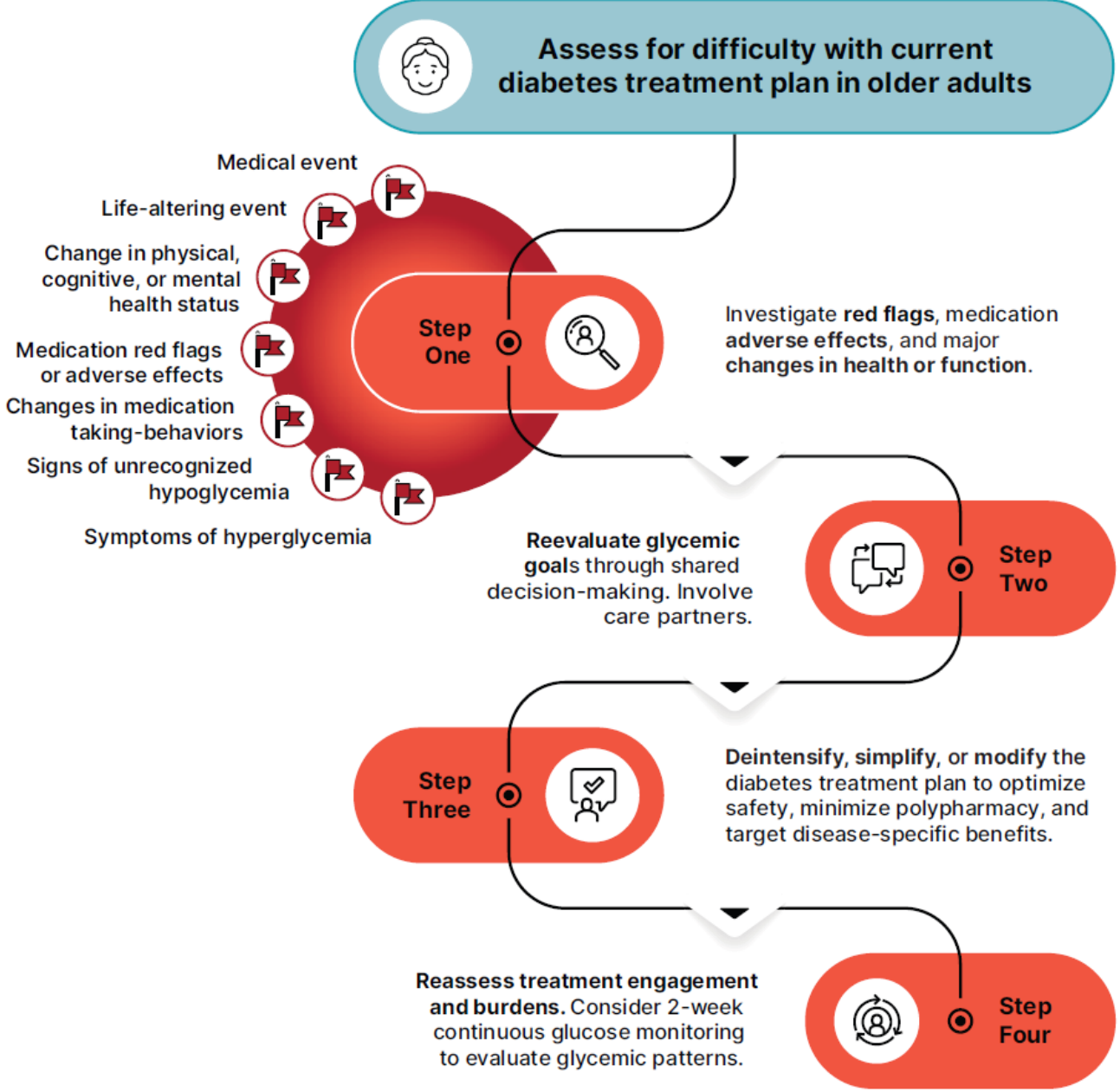


Figure 13.2
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Figure 13.2—Stepwise approach for assessing difficulties in the diabetes treatment plan; reevaluating glycemic goals through shared decision-making; deintensifying, simplifying, or modifying the treatment plan; and reassessing the safety and burdens of any interventions. Created using recommendations from Munshi et al. (41).

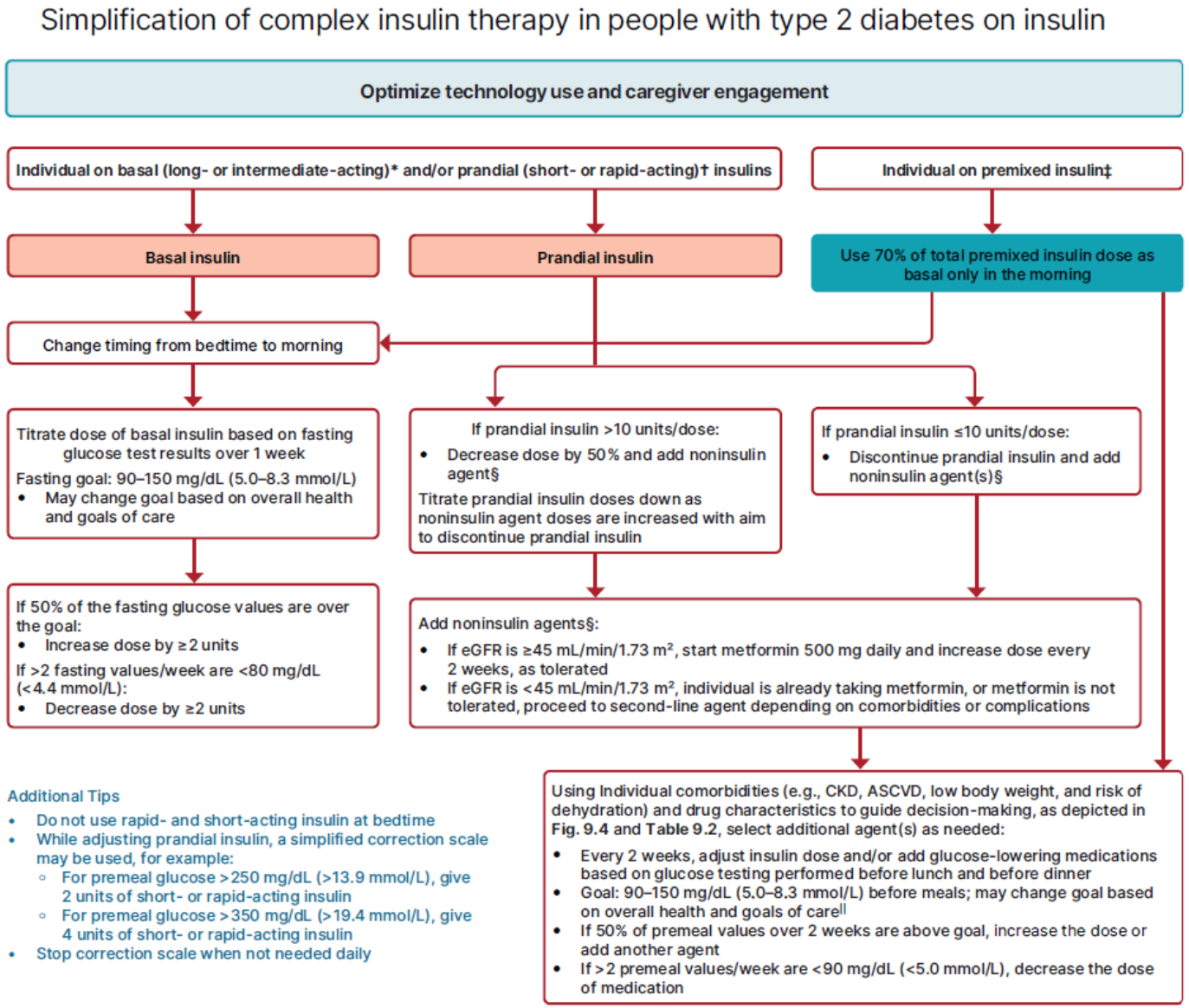


Figure 13.3—Algorithm to simplify insulin administration plans in older individuals. ASCVD, atherosclerotic cardiovascular disease; CKD, chronic kidney disease; eGFR, estimated glomerular filtration rate. *Basal insulins: glargine U-100 and U-300, degludec, and human NPH. †Prandial insulins: short-acting (regular human insulin) or rapid-acting (lispro, aspart, and glulisine). ‡Premixed insulins: 70/30, 75/25, and 50/50 products. §Examples of noninsulin agents include metformin, sodium-glucose cotransporter 2 inhibitors, dipeptidyl peptidase 4 inhibitors, glucagon-like peptide 1 receptor agonists (GLP1-RAs), and dual glucose-dependent insulinotropic polypeptide (GIP) and GLP-1 RAs. ||See **Table 13.2**. Adapted with permission from Munshi et al. (151).

Figure 13.3
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Table 13.3—Considerations for treatment plan simplification and deintensification/deprescribing in older adults with diabetes

Characteristics and health status of person with diabetes	Reasonable glycemic goal	Rationale/considerations	When may medication plan simplification be required?	When may treatment deintensification be required?
Healthy (few chronic illnesses, intact cognitive and functional status)	A1C <7.0–7.5% (<53–58 mmol/mol)	• Healthy individuals can usually perform complex tasks for glycemic management • During acute illness, individuals may be at risk for medication administration errors	• Severe or recurrent hypoglycemia on insulin regardless of A1C • Wide glucose excursions • Cognitive or functional decline following acute illness	• Severe or recurrent hypoglycemia on insulin or sulfonylureas, regardless of A1C • Wide glucose excursions • Polypharmacy
Complex/intermediate (multiple chronic illnesses or ≥2 ADL impairments or mild to moderate cognitive impairment)	A1C <8.0% (<64 mmol/mol)	• Comorbidities may affect self-management abilities and capacity to avoid hypoglycemia	• Severe or recurrent hypoglycemia on insulin therapy, regardless of A1C • Difficulty managing insulin plan • Significant change in social circumstances, (e.g., loss of care partner or change in living situation or finances)	• Severe or recurrent hypoglycemia on insulin or sulfonylureas, regardless of A1C • Wide glucose excursions • Polypharmacy

Table 13.3
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Community-dwelling individuals receiving short-term care in a skilled nursing facility	Avoid reliance on A1C; glucose goal 100–200 mg/dL (5.6–11.1 mmol/L)	• Glycemic management is important for recovery, wound healing, and avoidance of infections • Recovery from acute illness may impair cognitive function • More support may be needed on transition to home	• Consider reinstating prehospitalization treatment if it increased in complexity during hospitalization	• Weight loss, anorexia, short-term cognitive decline, and/or loss of physical functioning
Very complex/poor health (PALTC or end-stage chronic illnesses or moderate to severe cognitive impairment or ≥2 ADL impairments)	Avoid reliance on A1C and avoid hypoglycemia and symptomatic hyperglycemia	• No benefits of tight glycemic goals in this population • Hypoglycemia should be avoided • Most important outcomes are maintenance of cognitive and functional status	• The individual would like to decrease the number of injections and finger-stick blood glucose monitoring • The individual has an inconsistent eating pattern	• Cognitive dysfunction, depression, anorexia, or inconsistent eating pattern while taking sulfonylureas or meglitinides • Taking any diabetes medications without clear benefits

Table 13.3 (continued)
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

At the end of life	Avoid hypoglycemia and symptomatic hyperglycemia	• Goal is to provide comfort and avoid tasks or interventions that cause pain or discomfort • Care partners are important in providing medical care and maintaining quality of life	• Pain or discomfort caused by treatment (e.g., injections or finger sticks) • Excessive stress of care partners due to treatment complexity	• Taking any diabetes medications without clear benefits in improving symptoms and/or comfort

Treatment plan simplification refers to changing strategy to decrease the complexity of a medication plan (e.g., fewer administration times and fewer blood glucose checks) and decreasing the need for calculations (such as sliding-scale insulin calculations or insulin-carbohydrate ratio calculations). Deintensification/deprescribing refers to decreasing the dose or frequency of administration of a treatment or discontinuing a treatment altogether. ADL, activities of daily living; PALTC, post-acute and long-term care. Created using information from Munshi et al. 2016 (151) and 2017 (194).

Table 13.3 (continued)
Older Adults:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S277-S296

Treatment in Post-Acute and Long-Term Care Settings

13.16 Recommend diabetes education/training (including that for CGM devices, insulin pumps, and advanced insulin delivery systems) for the staff of post-acute and long-term care settings to improve the management of older adults with diabetes. **E**

13.17 People with diabetes residing in post-acute and long-term care settings need careful assessment of mobility, mentation, medications, and management preferences to establish individualized glycemic goals and to make appropriate choices of glucose-lowering agents and devices (including CGM devices, insulin pumps, and advanced insulin delivery systems) based on their clinical and functional status. **E**

For hypoglycemia, the following alert strategy could be considered:

1. Call a health care professional immediately in cases of low blood glucose levels (<70 mg/dL [<3.9 mmol/L]). However, treatment of hypoglycemia should not be delayed.
2. Call as soon as possible when
    a) glucose values are 70–100 mg/dL (3.9–5.6 mmol/L) (treatment plan may need to be adjusted),
    b) two or more blood glucose values >250 mg/dL (>13.9 mmol/L) are observed within a 24-h period accompanied by a significant change in clinical status,
    c) glucose values are consistently >250 mg/dL (>13.9 mmol/L) within a 24-h period,
    d) glucose values are consistently >300 mg/dL (>16.7 mmol/L) over 2 consecutive days,
    e) any reading is too high for the glucose monitoring device, or
    f) the individual is sick, with symptomatic hyperglycemia, vomiting, fever, lethargy, or poor oral intake

End-of-Life Care

13.18 When palliative care is needed in older adults with diabetes, health care professionals should discuss goals and intensity of care with people with diabetes and their care partners. Strict glucose, blood pressure, and lipid management are not necessary; consider deintensification or simplification of medication plans and prioritize symptom management. **E**

13.19 Prioritize the overall comfort, prevention of distressing symptoms, and preservation of quality of life and dignity as primary goals for diabetes management at the end of life. **C**

Different categories have been proposed for diabetes management in those at the end of life (77):

1. A stable individual: Continue with the person's previous medication plan, with a focus on 1) the prevention of hypoglycemia and 2) the management of hyperglycemia using blood glucose monitoring, keeping levels below the renal threshold of glucose, and hyperglycemia-mediated dehydration. There is no role for A1C monitoring.

2. An individual with organ failure: Preventing hypoglycemia is of greatest significance. Dehydration must be prevented and treated. In people with type 1 diabetes, insulin administration may be reduced as the oral intake of food decreases but should not be stopped. For those with type 2 diabetes, agents that may cause hypoglycemia should be reduced in dose. The main goal is to avoid hypoglycemia, allowing for glucose values in the upper level of the desired goal range.

3. A dying individual: For people with type 2 diabetes, the discontinuation of all medications may be a reasonable approach, as these individuals are unlikely to have any oral intake. In people with type 1 diabetes, there is no consensus, but a small amount of basal insulin may maintain glucose levels and prevent acute hyperglycemic complications and symptom burden.

Section 14.

Children and Adolescents

(https://doi.org/10.2337/dc26-S014)

Diabetes Self-Management Education and Support (Type 1)

14.1 Children and adolescents with diabetes, and their parents or caregivers (for individuals aged <18 years), should receive comprehensive, culturally sensitive, and developmentally appropriate individualized diabetes self management education and support according to reference standards at diagnosis and routinely thereafter. **B**

Nutrition Therapy

14.2 Provide individualized medical nutrition therapy for children and adolescents with prediabetes or diabetes, emphasizing key nutrition principles (i.e., more nonstarchy vegetables, whole fruits, legumes, whole grains, nuts and seeds, and low-fat dairy products and less sugar-sweetened beverages, sweets, meat, refined grains, and processed or ultraprocessed foods), **B** as an essential component of the overall treatment plan.

14.3 Monitor carbohydrate intake, whether by carbohydrate counting or experience-based estimation, as a key component to optimizing glycemic and weight management for children and adolescents with diabetes. **B**

14.4 Educate children and adolescents and their caregivers on the potential need to adjust insulin with high-fat and high-protein meals. Meal composition affects postprandial glucose excursions. **A**

14.5 In children and adolescents with diabetes, provide comprehensive nutrition education at diagnosis, and at least annually and ideally more frequently, by an experienced registered dietitian nutritionist to assess the eating pattern in relation to weight status, age-appropriate growth, risk for disordered eating behaviors, and cardiovascular disease risk factors. **E**

Physical Activity and Exercise

14.6 Physical activity is recommended for all children and adolescents with diabetes with the goal of 60 min of moderate- to vigorous-intensity aerobic activity daily, with vigorous muscle strengthening and bone-strengthening activities at least 3 days per week. **B**

14.7 Children and adolescents with diabetes using insulin therapy and their parents should monitor glucose levels before, during, and after physical activity and receive education about personalized glycemic goals according to the type and intensity of the planned physical activity. **C**

14.8 Children and adolescents with diabetes and their parents or caregivers should be educated on strategies to prevent hypoglycemia during, after, and overnight following physical activity or exercise. Treatment for hypoglycemia should be accessible before, during, and after engaging in activity. **C**

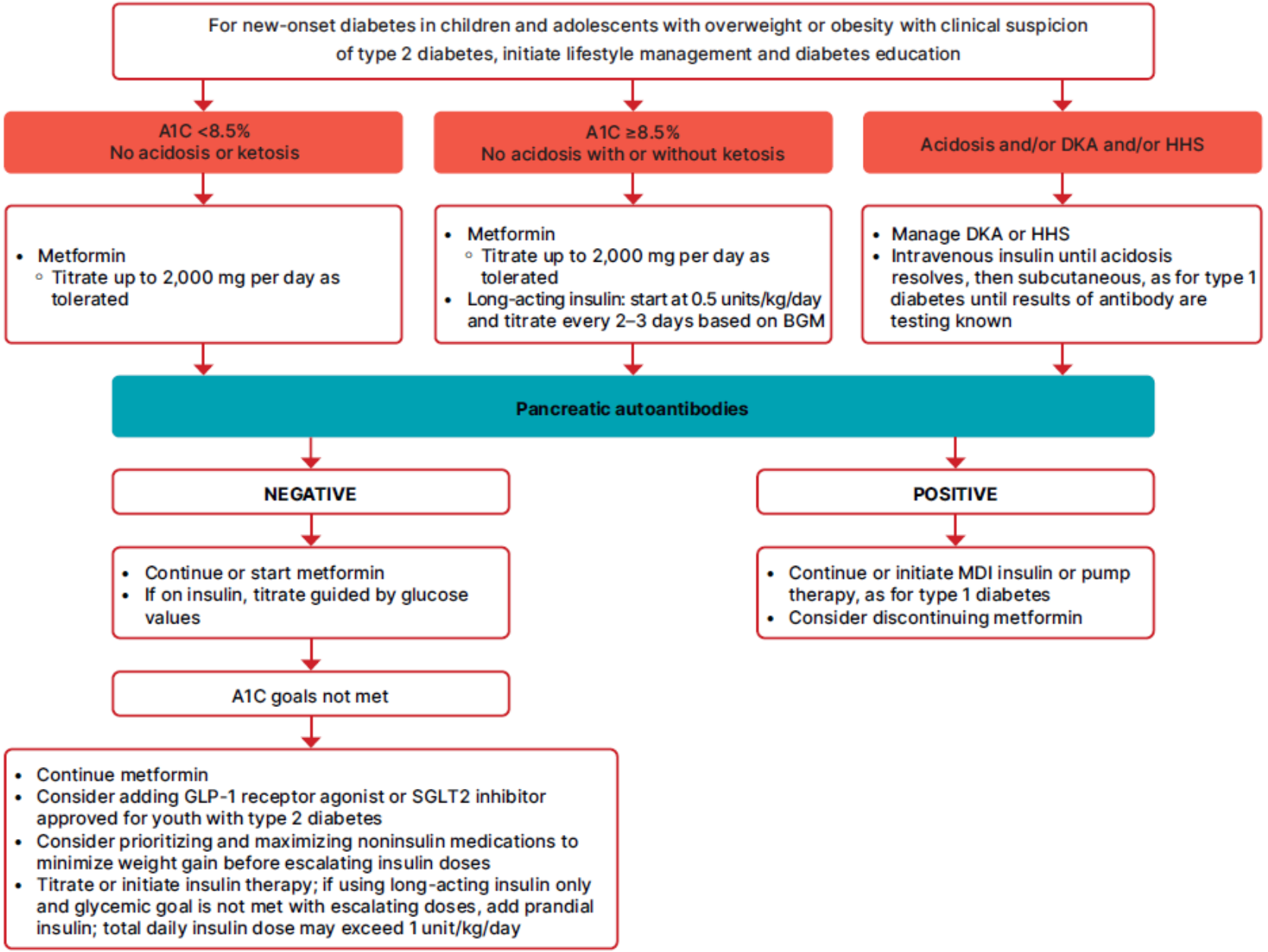


Figure 14.1
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Figure 14.1—Management of new-onset diabetes in youth with overweight or obesity with clinical suspicion of type 2 diabetes. A1C 8.5% = 69 mmol/mol. BGM, blood glucose monitoring; CGM, continuous glucose monitoring; DKA, diabetic ketoacidosis; GLP-1, glucagon-like peptide 1; HHS, hyperosmolar hyperglycemic state; MDI, multiple daily injections; SGLT2, sodium–glucose cotransporter 2. Adapted from the ADA position statement "Evaluation and Management of Youth-Onset Type 2 Diabetes" (3).

Psychosocial Care

14.9 At diagnosis and during routine follow-up care, screen children and adolescents with type 1 diabetes, **B** type 2 diabetes, **B** and other forms of diabetes **E** for psychosocial concerns (e.g., diabetes distress, depressive symptoms, anxiety, disordered eating behaviors), family factors, and behavioral health concerns that could affect diabetes management with age appropriate standardized and validated tools. Refer to a qualified behavioral health professional, preferably experienced in childhood diabetes, when indicated.

14.10 Behavioral health professionals should be considered integral members of the pediatric diabetes interprofessional team. **E**

14.11 Encourage developmentally appropriate family involvement in diabetes management tasks for children and adolescents with type 1 diabetes, type 2 diabetes, **B** and other forms of diabetes, **E** recognizing that premature or unsupportive transfer of diabetes care responsibility to children and adolescents can contribute to diabetes distress, lower engagement in diabetes self-management behaviors, and deterioration in glycemia.

Psychosocial Care (continued)

14.12 Health care professionals should screen for food insecurity, housing stability, health literacy, financial barriers, and social or community support and apply that information to treatment decisions. **E**

14.13 Health care professionals should consider asking school-aged children and adolescents with type 1 diabetes, **B** type 2 diabetes, **E** and other forms of diabetes, **E** along with their parents or caregivers, about social adjustment (peer relationships) and school performance to determine whether further intervention is needed.

14.14 Offer adolescents with diabetes time by themselves with their health care professional(s) at a developmentally appropriate age. **E**

14.15 During adolescence, generally during the stage of pubertal growth and development, incorporate reproductive health and preconception counseling into routine diabetes clinic visits for all individuals of childbearing potential because of the risk of adverse pregnancy outcomes in this population. **A**

Glycemic Monitoring, Insulin Delivery, and Goals

14.16 Continuous glucose monitoring (CGM) should be offered for diabetes management at diagnosis or as soon as possible in children and adolescents with diabetes who are capable of using the device safely (either by themselves or with caregivers). **A** The choice of device should be made based on the individual's and family's circumstances, desires, and needs.

14.17 Offer automated insulin delivery (AID) systems for diabetes management to children and adolescents with type 1 diabetes who are capable of using the device safely (either by themselves or with caregivers). Choice of device should be made based on the individual's and family's circumstances, desires, and needs. **A**

14.18 Offer open-loop insulin pump therapy for type 1 diabetes management to children and adolescents on multiple daily injections who are capable of using the device safely (either by themselves or with caregivers) if unable to use AID systems. Choice of device should be made based on the individual's and family's circumstances, desires, and needs. **A**

Glycemic Monitoring, Insulin Delivery, and Goals (continued)

14.19 Students with diabetes must be supported at school in the use of diabetes technology, including CGM, insulin pumps, connected insulin pens, and AID systems, as prescribed by their diabetes care team. **C**

14.20 A1C goals must be individualized and reassessed over time. An A1C of <7% (<53 mmol/mol) is appropriate for most children and adolescents with diabetes. **B**

14.21 Less stringent A1C goals (such as <7% [<53 mmol/mol] or <7.5% [<58 mmol/mol]) may be appropriate for children and adolescents with diabetes who cannot articulate symptoms of hypoglycemia; have hypoglycemia unawareness; cannot access advanced insulin delivery technology and/or CGM; cannot check blood glucose regularly; or have nonglycemic factors that increase A1C. **B**

14.22 Even less stringent A1C goals may be appropriate for children and adolescents with diabetes and a history of severe hypoglycemia or limited life expectancy or where the harms of treatment are greater than the benefits. **B**

Glycemic Monitoring, Insulin Delivery, and Goals (continued)

14.23 Health care professionals may reasonably suggest more stringent A1C goals (such as <6.5% [<48 mmol/mol]) for selected children and adolescents with diabetes if they can be achieved without significant hypoglycemia, excessive weight gain, negative impacts on well-being or mental health, or undue burden of care or in those who have nonglycemic factors that decrease A1C (e.g., lower erythrocyte life span). Lower goals may also be appropriate during the honeymoon phase. **B**

14.24 For children and adolescents with diabetes, CGM metrics derived from CGM use over the most recent 14 days (or longer) are recommended to be used in conjunction with or without A1C whenever possible. **B**

Screening and Diagnosis (Type 2)

14.25 Consider risk-based screening for prediabetes and/or type 2 diabetes after the onset of puberty or after 10 years of age, whichever occurs earlier, in children with overweight (BMI ≥85th to <95th percentile) or obesity (BMI ≥ 95th percentile) and who have one or more additional risk factor for diabetes (**see Table 2.5 for evidence grading** of other risk factors).

14.26 If screening is normal, repeat screening at a minimum of 2-year intervals **E** or more frequently if BMI is increasing. **C**

14.27 Fasting plasma glucose, 2-h plasma glucose during a 75-g oral glucose tolerance test, elevated random glucose with symptoms of hyperglycemia, and/or A1C can be used to test for prediabetes or type 2 diabetes in children and adolescents. **B**

14.28 Children and adolescents with overweight or obesity with hyperglycemia in whom the diagnosis of type 2 diabetes is being considered should have a panel of pancreatic autoantibodies tested to exclude the possibility of autoimmune type 1 diabetes. **B**

Management – Lifestyle (Type 2)

14.29 Provide children and adolescents with overweight or obesity and type 2 diabetes and their families with developmentally and culturally appropriate comprehensive lifestyle programs integrated with diabetes management to achieve at least a 7–10% decrease in excess weight. **B**

14.30 Given the necessity of long term weight management for children and adolescents with diabetes, lifestyle intervention should be based on a chronic care model and offered in the context of diabetes care. **E**

Management – Glycemic Goals (Type 2)

14.31 For children and adolescents with type 2 diabetes, glycemic status should be assessed at least every 3 months or as frequently as clinically indicated. **E**

14.32 Consider setting an A1C goal of <6.5% (<48 mmol/mol) for most children and adolescents with type 2 diabetes who have a low risk of hypoglycemia. For those at higher risk of hypoglycemia, A1C goals should be individualized as clinically appropriate. **C**

Pharmacologic Management (Type 2)

14.33 Initiate pharmacologic therapy with consideration of multiple add-on therapies early on, in addition to behavioral counseling for healthful nutrition and physical activity changes, at diagnosis of type 2 diabetes. **A**

14.34 In individuals with incidentally diagnosed type 2 diabetes (A1C <8.5% [<69 mmol/mol] and asymptomatic), metformin is the initial pharmacologic treatment of choice unless contraindicated by kidney function. **A**

14.35 Children and adolescents with marked hyperglycemia (A1C ≥8.5% [≥ 69 mmol/mol]) without acidosis at diagnosis should be treated initially with long-acting insulin while metformin is initiated and titrated. **B**

14.36 Initiate subcutaneous or intravenous insulin treatment in individuals with ketoacidosis to rapidly correct the hyperglycemia and the metabolic derangement. Once acidosis is resolved, metformin should be initiated while subcutaneous insulin therapy is continued. **A**

14.37 In individuals presenting with severe hyperglycemia (blood glucose ≥ 600 mg/dL [≥ 33.3 mmol/L]), consider assessment and treatment as needed for hyperglycemic hyperosmolar state. **A**

Pharmacologic Management (Type 2, continued)

14.38 If individualized glycemic goals are not achieved or maintained with metformin (with or without long-acting insulin), glucagon-like peptide 1 receptor agonist (GLP-1 RA) and/or sodium–glucose cotransporter 2 inhibitor(SGLT2i) should be considered in children and adolescents with type 2 diabetes of approved ages. **A**

14.39 For children and adolescents with type 2 diabetes not meeting individualized glycemic goals, consider maximizing noninsulin therapies (metformin, GLP-1 RA, and SGLT2i) before initiating and/or intensifying the insulin therapy plan. **E**

14.40 In individuals with type 2 diabetes initially treated with insulin and metformin and/or other glucose lowering medications who are meeting glucose goals based on blood glucose monitoring or CGM, reducing or discontinuing insulin should be considered. **B**

Metabolic Surgery (Type 2)

14.41 Consider metabolic surgery for the treatment of adolescents with type 2 diabetes who have class 2 obesity or higher (BMI 35 to <40 kg/m^2 or 120 to <140% of the 95th percentile for age and sex, whichever is lower) and who have elevated A1C and/or serious comorbidities despite lifestyle and pharmacologic intervention. **A**

14.42 Metabolic surgery for adolescents with type 2 diabetes should be performed only by an experienced surgeon working as part of a well organized and engaged interprofessional team, including a surgeon, endocrinologist, registered dietitian nutritionist, behavioral health specialist, and nurse. **A**

Prevention and Management of Diabetes Complications Among All Children and Adolescents With Diabetes

Recommendations **14.43–14.79**, regarding type 1 and type 2 diabetes complications, are presented in Table 14.1 to facilitate direct comparison.

Table 14.1

Table 14.1—Recommendations for diabetes-associated conditions in children and adolescents

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
Dyslipidemia	**14.43** For children and adolescents with type 1 diabetes, lipid screening should be performed soon after diagnosis, preferably after glycemia has improved and age is ≥2 years. If initial LDL cholesterol is ≤100 mg/dL (≤2.6 mmol/L), subsequent testing should be performed at 9–11 years of age B and repeated every 3 years. E	**14.44** In children and adolescents with type 2 diabetes, lipid screening should be performed soon after diagnosis, preferably after glycemia has improved and annually thereafter. B	**14.45** The LDL cholesterol goal is <100 mg/dL (<2.6 mmol/L). E **14.46** In children and adolescents with diabetes, if lipids are abnormal, initial therapy should consist of optimizing glycemia and medical nutritional therapy to limit calories from fat to 25–30% and saturated fat to <7%, limit cholesterol to <200 mg/day, avoid *trans* fats, and aim for ~10% calories from monounsaturated fats for elevated LDL. For elevated triglycerides, MNT should also focus on decreasing carbohydrate intake and increasing foods rich in n-3 fatty acids in addition to the above changes. A **14.47** In children and adolescents with diabetes, if LDL cholesterol remains >130 mg/dL (>3.4 mmol/L) after 6 months of nutrition intervention, initiate therapy with a statin, with a goal of LDL <100 mg/dL (<2.6 mmol/L). Due to the potential teratogenic effects, individuals of childbearing potential should receive reproductive counseling, and statins should be avoided in individuals of childbearing potential who are not using reliable contraception. B **14.48** In children and adolescents with diabetes, if triglycerides are >400 mg/dL (>4.7 mmol/L) fasting or >1,000 mg/dL (>11.6 mmol/L) nonfasting, optimize glycemia and begin fibrate, with a goal of <400 mg/dL (<4.7 mmol/L) fasting to reduce risk for pancreatitis. C

Table 14.1, continued

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
Hypertension			**14.49** In children and adolescents with diabetes, BP should be measured at every clinic visit. In children and adolescents with high BP (BP ≥90th percentile for age, sex, and height or, in adolescents aged ≥13 years, ≥120/80 mmHg) on three separate measurements, ambulatory BP monitoring should be strongly considered. B **14.50** Excess weight increases cardiovascular event rates among people with diabetes and should be addressed with MNT, intensive lifestyle interventions focusing on dyslipidemia, hypertension, hyperglycemia along with adjunct pharmacotherapy, and/or bariatric surgery as appropriate. C **14.51** In children and adolescents with diabetes, after excluding secondary hypertension, treatment of elevated BP (defined as 90th to <95th percentile for age, sex, and height or, in adolescents aged ≥13 years, 120–129/<80 mmHg) is lifestyle modification focused on healthy nutrition, physical activity, sleep, and, if appropriate, weight management. C **14.52** For children and adolescents with diabetes, after excluding other causes, in addition to lifestyle modification, ACE inhibitors or ARBs should be started for treatment of

Continued on p. S309

Table 14.1, continued
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Table 14.1, continued

Table 14.1—Continued

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
			confirmed hypertension (defined as BP consistently ≥95th percentile for age, sex, and height or, in adolescents aged ≥13 years, BP ≥130/80 mmHg). Due to the potential teratogenic effects, individuals of childbearing potential should receive reproductive counseling, and ACE inhibitors and ARBs should be avoided in individuals of childbearing potential who are not using reliable contraception. B
			14.53 For children and adolescents the goal of hypertension treatment is BP <90th percentile for age, sex, and height or, in adolescents aged ≥13 years, BP <130/80 mmHg. C
Nephropathy	**14.54** For children and adolescents with type 1 diabetes, nephropathy screening should be obtained at puberty or at age ≥11 years, whichever is earlier, once the youth have had diabetes for 5 years and annually thereafter. B	**14.55** In children and adolescents with type 2 diabetes, nephropathy screening should be performed at the time of diagnosis and annually thereafter. An elevated UACR (>30 mg/g creatinine) should be confirmed on two of three samples. B	**14.56** In children and adolescents with diabetes, nephropathy screening should be performed with a random spot urine sample. Consider obtaining a morning sample if there are effects of exercise or orthostatic changes. An elevated UACR (>30 mg/g creatinine) should be confirmed on two of three samples over a 6-month period. B
			14.57 Determine eGFR at the time of diagnosis and annually thereafter. E

Table 14.1, continued
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Table 14.1, continued

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
			14.58 In nonpregnant children and adolescents with diabetes, either an ACE inhibitor or an ARB is recommended for those with moderately increased albuminuria (UACR 30–299 mg/g creatinine) B and is strongly recommended for those with severely increased albuminuria (UACR ≥300 mg/g creatinine) and/or eGFR <60 mL/min/1.73 m^2 to maximally tolerated dose to prevent the progression of kidney disease and reduce cardiovascular events. A If one class is not tolerated, the other should be substituted. B Due to the potential teratogenic effects, individuals of childbearing potential should receive reproductive counseling, and ACE inhibitors and ARBs should be avoided in individuals of childbearing potential who are not using reliable contraception. B **14.59** For children and adolescents with nephropathy, continue monitoring (every 3–6 months and/or as indicated by UACR and eGFR) to detect disease progression. E **14.60** Refer to nephrology in case of uncertainty of etiology, worsening UACR, or decrease in eGFR. E
Retinopathy	**14.61** An initial dilated and comprehensive diabetes eye examination is recommended once youth have had type 1 diabetes for 3–5 years, provided they are aged ≥11 years or puberty has started, whichever is earlier. B	**14.62** Retinopathy screening in children and adolescents with type 2 diabetes should be performed at or soon after diagnosis and annually thereafter. C	**14.63** Optimizing glycemia is recommended to decrease the risk or slow the progression of retinopathy. B

Continued on p. S310

Table 14.1, continued
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Table 14.1, continued

Table 14.1—Continued

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
	14.64 For children and adolescents with type 1 diabetes, after the initial examination, repeat dilated and comprehensive eye examination or retinal photography is recommended every 2 years. Less frequent examinations, every 4 years, may be acceptable on the advice of an eye care professional and based on risk factor assessment, including a history of A1C <8% (<64 mmol/mol). B	**14.65** Less frequent examination (every 2 years) using dilated eye examination or retinal photography may be considered if achieving glycemic goals and a normal eye exam. C	**14.66** Programs that use nondilated retinal photography (with remote reading or use of a validated assessment tool) can be appropriate screening strategies for diabetic retinopathy. B
Neuropathy	**14.67** For children and adolescents with type 1 diabetes, consider an annual comprehensive foot exam at the start of puberty or at age ≥11 years, whichever is earlier, once the youth have had type 1 diabetes for 5 years. The examination should include inspection, assessment of foot pulses, pinprick, 10-g monofilament sensation tests, testing of vibration sensation using a 128-Hz tuning fork, and ankle reflex tests. B	**14.68** Screen children and adolescents with type 2 diabetes for the presence of neuropathy by foot examination at diagnosis and annually. The examination could include inspection, assessment of foot pulses, pinprick, 10-g monofilament sensation tests, testing of vibration sensation using a 128-Hz tuning fork, and ankle reflex tests. C	
MASLD		**14.69** Evaluate children and adolescents with type 2 diabetes for MASLD (by measuring AST and ALT) at diagnosis and annually thereafter. B **14.70** Consider referral to gastroenterology for persistently elevated or worsening transaminases. B	

Table 14.1, continued

Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
Obstructive sleep apnea		**14.71** In children and adolescents with diabetes, screening for symptoms of sleep apnea should be done at least annually, and referral to a pediatric sleep specialist is recommended for evaluation and a polysomnogram, if indicated. Obstructive sleep apnea should be treated when documented. B
Polycystic ovary syndrome		**14.72** Evaluate for polycystic ovary syndrome in adolescent girls with diabetes, including laboratory studies, when clinically indicated. B **14.73** Metformin, in addition to lifestyle modification, is likely to improve the menstrual cyclicity and hyperandrogenism in adolescent girls with diabetes. E
Cardiovascular disease		**14.74** Excessive weight gain increases cardiovascular event rates among children and adolescents with diabetes and should be addressed with lifestyle and obesity pharmacotherapy as appropriate. C

Continued on p. S311

Table 14.1, continued
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Table 14.1, continued

Table 14.1—Continued

	Type 1 diabetes recommendations	Type 2 diabetes recommendations	Type 1 and 2 diabetes recommendations
Autoimmune conditions	**14.75** In children and adolescents with type 1 diabetes, assess for autoimmune conditions outside of critical illness if clinically indicated. B		
Thyroid disease	**14.76** In children and adolescents with type 1 diabetes, measure thyroid-stimulating hormone concentrations at diagnosis when clinically stable or soon after optimizing glycemia. If normal, suggest rechecking every 1–2 years or sooner if the individual has positive thyroid antibodies or develops symptoms or signs suggestive of thyroid dysfunction, thyromegaly, an abnormal growth rate, or unexplained glycemic variability. B		
Celiac disease	**14.77** Screen children and adolescents with type 1 diabetes outside of critical illness for celiac disease by measuring IgA tissue tTG antibodies, with documentation of acceptable serum IgA levels for the local assay, soon after the diagnosis of diabetes, or IgG tTG and deamidated gliadin antibodies if IgA is deficient. B **14.78** Repeat screening for celiac disease within 2 years of type 1 diabetes diagnosis and then again after 5 years and consider more frequent screening in children and adolescents who have symptoms or a first-degree relative with celiac disease. B **14.79** Prescribe a gluten-free eating pattern for children and adolescents with confirmed celiac disease to avoid nutritional complications. Refer children and adolescents and their caregivers to a registered dietitian nutritionist experienced in managing both diabetes and celiac disease. B		

ARB, angiotensin receptor blocker; BP, blood pressure; eGFR, estimated glomerular filtration rate; MASLD, metabolic dysfunction–associated steatotic liver disease; MNT, medical nutrition therapy; tTG, tissue transglutaminase; UACR, urinary albumin-to-creatinine ratio.

Table 14.1, continued
Children and Adolescents:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S297-S320

Substance Use Among All Children and Adolescents With Diabetes – Tobacco, Electronic Cigarettes, Alcohol, and Cannabis

14.80 Screen children and adolescents with diabetes for tobacco/nicotine vaping, substance use, and alcohol use at diagnosis and regularly thereafter, **C** discourage use and provide appropriate referrals to cessation programs as needed. **A**

14.81 Advise all children, adolescents, and young adults with diabetes not to use cannabis recreationally in any form. **E**

Transition from Pediatric to Adult Care

14.82 Diabetes care teams should implement transition preparation programs beginning in early adolescence and, at the latest, at least 1 year before the anticipated transfer from pediatric to adult health care. **E**

14.83 Interprofessional adult and pediatric health care teams should provide support and resources for adolescents, young adults, and their families prior to and during the transfer process from pediatric to adult health care. **C**

14.84 Pediatric diabetes specialists should partner with adolescents with diabetes and their caregivers to engage in shared decision-making for the timing of transfer to an adult diabetes specialist. There is no age specific cutoff for young people with diabetes to transfer to an adult diabetes specialist. **E**

Section 15

Management of Diabetes in Pregnancy

(https://doi.org/10.2337/dc26-S015)

Preconception Counseling

15.1 Starting at puberty and continuing in all people with diabetes and childbearing potential, preconception counseling should be incorporated into routine diabetes care. **A**

15.2 Family planning should be discussed, and effective contraception (with consideration of long-acting, reversible contraception) should be prescribed and used until an individual's treatment plan and A1C are optimized for pregnancy. **A**

15.3 Preconception counseling should address the importance of achieving glucose levels as close to normal as is safely possible without excessive hypoglycemia, ideally A1C <6.5% (<48mmol/mol), to reduce the risk of congenital anomalies, preeclampsia, macrosomia, preterm birth, and other complications. **A**

15.4 Individuals with a history of gestational diabetes mellitus (GDM) should seek preconception screening for diabetes and preconception care to identify and treat hyperglycemia and prevent congenital malformations and other adverse maternal and fetal outcomes. **E**

Preconception Care

15.5 Individuals with preexisting diabetes who are planning a pregnancy should ideally begin receiving interprofessional care prior to conception, which includes an endocrinology health care professional, maternal-fetal medicine specialist, registered dietitian nutritionist, and diabetes care and education specialist, when available. **B**

15.6 In addition to focused attention on achieving glycemic goals, **A** standard preconception care should be augmented with extra focus on nutrition, physical activity, diabetes self-care education, and screening for diabetes comorbidities and complications. **B**

15.7 Individuals with preexisting diabetes who are planning a pregnancy or who have become pregnant should be counseled on the risk of development and/or progression of diabetic retinopathy. Dilated eye examinations should occur ideally before pregnancy as well as in the first trimester, and then pregnant individuals should be monitored every trimester and for 1 year postpartum as indicated by the degree of retinopathy and as recommended by the eye care health care professional. **B**

Table 15.1—Checklist for preconception care for people with prediabetes, diabetes, or a history of gestational diabetes mellitus

Preconception education should include:

- ☐ Comprehensive nutrition assessment and recommendations for:
  - Overweight and obesity or underweight
  - Meal planning
  - Correction of nutritional deficiencies
  - Caffeine intake
  - Safe food preparation technique
- ☐ Lifestyle recommendations for:
  - Regular moderate exercise
  - Avoidance of hyperthermia (hot tubs)
  - Adequate sleep
- ☐ Comprehensive diabetes self-management education
- ☐ Counseling on diabetes in pregnancy per current standards, including natural history of insulin resistance in pregnancy and postpartum; preconception glycemic goals; avoidance of DKA or severe hyperglycemia; avoidance of severe hypoglycemia; progression of retinopathy in individuals with preexisting diabetes; PCOS (if applicable); fertility in people with diabetes; genetics of diabetes; risks to pregnancy including miscarriage, stillbirth, congenital malformations, macrosomia, preterm labor and delivery, hypertensive disorders in pregnancy
- ☐ Supplementation
  - Folic acid supplement (400–800 µg/day routine)
  - Appropriate use of over-the-counter medications and supplements

Health assessment and plan should include:

- ☐ General evaluation of overall health
- ☐ Evaluation of diabetes and its comorbidities and complications, including DKA or severe hyperglycemia; severe hypoglycemia/hypoglycemia unawareness; barriers to care; comorbidities such as hyperlipidemia, hypertension, MASLD, PCOS, and thyroid dysfunction; complications such as macrovascular disease in individuals with preexisting diabetes, nephropathy, neuropathy (including autonomic bowel and bladder dysfunction), and retinopathy
- ☐ Evaluation of obstetric or gynecologic history, including a history of cesarean section, congenital malformations or fetal loss, current methods of contraception, hypertensive disorders of pregnancy, postpartum hemorrhage, preterm delivery, previous macrosomia, Rh incompatibility, and thrombotic events (DVT/PE)
- ☐ Review of current medications and appropriateness during pregnancy

Screening should include:

- ☐ Diabetes complications and comorbidities, including comprehensive foot exam; comprehensive ophthalmologic exam; ECG in individuals starting at age 35 years who have cardiac signs or symptoms or risk factors and, if abnormal, further evaluation; lipid panel; serum creatinine; TSH; and urine albumin-to-creatinine ratio
- ☐ Anemia
- ☐ Genetic carrier status (based on history):
  - Cystic fibrosis
  - Sickle cell anemia
  - Tay-Sachs disease
  - Thalassemia
  - Others if indicated
- ☐ Infectious disease (per ACOG guidelines)

Preconception plan should include:

- ☐ Immunizations (per ACOG guidelines) (203–205)
- ☐ Nutrition and medication plan to achieve glycemic goals prior to conception, including appropriate implementation of blood glucose monitoring, continuous glucose monitoring (if indicated and appropriate), and pump technology (if indicated and appropriate)
- ☐ Contraceptive plan to prevent pregnancy until glycemic goals are achieved
- ☐ Management plan for general health, gynecologic concerns, comorbid conditions, or complications, if present, including hypertension, nephropathy, retinopathy; Rh incompatibility; and thyroid dysfunction

Created using information from American College of Obstetricians and Gynecologists (ACOG) (5) and others (28,30). DKA, diabetic ketoacidosis; DVT/PE, deep vein thrombosis/pulmonary embolism; ECG, electrocardiogram; MASLD, metabolic dysfunction-associated steatotic liver disease; PCOS, polycystic ovary syndrome; TSH, thyroid-stimulating hormone.

Table 15.1
Management of Diabetes in Pregnancy:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S321-S338

Glycemic Goals in Pregnancy

15.8 Fasting, preprandial, and postprandial blood glucose monitoring are recommended in individuals with diabetes in pregnancy to achieve optimal glucose levels. Glucose goals are fasting plasma glucose <95 mg/dL (<5.3 mmol/L) and either 1-h postprandial glucose <140 mg/dL (<7.8 mmol/L) or 2-h postprandial glucose <120 mg/dL (<6.7 mmol/L). **B**

15.9 Due to increased red blood cell turnover, A1C is slightly lower during pregnancy in people with and without diabetes. Ideally, the A1C goal in pregnancy is <6% (<42 mmol/mol) if this can be achieved without significant hypoglycemia, but the goal may be relaxed to <7% (<53 mmol/mol) if necessary to prevent hypoglycemia. **B**

15.10 Continuous glucose monitoring (CGM) can help to achieve glycemic goals (e.g., time in range, time above range) **A** and A1C goal **B** in type 1 diabetes and pregnancy and may be beneficial for other types of diabetes in pregnancy. **E**

Glycemic Goals in Pregnancy

15.11 Recommend CGM to pregnant individuals with type 1 diabetes. **A** In conjunction with aims to achieve traditional pre- and postprandial glycemic goals, CGM can reduce the risk for large-for-gestational-age infants and neonatal hypoglycemia in pregnancy complicated by type 1 diabetes. **A**

15.12 CGM metrics may be used in combination with blood glucose monitoring to achieve optimal pre- and postprandial glycemic goals. **E**

15.13 Commonly used estimated A1C and glucose management indicator calculations should not be used in pregnancy as estimates of A1C. **C**

Table 15.2—Blood glucose goals in pregnancies associated with diabetes

	Blood glucose goal		
Glucose measurement	Type 1 diabetes or type 2 diabetes^	GDM treated with insulin	GDM not treated with insulin
Fasting glucose	70–95 mg/dL (3.9–5.3 mmol/L)	70–95 mg/dL (3.9–5.3 mmol/L)	<95 mg/dL (<5.3 mmol/L)
1-h postprandial glucose	110–140 mg/dL* (6.1–7.8 mmol/L)	110–140 mg/dL* (6.1–7.8 mmol/L)	<140 mg/dL* (<7.8 mmol/L)
2-h postprandial glucose	100–120 mg/dL (5.6–6.7 mmol/L)	100–120 mg/dL (5.6–6.7 mmol/L)	<120 mg/dL (<6.7 mmol/L)

Gestational diabetes mellitus (GDM) blood glucose goals shown are recommended by the Fifth International Workshop-Conference on Gestational Diabetes Mellitus (45). ^Lower glucose limits do not apply to individuals with type 2 diabetes treated with nutrition alone. Aim for less stringent goals if these cannot be achieved without significant hypoglycemia, based on clinical experience and individualization of care. *Optimal goal includes either a 1-h postprandial glucose level or 2-h postprandial glucose level within column of type of diabetes.

Table 15.2
Management of Diabetes in Pregnancy:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S321-S338

Management of Diabetes in Pregnancy

15.14 Nutrition counseling before and during pregnancy should promote an eating pattern including fruits, vegetables, legumes, whole grains, nuts, seeds, fish, and other lean protein, which will provide a balance of macronutrients and healthy n-3 fatty acids. **C**

15.15 Lifestyle behavior change is an essential component of management of GDM and may suffice as treatment for many individuals. Insulin should be added if needed to achieve glycemic goals. **A**

15.16 Telehealth visits used in combination with in-person visits for pregnant people with GDM can improve outcomes compared with standard in person care alone. **A**

15.17 Insulin should be used to manage type 1 diabetes in pregnancy **A** and is the preferred agent for the management of GDM **A** and type 2 diabetes in pregnancy. **B**

15.18 Either multiple daily injections or insulin pump technology can be used in pregnancy complicated by type 1 diabetes. **C**

Management of Diabetes in Pregnancy (continued)

15.19 Automated insulin delivery (AID) systems with pregnancy-specific glucose targets are recommended for pregnant individuals with type 1 diabetes. **A**

15.20 AID systems without pregnancy-specific glucose targets or a pregnancy-specific algorithm may be considered for select pregnant individuals with type 1 diabetes when used with assistive techniques and working with experienced health care teams. **B**

15.21 Metformin and glyburide, individually or in combination, should not be used as first-line agents for management of diabetes in pregnancy, as both cross the placenta to the fetus **A** and may not be sufficient to achieve glycemic goals. **B** Other oral and noninsulin injectable glucose-lowering medications lack long-term safety data and are not recommended. **E**

15.22 Metformin, when used to treat polycystic ovary syndrome and induce ovulation, should be discontinued by the end of the first trimester. **A**

Preeclampsia and Aspirin

15.23 Pregnant individuals with type 1 or type 2 diabetes should be prescribed low-dose aspirin 100–150 mg/day starting at 12–16weeks of gestation to lower the risk of preeclampsia. **E** A dosage of 162 mg/day may be acceptable; **E** currently, in the U.S., low-dose aspirin is available in 81-mg tablets.

Pregnancy and Non-Glucose Lowering Drug Considerations

15.24 In pregnant individuals with diabetes and chronic hypertension, a blood pressure threshold <140/90mmHg is recommended for initiation and titration of therapy for better pregnancy. **A** Therapy should be deintensified if blood pressure is <90/60 mmHg. **E**

15.25a Potentially harmful medications in pregnancy (e.g., ACE inhibitors, angiotensin receptor blockers, mineralocorticoid receptor antagonists) should be stopped prior to conception and avoided in sexually active individuals of childbearing potential who are not using reliable contraception. **B**

15.25b In most circumstances, lipid-lowering medications should be stopped prior to conception and avoided in sexually active individuals of childbearing potential with diabetes who are not using reliable contraception. **B** In some circumstances (familial hypercholesterolemia, severe hypertriglyceridemia, prior atherosclerotic cardiovascular disease event), lipid-lowering therapy may be continued when the benefits outweigh risks. **E**

Postpartum Care

15.26 Insulin requirements need to be evaluated and adjusted for individuals requiring insulin after delivery because insulin resistance decreases dramatically immediately postpartum. **C**

15.27 A contraceptive plan should be discussed and implemented with all people with diabetes of childbearing potential. **A**

15.28 Breastfeeding efforts are recommended for all individuals with diabetes. **A** Breastfeeding is recommended for individuals with a history of GDM for multiple benefits, **A** including a reduced risk for type 2 diabetes later in life. **B**

15.29 Postpartum care should include psychosocial assessment and support for self-care. **E**

15.30 Screen individuals with a recent history of GDM at 4–12 weeks postpartum, using the 75-g oral glucose tolerance test and clinically appropriate nonpregnancy diagnostic criteria. **B**

15.31 Individuals with a history of GDM should have lifelong screening for the development of type 2 diabetes or prediabetes every 1–3 years. **B**

15.32 Individuals with overweight or obesity and a history of GDM found to have prediabetes should receive intensive lifestyle interventions and/or metformin to prevent diabetes. **A**

Section 16.

Diabetes Care in the Hospital

(https://doi.org/10.2337/dc26-S016)

Hospital Care Delivery Standards

16.1 Perform an A1C test on all people with diabetes or hyperglycemia (random blood glucose >140 mg/dL [>7.8 mmol/L]) at the time of admission to the hospital if no A1C test result is available from the prior 3 months. **B**

16.2 Institutions should implement protocols using validated written or computerized provider order entry sets for management of dysglycemia in the hospital that allow for a personalized approach. **B**

Diabetes Care Specialists in the Hospital

16.3 When caring for hospitalized people with diabetes (with an existing or new diagnosis) or stress hyperglycemia, consult with a specialized diabetes or glucose management team when available. **B**

Glycemic Goals in Hospitalized Adults

16.4a Insulin should be initiated or intensified for treatment of persistent hyperglycemia starting at a threshold of ≥180 mg/dL (≥10.0 mmol/L) (confirmed on two occasions within 24 h) for the majority of critically ill individuals (those in the intensive care unit [ICU]). **A**

16.4b Insulin and/or other glucose lowering therapies should be initiated or intensified for treatment of persistent hyperglycemia starting at a threshold of ≥180 mg/dL (≥10.0 mmol/L) (confirmed on two occasions within 24 h) for the majority of noncritically ill individuals (those not in the ICU). **B**

16.5a Once therapy is initiated, a glycemic goal of 140–180 mg/dL (7.8–10.0 mmol/L) is recommended for most critically ill individuals (those in the ICU) with hyperglycemia. **A** More stringent individualized glycemic goals may be appropriate for selected critically ill individuals if they can be achieved without significant hypoglycemia. **B**

16.5b For noncritically ill individuals (those not in the ICU), a glycemic goal of 100–180 mg/dL (5.6–10.0 mmol/L) is recommended if it can be achieved without significant hypoglycemia. **B**

Continuous Glucose Monitoring

16.6 In people with diabetes using a personal continuous glucose monitoring (CGM) device, the use of CGM should be continued during hospitalization if clinically appropriate, with confirmatory point-of-care (POC) blood glucose measurements for insulin dosing decisions and hypoglycemia assessment, if resources and training are available, and according to an institutional protocol. **B**

Insulin Pump use in the Hospital

16.7 Continue use of insulin pump including automated insulin delivery in people with diabetes who are hospitalized when clinically appropriate. This is contingent upon availability of necessary supplies, resources, training, ongoing competency assessments, and implementation of institutional diabetes technology protocols. **C**

Insulin Therapy

16.8a Continuous intravenous insulin infusion is recommended for achieving glycemic goals and avoiding hypoglycemia in critically ill individuals. **A**

16.8b Basal insulin or a basal plus correction insulin plan is the preferred treatment for noncritically ill hospitalized individuals with poor or no oral intake. **A**

16.9 An insulin plan with basal, prandial, and correction components is the preferred treatment for most noncritically ill hospitalized individuals with adequate nutritional intake. **A**

16.10 For most individuals, sole use of a correction or supplemental insulin without basal insulin (formerly referred to as a sliding scale) in the inpatient setting is discouraged. **A**

Noninsulin Therapies

16.11 It is recommended that use of a sodium–glucose cotransporter 2 inhibitor be initiated or continued during hospitalization if indicated for heart failure, providing there are no contraindications. **A**

Hypoglycemia

16.12 A hypoglycemia management protocol should be adopted by all health systems. A plan for identifying, treating, and preventing hypoglycemia should be established for each individual. Episodes of hypoglycemia in the hospital should be documented in the health record and tracked to inform quality improvements. **C**

16.13 Treatment plans should be reviewed and changed as necessary to prevent hypoglycemia and recurrent hypoglycemia when a blood glucose value of <70 mg/dL (<3.9 mmol/L) is documented. **C**

Perioperative Care

16.14 To improve postoperative outcomes after elective surgery, a preoperative A1C goal <8% (<64 mmol/mol) is recommended within 3 months, with individualized risk-to-benefit ratio assessment. **C** The 14-day glucose management indicator goal <8% and/or time in range >50% can also be used. **E**

16.15 Blood glucose before, during, and after surgery should be monitored and maintained between 100 and 180 mg/dL (5.6 and 10.0 mmol/L). Goals may differ depending on the surgery, risk for hypoglycemia, and glucose-lowering therapy. **C**

In individuals undergoing noncardiac general surgery, basal insulin plus premeal short- or rapid-acting insulin (basal-bolus) coverage has been associated with improved glycemic outcomes and lower rates of perioperative complications compared with the reactive, correction-only short- or rapid-acting insulin coverage alone with no basal insulin dosing (74,145). In addition to glycemic goals, the following points need attention:

1.Stricter perioperative glycemic goals are not advised because they may not improve outcomes and are associated with increased hypoglycemia (146). 2. Blood glucose should be monitored at least every 2–4 h while the individual takes nothing by mouth, and insulin should be administered as needed. Insulin is the only recommended glucose-lowering medication in the perioperative period.

3. CGM should not be used alone for glucose monitoring during surgery (147).

4. Metformin and other oral glucose lowering agents should be held on the day of surgery or procedure. See below for SGLT2 inhibitor and GLP-1 RA use in the perioperative period.

5. Individuals using an insulin pump may continue pump use during surgery if this is consistent with institutional policies and surgical procedures (148). This would require an interprofessional approach and adequate support systems in the hospital. If an insulin pump cannot be used during surgery, an alternative plan (insulin infusion or basal plus correctional insulin) should be initiated before surgery. Compared with usual dosing, a reduction of 25% of basal insulin dose given the evening before surgery is more likely to achieve perioperative blood glucose goals with a lower risk for hypoglycemia (149). Insulin dose reductions also include NPH insulin to one-half of the dose or long-acting basal insulin analogs to 75–80% of the dose or adjustment of insulin pump (if not in automated mode) basal rates based on the type of diabetes and clinical judgment. However, the decision needs to be individualized, as the dose reduction may not be appropriate for some people with type 1 diabetes.

Diabetic Ketoacidosis and Hyperglycemic Hyperosmolar State

16.16 Manage diabetic ketoacidosis (DKA) and hyperglycemic hyperosmolar state (HHS) by administering intravenous fluids, insulin, and electrolytes.(Fig. 16.1) and by closely monitoring during treatment, ensuring timely and bridged transition to maintenance subcutaneous insulin administration, and identifying and treating the precipitating cause. **A**

16.17 The discharge planning process should include education on the recognition, prevention, and management of DKA and/or HHS for all individuals affected by or at high risk for these events to prevent recurrence and readmission. **B**

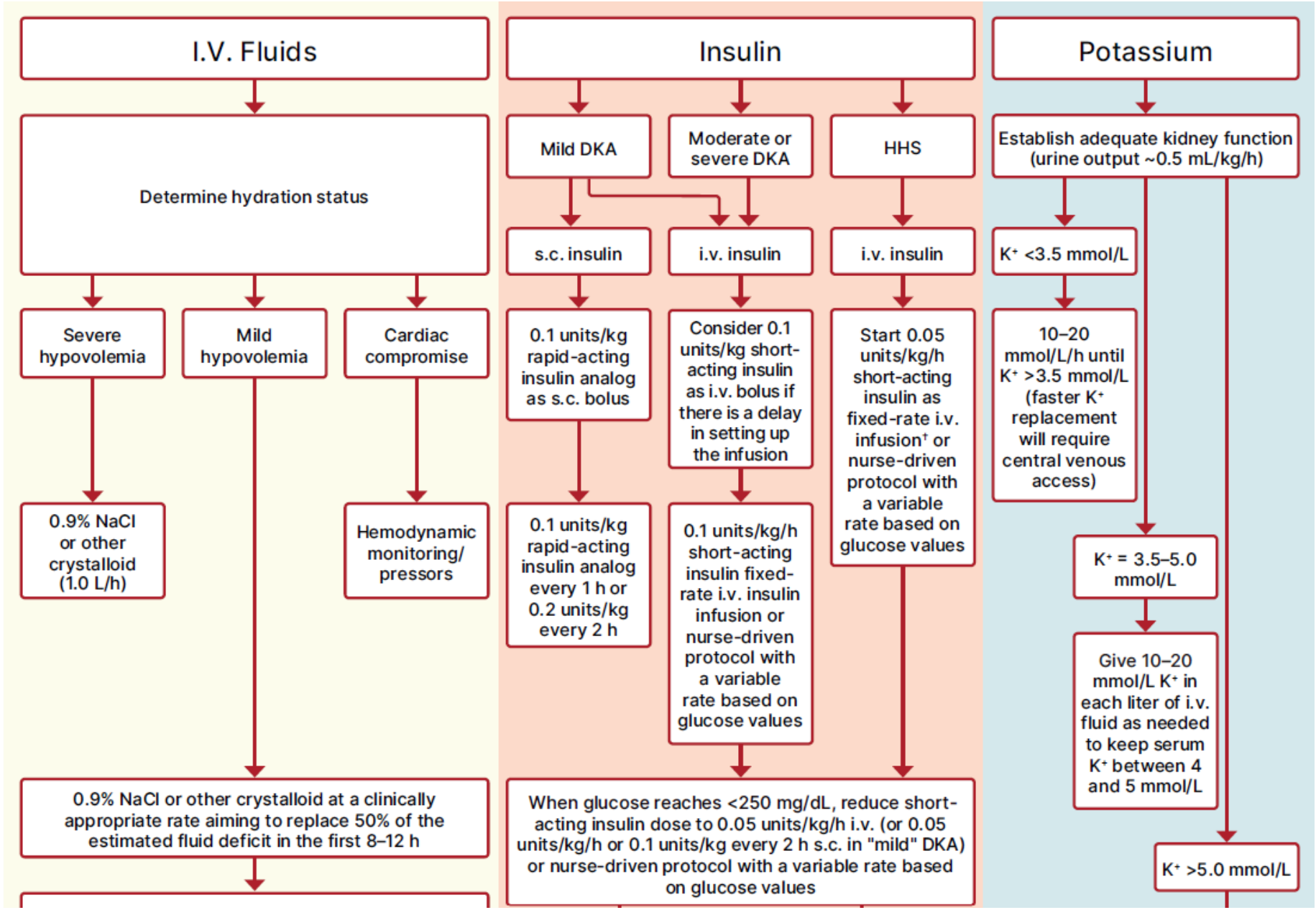
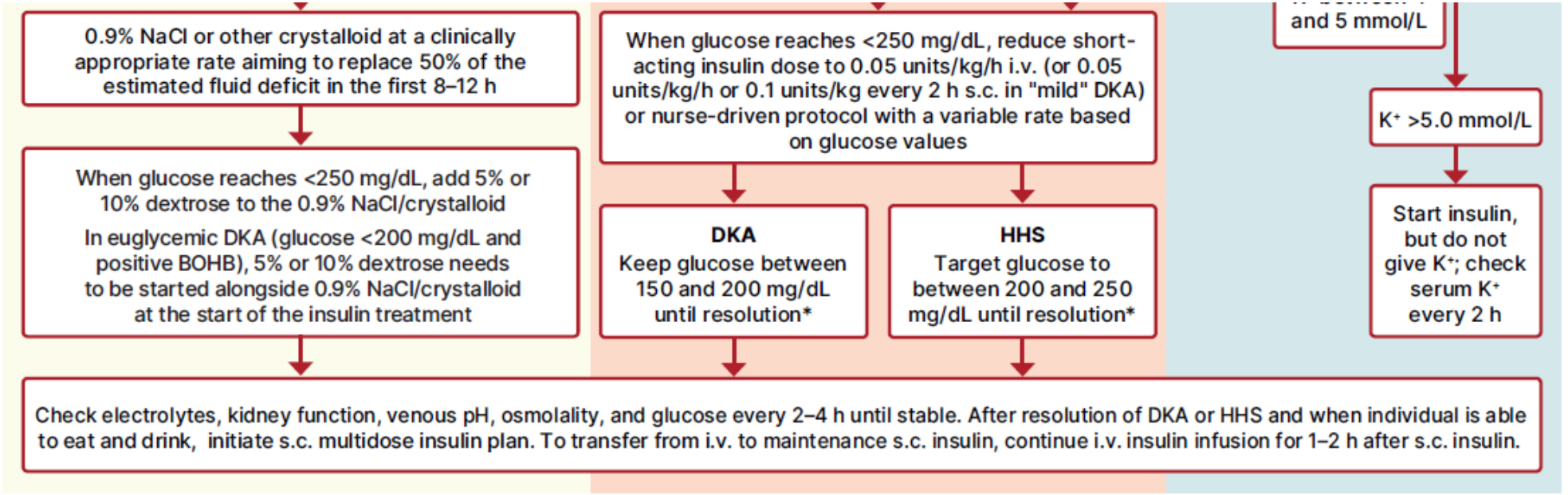


Figure 16.1
Diabetes Care in the Hospital:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S339-S355



† Some have recommended that insulin be withheld until glucose has stopped dropping with fluid administration alone.

* Definitions of resolution (use clinical judgment and do not delay discharge or level of care if these are not met):

- DKA: Venous pH >7.3 or bicarbonate >18 mmol/L and plasma/capillary ketones <0.6 mmol/L
- HHS: Calculated serum osmolality falls to <300 mOsm/kg and urine output is >0.5 mL/kg/h and glucose is <250 mg/dL

150 mg/dL = 8.3 mmol/L
200 mg/dL = 11.0 mmol/L
250 mg/dL = 13.9 mmol/L
300 mg/dL = 16.6 mmol/L

ⓘ Bicarbonate should only be considered if pH is <7.0

ⓘ Phosphate should not be given unless there is muscle weakness, respiratory compromise, and a phosphate <1.0 mmol/L or <0.32 mmol/L

Figure 16.1—Treatment pathways for diabetic ketoacidosis (DKA) and hyperglycemic hyperosmolar state (HHS). BOHB, β-hydroxybutyrate. Adapted from Umpierrez et al. (165).

Figure 16.1 (continued)
Diabetes Care in the Hospital:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S339-S355

Table 16.1—Diagnostic criteria for DKA and HHS

	DKA
Diabetes/hyperglycemia	Glucose ≥200 mg/dL (11.1 mmol/L) or prior history of diabetes
Ketosis	β-Hydroxybutyrate concentration ≥3.0 mmol/L or urine ketone strip 2+ or greater
Metabolic acidosis	pH <7.3 and/or bicarbonate concentration <18 mmol/L
	HHS
Hyperglycemia	Plasma glucose ≥600 mg/dL (33.3 mmol/L)
Hyperosmolarity	Calculated effective serum osmolality >300 mOsm/kg (calculated as [2 × Na^+ (mmol/L) + glucose (mmol/L)] or total serum osmolality >320 mOsm/kg [2 × Na^+ (mmol/L) + glucose (mmol/L) + urea (mmol/L)]
Absence of significant ketonemia	β-Hydroxybutyrate concentration <3.0 mmol/L OR urine ketone strip less than 2+
Absence of acidosis	pH ≥7.3 and bicarbonate concentration ≥15 mmol/L

Adapted from Umpierrez et al. (165).

Table 16.1
Diabetes Care in the Hospital:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S339-S355

Table 16.2—Clinical presentation in people with diabetes with DKA and HHS

DKA	HHS
Develops over hours to days	Develops over days to a week
Usually alert	Change in cognitive state common
Polyuria, polydipsia, weight loss, and dehydration	
Nausea, vomiting, and abdominal pain	Often copresenting with other acute illness
Kussmaul respiration	
One-third of hyperglycemic emergencies have a hybrid DKA-HHS presentation	

Adapted from Umpierrez et al. (165).

Table 16.1
Diabetes Care in the Hospital:
Standards of Care in Diabetes - 2026 Diabetes Care 2026;49(Suppl. 1):S339-S355

Transition from the Hospital to the Ambulatory Setting

16.18 structured discharge plan should be tailored to the individual with diabetes. **B** For those not being discharged to home, consider the capabilities of the facility for diabetes management. **E**

Section 17.

Diabetes Advocacy

(https://doi.org/10.2337/dc26-S017)

Select Diabetes Advocacy Statements

- Diabetes Care in the School Setting
- Diabetes and Driving
- Diabetes Management in Detention Facilities
- Care of Young Children with Diabetes in the Childcare and Community Setting
- Insulin Access and Affordability
- Diabetes and Employment



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